

Dr. Agarwal's Health (AGARWALEYE)

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Subject: Analyst/ Earnings Call – Transcript
Pursuant to Regulation 30 and 46(2)(oa) of the SEBI (LODR) Regulations, 2015, please find enclosed the transcript of the analyst/ earnings conference call held on February 17, 2025.

The said transcript is also available on the website of the Company at <https://dragarwals.co.in/dr-agarwals-health-care/#analyst-earnings-call>

Yours Truly,
For Dr. Agarwal's Health Care Limited

Thanikainathan Arumugam
Company Secretary and Compliance Officer

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“Dr. Agarwal 's Health Care Limited
Q3 & 9M FY25 Earnings Conference Call”
February 17, 2025

MANAGEMENT : DR. ADIL AGARWAL – WHOLE -TIME DIRECTOR AND
CHIEF EXECUTIVE OFFICER – DR. AGARWAL 'S
HEALTH CARE LIMITED
MR. YASHWANTH VENKAT – CHIEF FINANCIAL
OFFICER – DR. AGARWAL 'S HEALTH CARE LIMITED
MS. AASHNA DHARIA – HEAD OF INVESTOR

RELATIONS -- DR. AGARWAL 'S HEALTH CARE LIMITED

MODERATOR : MR. TUSHAR MANUDHANE -- MOTILAL OSWAL FINANCIAL SERVICES

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Moderator: Ladies and gentlemen, good day and welcome to the Dr. Agarwal's Health Care Limited 's Q3 and 9 Months FY25 Earnings Conference Call . As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch -tone phone.

I now hand the conference over to Mr. Tushar Manudhane from Motilal Oswal Financial Services. Thank you and over to you, sir.

Tushar Manudhane: Thank you, Ru tuja. Good morning and a warm welcome for 3Q FY25 earnings call of Dr. Agarwal's Health Care . From the management side, we have Dr. Adil Agarwal, CEO, Mr. Yashwant h Venkat, CFO and Ms. Aashna Dharia , Head of Investor Relations.

Now I hand over the call to Ms. Aashna, Head of Investor Relations at Dr. Agarwal 's. Over to you, Aashna.

Aashna Dharia: Thank you, Tushar. Good morning, everyone. We appreciate everyone joining us today for our Q3 and 9 months FY25 earnings call. Our financial results, press release and investor presentations have been released and are available on our website as well as on the exchanges. Before we continue, we want to remind everyone that this call is being recorded and the transcript will be made available on our website afterwards. Additionally, please be aware that today's discussion may include certain forward -looking statements which should be considered in the light of the risks our business faces. Please refer to the detailed statement on Page 2 of our Investor Presentation .

It is now my pleasure to hand over the call to Dr. Adil who will share his insights and remarks. Dr. Adil, over to you.

Adil Agarwal: Thank you, Aashna and good morning to all of you. I would like to welcome you all to Dr. Agarwal 's Health Care 's maiden earnings call as a publicly listed entity. I'm Dr. Adil Agarwal. I'm the CEO of Dr. Agarwal 's Health Care Limited. As you all know, the company got listed on the 4th of February, 2025 and I would like to take this opportunity first to thank all of our patients, our doctors, our employees, our shareholders, our investors and all stakeholders who have supported us in this journey.

Let me begin by providing you an update on Q3 financials and the 9 month performance of the year. We are pleased to report a strong financial performance with a 28.6% year -on-year revenue growth, reaching INR443 crores for Q3 FY 25. Our EBITDA also saw a strong growth of 26.3%, totalling INR128 crores, which resulted in EBITDA margin of 28.8% for the third quarter .

Our PAT stood at INR28 crores, reflecting a growth of 25% year -on-year, with a PAT margin of 6.4%. Now, correspondingly, we recorded a year -on-year revenue growth of 27.2%, reaching INR1,281 crores for 9 months FY25. Our EBITDA also grew by 27.5%, totalling INR356 crores, Dr. Agarwal 's Health Care Limited

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with an EBITDA margin of 27.8% in the 9 months. Our PAT stood at INR68 crores, reflecting a 26.2% year -on-year growth, with a PAT margin of 5.3% for the 9 months.

Now, this revenue growth has largely been driven by increase in surgical volumes, a shift towards high-end surgeries and enhanced patient footfalls from our business development and marketing initiatives. Our team at Dr. Agarwal's has worked tirelessly to build a network of 221 hospitals across India and Africa, serving approximately 17.7 lakh patients and performing over 2.1 lakh surgeries during the 9 months , ending December 2024.

In India, we operate through a network of 205 facilities , spread across 14 states and 4 union territories, covering 124 cities. Our presence is highly diversified with 33% of our facilities located in Tier 1 towns, 60% in other cities and 7% in

ternationally. This strategic distribution

ensures that we provide accessible and high -quality Eye Care to a broad and diverse patient base.

We have expanded our network from 209 facilities in H1 FY 2025 to 221 facilities, marking a net addition of 12 facilities for the quarter. In this year alone, we have added 42 new facilities.

Now, if you look at our business model, we operate through something which we call a hub -

and-spoke business model, which is built around three different formats of our hospitals. The largest are what we call the Tertiary hospitals, then you have the Day Care Surgical Centres, which are called the Secondary centres, and the smallest formats are called the Primary centres, or what we define as the Eye Clinics.

We work on an asset-light model, enabling us to scale effectively with minimum capex. As of December 31, 2024, we have 28 tertiary facilities and 193 spokes, which include 132 secondary and 61 primary centres. If you look at our market expansion strategy, it is designed to strengthen our presence in our core markets, while targeting under-penetrated clusters to create new growth opportunities.

In the fourth quarter, already we have launched three new facilities to-date. Looking ahead, in Q4, we are targeting to launch another 8 to 10 additional surgical facilities, with 67 located across our core states, which are; Tamil Nadu, Maharashtra, Karnataka, Andhra Pradesh and Telangana. We will also expand into the north core states of Gujarat, Jammu and Odisha. Furthermore, we plan to open seven primary facilities in quarter four, FY 2025. These strategic expansions will significantly enhance our operational reach and further strengthen our market presence across key regions.

Now, I would like to hand over to our CFO, Mr. Yashwanth Venkat, who will give us a deeper dive into our financial performance for the year.

Yashwanth Venkat: Thank you, Dr. Adil. I will start by providing a breakdown of our revenue across segments. Surgical revenues form the main pillar of our service offering, contributing over 65% to group revenue. Diagnosis, consultations and non-surgical treatments account for 14%, while the sale of optical products and pharmacy contribute to 21% of the total revenues.

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During Q3 FY 2025, we performed 72,815 surgeries, resulting in a 35.7% year-on-year growth, whereas in nine months of FY 2025, we performed about 2.1 lakh surgeries, reflecting in a 31.8% year-on-year growth. Cataract surgeries have been the major contributor, accounting for approximately 75% of the volumes, followed by refractive surgeries at around 5%. In nine months of FY 2025 volume of cataracts and refractive surgeries have grown by approximately 28% year-on-year.

The payer mix has remained relatively consistent over time, with 62% of the payments made through cash/self-paying in patients, 27% through insurance and TPA and nearly 12% from government schemes. There has been a greater focus on India, with its contribution to the overall group revenue increasing from 86.8% in nine months of FY 2024 to 89.9% in nine months of FY 2025.

Revenue from Operations India for Q3 FY 2025 stood at INR390 crores, reflecting a growth of 34.5% year-on-year. Revenue from Operations India for nine months of FY 2025 totalled to about INR1,125 crores, marking a growth of 31.7% year-on-year. The number of non-source facilities has risen from 17 facilities as of March 2022 to 59 as of December 2024, with the group predominantly expanding in Punjab, Maharashtra and Gujarat.

Mature facilities are those facilities which have been owned or operated by the group for more than three years. As of December 2024, we operated about 100 mature facilities. The revenue from mature facilities has risen by 16.6%, totalling nearly INR291 crores in the third quarter of FY 2025.

The revenue

from mature facilities for India has grown by 21.2% in Q3 FY 2025. The revenue from mature facilities has increased by 15%, reaching about INR875 crores in the nine-month period and correspondingly for India it has grown by 18.5%.

Now let's move on to discussing two major heads on the expense side - Cost of Goods Sold and doctor cost. Cost of Goods Sold in Q3 FY 2025 has been slightly upwards due to greater focus towards higher-end procedures. Out of the 1.5 lakh cataract surgeries performed, about close to 31,000 were higher-end procedures, which was about close to 21% of the overall cataract surgeries performed, up from 19.2% in FY 2024.

Doctor costs in Q3 FY 2025 were impacted by addition of new facilities and also full-year impact of branches set-up/acquired in the previous year. Other expenses include a one-time cost of nearly INR4 crores for production expenses relating to our marketing segment.

In conclusion, we are pleased to report strong financial results for Q3 FY 2025, driven by solid year-on-year growth across key metrics. Our revenue increased by 29.5%, EBITDA grew by 26.3% and PAT by 25%, reflecting in the strength of our operations and continued strategic execution.

Furthermore, our year-to-date performance demonstrates a consistent upward trajectory with a 27.2% increase in revenue, 27.5% rise in EBITDA and 26.2% growth in PAT. These results

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reflect our commitment to delivering sustained growth and operational excellence, positioning us well for continued success in the upcoming quarters. Thank you.

Moderator: Thank you very much. Ladies and gentlemen, we will now begin the question-and-answer session. The first question is from the line of Gautam from Leo Capital.

Gautam: I had a question on our centre in Chennai. So, when is this flagship centre in Chennai expected to be operational and how much revenue growth do we expect from that over our existing base?

Adil Agarwal: This facility is expected to become operational in the next 9 to 10 months. Construction is underway and we should be looking at sometime early FY 2026 to begin operations at this particular centre. We would not like to comment on what the revenue guidance for that particular centre will be, but it will be standard in line with what our current tertiary centres are expected to grow at.

Gautam: And what would be the standard growth rate for the other centres?

Adil Agarwal: So, if you look at our existing centres, our mature facilities are growing at an average of 18%. I think that's a fair estimate which we can expect from our mature facilities.

Gautam: So that is what we can expect from this as well at least?

Adil Agarwal: Yes.

Moderator: The next question is from the line of Ankit Shah, an Individual Investor.

Ankit Shah: I had one question which was on the growth rate expected over the next 2-3 years, because we are aggressively expanding plus we have old mature centres. So what is the growth both in terms of revenue and EBITDA?

The second is, today our margins are low I am assuming because we are expanding aggressively.

So, there will be some burn on the older centres which would take some time to get to a stable state. So what would be a steady-state EBITDA margin that one should look at?

Adil Agarwal: Sure, Ankit. So I will request my CFO to give you some guidance on revenue and EBITDA. But before he answers that, quickly, our same-store sales growth for the last 3 years which is defined as how you see our mature facilities have been growing by around 18%. We expect a combination of that plus whatever new centre additions we have and we will continue to grow similarly to where we have grown in the last 9 months. Yashwanth, do you want to give guidance in terms of how EBITDA margins have progressed?

Yashwanth Venkat: See, just to add to

Dr. Adil's point, in terms of revenue, the revenue from our mature facilities and the newer facility additions over the next 2-3 year perspective, should be close to about 20%. Now, in terms of EBITDA margins, we believe that the EBITDA margins would sustain at the current levels considering the fact that we will be opening about close to 45 to 50 facilities this year as well.

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Ankit Shah: And we have this listed subsidiary. So, because there will be a dilution whenever you merge that, is there any ballpark of what dilution one should look at from that perspective?

Adil Agarwal: So, we have not yet started exploring combinations with our bankers on this particular matter, but our endeavour is, in the next few months, it will start to work and then we will have some information for our shareholders in terms of what the swap issues will look like. But our endeavour is to get this done within a time span of 3 years and ideally sooner than that.

Moderator: The next question is from the line of Ankur Shah from Quasar Capital.

Ankur Shah: Congrats on the listing. Sir, three questions. Sir, one is, since our hospital model is quite different from some of the listed hospitals which are operating large hospitals, it is more sort of a retail clinical setup. So, sir, how do you see -- like, how should we read the capacity utilization? How should we see the average revenue per patient? What is the price hike which you'll expect over a period of next three years? Can you give some linkage on that?

Adil Agarwal: Sure. So, to help you understand this, we will explain to you the unit economics of the three different types of hospitals that we have. As I was mentioning on my opening statement, we have the largest format which is called the tertiary hospitals. Now, these tertiary hospitals average anywhere between 8,000 to 12,000 square feet in size.

Then you have the secondary facilities which average between 4,000 to 7,000 square feet. And then you have the clinics which average about 750 to 1,000 square feet in size. I would request Mr. Yashwanth Venkat to give you an update on what the average revenue for these facilities look like and what is the kind of capex you look at, which will give you a sense in terms of how we think about revenue and capacity utilization from some of these centres.

Yashwanth Venkat: Sure. In terms of capex, for a typical primary facility, the capex is between INR30 lakhs to

INR35 lakhs. And in terms of square feet area, between 750 up to 1,500 square feet. When the primary facility is, say, at a maturity level of about close to three years, we should be looking at somewhere between INR12 lakhs to INR15 lakhs revenue on a monthly basis with a margin about close to 15% at a store level.

This is as far as primary facilities are concerned. Secondary facilities, the capex range from about close to INR4.5 crores up to INR7 crores. In terms of square feet area, between 4,000 to 7,000 will be the square feet area. One anomaly to this is Mumbai, wherein we have secondary facilities even as low as 2,000, 2,500 square feet.

In terms of maturity profile, for a secondary facility, by the end of third year, on an average, the revenue on an annual basis will be close to about INR9 crores to INR9.5 crores with a store level operating margin between 22 % to 24%. At the end of, say, the fifth year, this revenue should trend between INR13 crores to INR15 crores, store level operating margins between 32 % to 33%.

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Coming to the largest format, the tertiary facility. Tertiary facilities are generally greater than 8,000 square feet. This is the average across India, again barring Mumbai. In terms of the set-up cost for a tertiary facility, between INR10 crores to INR12 crores on an average basis,

excluding the centres of excellence.

In terms of maturity, by the third year, typical tertiary facility, the revenue will be close to about INR10 crores to INR10.5 crores with a store level operating margins between 20 % to 22%. By the fifth year, around INR15 crores to INR16 crores in terms of revenue and in terms of store level operating margins, close to about 30 % to 32%.

Ankur Shah: Okay. And, sir, can you help me with the price hike number, let's say, at least in the last past three to four years, on an average, price hike figures?

Adil Agarwal: Yeah. So if you look at our same-store sales growth, it's a component of three factors. It's a component of patient footfalls. It's a combination of pricing. And the third thing which we do is we do something which we call a higher end surgery. So there is a product mix, which is improved there. In the last two, three years, we've seen an average price increase of about 2% to 3% per year.

Ankur Shah: 2% to 3%. Okay. Sure. And, sir, again, because it is more linked to retail format, can we know the EBITDA margin because the Ind AS, you know, it accounts for some of the rental costs. So can we get the actual margins post the rental cost?

Yashwanth Venkat: Sure. See, you just have to subtract 7% to 7.5% from the Ind AS margin to derive the actual iGAAP margins. This 7% to 7.5% has been the number over the last three years.

Ankur Shah: And that's across facility or you all have an ownership model as well?

Adil Agarwal: No. So if you see, out of the 221 facilities, 220 are leased out, only one hospital is owned. So effectively, leased is a significant component, which is what he's saying is about 7% to 7.5%, which you deduct from your Ind AS margins to arrive at your iGAAP EBITDA margins.

Ankur Shah: Okay, sure. And just last question on the structure front. I think one of the participants did ask, but I'll just try to go more detailed because let's say, both the companies are listed, both the companies are having same brand names.

Arguably, both the companies are, trading at very different valuations. And obviously, the old listed company is maybe much more profitable than the just listed in terms of margins in terms of establishment. So how would you, think of this merger, considering the minority shareholders sitting out there?

Adil Agarwal: So from management perspective, it is our fiduciary responsibility to take care of our minority shareholders in the listed subsidiary, which is why when we get down to this process, we will do a fair valuation of both the entities and based on that fair valuation, we will do a share swap of both the entities.

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Moderator: Sorry to interrupt you, Mr. Shah. May we request you to please rejoin the queue? We have other participants waiting for the turn.

Ankur Shah: Sure, sure. Thank you. Thank you so much. All the best.

Moderator: The next question is from the line of Yash from Stallion Asset. Please go ahead.

Yash: Hi, thank you for the opportunity. Just wanted to understand your target for additional number of Eye Centres in the next three years?

Adil Agarwal: So in the nine months, we have added 42 new facilities and we are starting to open about 8 to 10

new facilities in this quarter. I think that's a fair estimate of what we should expect over the next few years to expand. At least for the next two to three years, this is what we're looking for.

Yash: Okay. Got it. Got it. And any sort of plans for acquisition of smaller Eye Hospitals or any inorganic expansion that you have in your mind?

Adil Agarwal : So we predominantly are focusing only on organic expansion. That said, we continue to look at strategic opportunities if we are looking at expanding into new markets. Right now, we don't have anything for you, but as we continue to explore and as our business develops

development teams

continue to look at strong partners to work with, we will continue to explore those.

Assuming that they come at the right price and you have the right cultural fit from some of these partners, we will look at exploring those opportunities. But that said, we don't have anything right now concrete for us to look at.

Yash: Okay got it. Thank you.

Moderator: Thank you. The next question is on the line of Chintan Sheth from Girik Capital. Please go ahead.

Chintan Sheth: Hi Doctor. Thank you for the opportunity. So if I look at the network we currently have, emerging will be Punjab and the northern clusters that we acquired recently. And then Maharashtra, again, a couple of years back we acquired and now expanding into Gujarat and Rajasthan.

So our core growth area or the region will be this market or will it continue to, you know, the 50 centres which we spoke about in terms of expansion moving forward on an annualized basis.

Where should we look at the expansion to be? And then the white spaces like Bihar, Chhattisgarh and the north-eastern part and obviously Jammu and other areas. So how should one look at your strategy in terms of expansion where we will be expecting growth going forward?

Adil Agarwal : So good question. If you see our core states are the four southern states of Tamil Nadu, Andhra, Telangana and Karnataka. And we also consider Maharashtra now a core state given that we have 26 centres located across Maharashtra.

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Out of these 26 centres, 15 are located in Mumbai and the rest are across Pune, Nashik and Satara. 75% to 80% of our new centre rollouts will come in these five core clusters and if you add Kerala to this as well. The remaining 15% to 20% will come from non-core clusters like Delhi-NCR, Punjab, Gujarat, Rajasthan and UP along with Madhya Pradesh. So about 75% of new centres will come in our existing markets. The remaining 25% will come in non-core markets.

Chintan Sheth: And the inorganic opportunities, if any, which you will be targeting will be in the wide spaces like North-East, Bihar, Telangana, Bihar, Chhattisgarh area or will it continue to be in the core like Gujarat or Rajasthan where the presence is still limited where we want to expand further and try to get more market share within that geography.

Adil Agarwal : Exactly. So Chintan, the acquisition strategy is based around us acquiring hospitals and getting access to strong partners in newer markets where we don't have a presence. Once we have established ourselves in a market and we have access to strong partners, then we go back to your base expansion strategy which is organic expansion.

For example, now that we have a good set of partners for us in Mumbai, we are looking at setting up more greenfield centres. For example, we have three new facilities coming up in Mumbai and at suburb, we have a centre coming up in Vashi, we have a centre coming up in Dombivili and we have a centre coming up in Virar. Similar strategy we use across all our core markets to predominantly expand in your core clusters using an organic approach.

Acquisitions are predominantly used for us to expand into new markets like how you have described. So if we are looking at entering into certain markets like Delhi, NCR or UP, we will be looking at acquisitions to enter into those markets.

Chintan Sheth: And on Africa, we are not focusing as aggressively as we are in India. We added one centre in 9 months. But how do we, given that most of the facilities will be mature enough to generate significant cash flows, how are we kind of set up in terms of repatriating that cash flows to India or whether that cash flow stays there? If we can throw some light on it and medium-term outlook in Africa, whether we want to stay in that market, given the geopolitical uncertainty has been the his

tory of that region, if you can give some sense of that?

Adil Agarwal : So Africa has been an interesting journey for us. We commenced our Africa operations in 2012. In the last 13 years now, we have set up 16 facilities in Africa. As you can see the trend over the last 3 years, the contribution from Africa has been terribly declining and as of the last 9 months, Africa contribution is only about 10.1% of the overall group revenues.

We are not looking at any significant expansion in Africa but that said, we have some quality facilities which we have set up which are doing great clinical work across countries like Mauritius, Kenya, Tanzania, Mozambique, just to name a few. We will continue to operate these centres .

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It does not take up any management bandwidth but the advantage which we have is we have first mover advantage in many of these countries and we are doing a good amount of clinical work in these countries. We believe that African operations will continue and we will continue to grow at a moderate rate but the kind of growth which we are seeing in India will not come in Africa.

Just to give you a sense in terms of our margin profile and cash flows, how they are managed, I will request our CFO to give you a sense in terms of how the African operations are doing in terms of financial performance.

Yashwanth Venkat: In terms of margin profile, the African operations continue to operate at similar margins as of the domestic operations , one. Second key point to notice, Africa is a self-sustaining business. The last funding which went from India to Africa was about 5 years back and in terms of cash flow, over the last 3 years, nearly about 9 to 12 crores have been repatriated to India in addition to whatever the existing amounts have been used to further expand into Africa as well. So in terms of expansion, as far as Africa is concerned, we will be looking to open 2 or 3 facilities on a yearly basis.

Chintan Sheth: Sure. If I may, last question. If I look at the post -minority growth on the profitability, the minority share has increased, I believe because of the Punjab acquisition where the minority holding is higher. If you can share, how should one look at post -minority growth, till the time the consolidation is still a couple of years away from here. Because that is scaling lower than the consolidated growth we are looking at.

Adil Agarwal : Yes. I'll just take that question. As of March 2024, out of INR95 crores of PAT, INR83 crores was the number which came from the owners of the company and INR12 crores was not from the owners. That number is currently at about close to 75% and 25% is the number which we are currently looking at out of INR67.72 crores. This number going forward over a 2 or 3 year period should come down to similar numbers as of March 31st, 2024. By 2026, we should be looking at about 84% to 16% and by 2027, we should be looking at 87% or 88%.

Moderator: The next question is from the line of Dinesh Pathak from Y2 Capital. Dinesh, go ahead.

Dinesh Pathak: Can you comment on the performance of Dr Agarwal's Eye Hospital as well? And I would request you to also have some slides like you have for the other entity , which got recently listed , also have a presentation for the other entity as well highlighting the key KPIs on an ongoing basis?

Adil Agarwal : Sure, Dinesh. We will share some of those details for both the listed entity and standard of business separately. But if you look at the P&L for the listed entity, we ended 9 months till December 2024 at a top line of INR 302 crores.

Comparatively, last year, December 2023, we had done INR 241 crores, which was a growth of 25.4%. Our EBITDA margins were at 30.4% for the 9 months, which stood at INR 91.9 crores.

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This signified a growth of 32.9% over the previous 9 months. PAT for the 9 months was at INR 38.7 crores, signifying a growth of 15.2% over the last 9 months, which was at INR33.6 crores.

Dinesh Pathak: Okay. And one more request, sir. If you can , in your presentation, just call out the rental element clearly and give the IndAS EBITDAs on an ongoing basis , it will be very useful.

Adil Agarwal : Sure, Ganesh. We will do that. We will call out directly.

Moderator: The next question is from the line of Dishant Jain from Quasar Capital .

Dishant Jain : Sir, I just have one question. How does the management bifurcate the growth and the geographical location between the listed and the parent company? Since both are listed , is there any pre -agreement that has been signed between both the companies for particular states or particular regions? I just wanted to understand that part, sir.

Adil Agarwal : Sure, Dishant . That's a good question. Now, if you see the way the companies, the way operations are divided is, most of the older centres , which were set up in the state of Tamil Nadu and in Chennai particularly, are all located in the listed subsidiary. So, if you see most of the expansion which happens in Tamil Nadu, will happen in the listed subsidiary. All of the expansion outside of Tamil Nadu, which includes all our southern states, which is Andhra, Telangana, Karnataka, Maharashtra, the north centres , west centres , and Africa, is all located in

the holding company, which is Dr. Agarwal 's Health Care Limited.

So there is a clear demarcation in terms of expansion, which happens outside of Tamil Nadu in the holding company, and the listed subsidiary has all Tamil Nadu centres , which continues to grow at its current rate. We are seeing robust growth in both the regions, given strong performance by our operations team across both the entities. The listed subsidiary has grown at about 25% revenue, and our holding company also has grown at about 27.2%.

Moderator: The next question is from the line of Dhawal Khut from Jefferies . Please go ahead.

Dhawal Khut: I just need a few data points. What is the organic growth for the company for 3Q as well as 9 - month? That's my question number one. Number two, also, can you i nform about how does the pricing look across various payer mix? Let's say cash is at 100. What will TPA be at, and what will the government payers be at? Those are my two questions.

Adil Agarwal : I'll answer your second question first, which is your payer mix. We'll update on the payer mix, and then I will come back to your first question in terms of how our organic centres or what we call the mature centres are growing. Yashwanth?

Yashwanth Venkat: In terms of payer mix, over the last three years, the paye r mix has remained relatively consistent, with about close to 88% coming through cash and self -paying patients, and about close to 26 % to 27% through insurance and TPA, and the rest 11 % to 12% from the government schemes.

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Now, one key point to notice, how we break down our business is that we break down our business into two categories, mature facilities and emerging facilities. Those facilities which have been owned or operated by us for more than three years are mature facilities. So the growth in mature facilities for the 9 -month period has been close to about 15% on an overall basis. The growth in mature facilities from India, have been by about 18.5%. This growth in mature facilities is akin to your same -store sales growth.

Dhawal Khut: Yashwanth , what I was trying to know is, like -for-like procedure, let's say cash has a certain rate, then what is the discount for the TPA patient and for the government patient? So, the pricing for like -for-like procedure across various peer groups.

Dr. Adil Agarwal : So, you are asking is there any discount f

or insurance patients? There is no discount for insurance

patients. Whatever the provider's charges are, they pay for particular procedures ; that is what we effectively do. There is no discount on that.

Dhawal Khut: So, the net realization for same procedure is comparable for TPA as well as cash. Is that correct understanding?

Yashwanth Venkat : See, as far as TPA is concerned, the two major parties are Medi Assist and MD India Health Care TPA. In terms of realization for those two parties, it is close to about INR 35,000, about INR 32,500 to INR35,000. Now coming to the overall contract mix, when you look at both cash as well as TPA, the net realization is close to about INR 38,000. So in terms of realizations, as far as TPA and the overall contracts are concerned, the difference will be close to about 8 % to 12%.

Dhawal Khut: Okay, got it. And for government channel, what will that difference be?

Yashwanth Venakt : For government, again, it varies from scheme to scheme. In India, the overall contribution from government schemes is close t o about only 6%.

Dhawal Khut: Okay. And on the organic growth side, do you have the numbers available?

Adil Agarwal: So, the way we categorize our business is , we classify them as whether it has an organic centre or we have acquired a particular centre . If it is less than three years old, we classify it as an emerging centre . And if it is greater than three years old, we classify it as a mature centre . So, as Yashwanth was alluding to, out of the 221 centres, now we have 100 mature facilities and 121 emerging facilities. These 100 mature facilities are growing at a CAGR of about 15% over the last nine months.

And that's the way we view the business and what matt ers is what are your mature centres growing at , because it is the same yardstick now. Whatever is the route through which that centre has been onboarded, the same yardstick is probably used to look at growth going forward. And those centres are now growing at about 15% year -on-year.

Moderator: The next question is from the line of Ashish, an Individual Investor .

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Ashish : My question is with respect to the future growth strategy with respect to opening the organic centres and virtual acquisition of hospitals or eye care centres in the rural territories, as you mentioned. And also, the second part of this question is...

Adil Agarwal : Ashish, just pardon me. Can you just speak a little louder because I couldn't hear the question clearly ?

Ashish: So my question is with respect to the acquisition strategy going forward with respect to the newer territories. As you mentioned that mainly the acquisition is in the newer territories, say, if you were to venture out into Delhi or UP or other geographies. So in such a scenario when you acquire a new hospital, does it include equity dilution as well to the owners of the newer hospital or these are like more of a cash trade or how does it happen?

And the second question is with respect to the growth strategy, is it fair to say that most of the growth in future would be through organic centres vis-a-vis through acquisition of hospitals in the newer geography?

Adil Agarwal : Absolutely correct. Our future will predominantly revolve around and which is won't be demonstrated over the last 9 months that most of the expansion will come through the organic growth. Only when we look at acquisitions to enter new markets we will consider doing acquisition. For example, what we have done in Varanasi early this year.

But just to give you a sense in terms of how we think about acquisitions and how that works, Yashwanth will give you an update.

Yashwanth Venkat : In terms of the structure, almost all of our acquisitions are done through business transfer agreements wherein we acquire 100% of the business on day one. However, the

payment is

structured as follows :70% of the total payout is done on day one. The rest 30% is paid out over a four year period linked to certain revenue milestones.

Ashish: I understand. Is there any equity issuance as well of the acquiring company to the owners, doctors of the acquisition target?

Adil Agarwal : So far we have not done any equity dilution for any of the acquisitions that we have done. All of them have been predominantly cash payouts.

Ashish: Sure, sir. Thank you so much.

Moderator: Thank you. The next question is from the line of Dishant Jain from Quasar Capital. Please go ahead.

Dishant Jain: Thanks for the opportunity again, sir. Can you just help me with the growth of healthcare and hospital in Tamil Nadu region particularly, is it possible?

Adil Agarwal : Yes please give us a second. So if you see right now the contribution from Tamil Nadu is approximately 40% of the overall group business , which comes from Tamil Nadu. Tamil Nadu

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has about 76 facilities if you look at the overall network. The second largest contributor is from Maharashtra which is about 15.7%, Karnataka is about 11.5%, Telangana is about 8.4% and Andhra Pradesh is about 6%.

So effectively these five states contribute about 82% of our growth revenues. From this, the biggest contributor is obviously Chennai which stands at about 18.7%. Now to give you a sense in terms of what the Tamil Nadu business is effectively growing at, you can compare it to what is happening in our listed subsidiary . Our listed subsidiary predominantly has all the Tamil Nadu centres and that entity is growing at about 25 % which is roughly around the range in terms of what our Tamil Nadu business is growing at.

See the last month 9 months INR 300 crores of revenue which has come in our listed sub is about 25% growth and most of the centres there are all located in Tamil Nadu which will approximately give you a sense in terms of where our Tamil Nadu business is growing at.

Dishant Jain: Okay. Fair enough. Thank you.

Moderator: Thank you. The next question is from the line of Tushar Manudhane from Motilal Oswal. Please go ahead.

Tushar Manudhane : So just wanted to understand how the competition is shaping us as far as the organized eye care chains are concerned across India and how are we better than the other just in case if you want to attribute key aspects around it?

Adil Agarwal : So, Tushar, this is obviously a good competitive space. You have players like ASG, you have Centre for Sight, we have Maxi vision, we have Eye Foundation which is there in the South.

Other than these, you also have a lot of regional players like Arvind eye hospital which is based in Tamil Nadu.

There is a hospital in Chennai called Sankara Nethralaya which has been doing a lot of great work. You have Narayana Nethralaya in Karnataka and LV Prasad in Hyderabad. These are some of the big names which are doing a lot of good work. We continue to focus on our growth strategies and we believe that we are the largest both in terms of revenue and the number of centres that we have.

I would not like to comment on the other hospitals, but I know a lot of these doctors very well and a lot of these promoters and they are doing a great amount of work and they are also

continuing to grow. I think the overall market is growing at a CAGR of 12% to 14% over the next few years and we believe the current number of centres which across all our organised players only have about 13% to 15% of the overall market which signifies that there is significant room for all the organised players to continue to grow at a significant rate.

Tushar Manudhane : Got it sir. And if you could further elaborate on this, apart from cataract surgeries which could be a big growth driver in terms of surgeries for the next 4 years to 5 years

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Adil Agarwal : So obviously cataract continues to be the biggest contributor to our group revenue. 76% of our total surgical volumes come predominantly from cataract surgeries, but that said other than cataract surgeries we also have refractive surgeries which have increased significantly. About 5.2% of all the surgeries performed is coming from refractive surgeries and that is continuing to be a significant contributor to our group revenue.

Other than this we have other surgeries like retinal surgeries, we have retinal injections, we have glaucoma surgeries and a bunch of other surgeries which contribute about 20.9%. The trend that we have been seeing in the last few years is both cataract and contribution from refractive surgeries is continue to increase, the growth in refractive surgeries is actually been quite significant.

We have added 43 new refractive machines in the last couple of years and both are again contributing to a significant increase in our top line growth coming from refractive procedures.

Tushar Manudhane : Is it more to do with the patient's inclination to eliminate the specks or because there has been mixed opinion at least as far as the survey goes in terms of having specs or not having specs?

Adil Agarwal : So two to three reasons which contributed . One is obviously there has been increase in patient awareness of some of these procedures. We have been teaching our patients and have been educating our patients on the safety of some of these procedures. Second, as you have an influx of better machines coming into the country and your surgeons are getting trained they are actually able to perform better procedures.

Third, there has been from a demographic perspective since COVID because of increase in screen usage time there has been an increase in refractive errors as well. Some of these factors have contributed to increase in refractive procedures and fourth is obviously with increase in consumption power patients are finding some of these procedures a little bit more affordable and they are opting for some of these procedures.

Last but not the least I think patients have started to take their health a lot more seriously.

Similarly eye health falls into that category which is why you are seeing patients opt for some of these procedures.

Tushar Manudhane : Got it sir. Thanks for that.

Moderator: Thank you. The next question is from the line of Sriram R a n Individual Investor. Please go ahead.

Sriram R: Thank you for the opportunity. Sir I have a question on Dr. Agarwal's Eye Hospital the existing listed entities. Now if I look at the historical financials of the entity, pre COVID the growth has been in single digit. Now post COVID there has been a surge in revenue. I just want to understand what happened post COVID for the overall group as such?

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Adil Agarwal : I think this has been a trend not just in Agarwal per se, but across all the other eye care players and all the other Health Care players per se. One of the trends which we did see is if you see in early on during COVID, our Chairman, Dr. Amar Agarwal gave us instructions to keep all the hospitals open. So we were one of the first hospitals to keep ourselves open.

Second, there has been a trend where patients have started to move away from smaller eye clinics and they've started to gravitate more towards the larger corporate -based hospitals. So you are starting to see better patient footfall coming in. As we continue to expand our footprint using our hub -and-spoke model, you have started to increase market share in many of these regions that we are present in.

So you're starting to see an increase in the revenue growth and footfall coming in because of that. Last but not the least, as we move towards

higher -end procedures, the average realization

in many of our different surgeries have also started to increase, which is also yielding to higher top-line. These are some of the factors which we have seen has changed in the last four to five

years, which is why you're seeing that kind of growth across both our entities.

Sriram R: Okay, so that's helpful. Now, if I were to bifurcate this growth into footfall and realization, how much would that be like how much would be the growth due to footfall and how much ballpark figures?

Adil Agarwal : So, just to give you a sense. I would say about 6% to 7% is coming from increase in footfall. 2 % to 3% is coming from pricing and about 6% to 7% would say is coming from better realizations.

Yashwanth, do you want to add on to this?

Yashwanth Venkat: Yeah, as Dr. Adil was explaining earlier, over a 3-year period from FY '22 to FY '24, in terms of the revenue per mature facility, we have grown at a CAGR of about 18%. This is akin to your same -store sales growth. If you have to break down this 18 %, this 18% can be broken down into about 7% to 8% because of increase in volumes. About close to 3% from increase in prices. And the rest 7% coming from due to the effect of premiumization.

Sriram R: Okay, sir. Okay. That's super helpful. Thank you so much. All the best.

Moderator: Thank you. The next question is from the line of Dhawal Khut from Jefferies. Please go ahead.

Dhawal Khut: Yes, hi. Thanks for taking my question again. Wanted to know what is the current ticket size for cataract, refractive, and the other surgeries that we have, maybe for quarter or 9 month? And second is, what will be the gross margin hierarchy for these three segments versus the corporate average?

Adil Agarwal: Sure, so if you look at our cataract surgery, we have three different forms of cataract surgery.

You have the lower end procedures, you have mid -end procedures, and you have the higher end procedures. But at a overall group level, our average cataract surgery is at about INR 38,000.

Refractive surgeries, which are again, a significant contributor, are about INR 75,000 to INR 80,000 are our average realization coming in from refractive procedures.

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If you look at the margin profile of the three main revenue items for us, one will be surgeries, second would be optical, and third would be pharmacies. I would request our CFO to just give you an update on what the gross margin profile of the three different categories are.

Yashwanth Venkat: At an overall level, the gross margins are between 77.5% to 77.7%. For surgeries, especially for cataracts, it is between 82% to 83%. For refractive, it is around 75%. For optical, it is close to 65%, as far as the gross margins are concerned. And pharmacy, between 45% to 50%. So on a blended basis, the gross margins are close to 77.5% to 77.7%.

Dhawal Khut: Just a follow -up, so pharmacy, usually, retailers don't have more than, let's say, 20% to 25% of a margin. So how come we are able to get that 45%, 50%? Is it a tender -driven procurement or something else that I'm missing? Because ideally, our margins should be in line with those of retailers, right?

Adil Agarwal: So if you see our pharmacies, we only focus on eye products. We don't like most of the other retailers sell all kinds of products, right? So given that we're only focused on eye products, given our economy of scale, we're also able to negotiate better prices with our vendors, which is why we believe our margins are a little higher compared to other retailers.

Dhawal Khut: Do we directly procure from various manufacturers or is it, we go through the regular channels and we procure it from the distributors?

Adil Agarwal: Both, we do both.

Dhawal Khut: Okay, thank you. Thanks for answering my questions.

Moderator: Thank you, the next question is

from the line of Ashish, an Individual Investor, please go ahead.

Ashish : Hello, sir, thanks for the opportunity again. My question was with respect to expansion by many of the hospitals as well, into new geographies, new facilities, and into day care procedures as well. So how would the eye care teams compete vis -a-vis with respect to main headline hospitals?

Adil Agarwal : So there is a fundamental differentiator between us and the multi -speciality hospitals, Ashish. The multi -speciality hospitals predominantly are much larger in terms of, the size and format, and it's focused across different specialties like cardiac sciences, ortho, oncology, etc. We are predominantly focused only on eye care. We work on a hub -and-spoke model. Our modus operandi is predominantly day care.

Patients come in, they get their procedures done, and they leave the same day. It is a specialty business, so it is difficult to compare in terms of what is happening here versus what is happening with the multi -speciality hospitals. We have specialists across all different specialties of ophthalmology, and if you see how we have grown over the last few years, it's a combination of both existing and setting up new centres .

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We go a lot more deeper into existing markets and continue to keep rolling out more centres . Multispecialty hospitals typically work on an operating bed model, which is, they have a metric called Arcot. We don't have any of those metrics available, so our business model is a completely different from the multispecialty hospitals.

Ashish : Sure, so my question was basically with respect to the premium surgical procedures, where the margins are also high and where the 67% of the remaining growth of the balance 18% is coming in, so would there be competition from that perspective into that premium surgery model? That's what precisely I was looking at?

Adil Agarwal : Okay, so you're saying compared to the other eye care players and other players from that perspective. So we believe that we have some of the best clinicians working with us who continue to deliver outstanding clinical performance. We also work with some of the best equipment manufacturers and have high -quality equipment available at our centres . For example, in cataract surgeries, we work with the likes of companies like Alcon and J&J. For refractive procedures, we have the best ZEISS machines, Alcon machines, and J&J machines, again. We see many more surgeons that are very well -trained in performing these procedures, and we continue to deliver best results as long as we continue to deliver quality clinical outcomes. We believe that patients will come to us and will be happy with our performance.

Ashish : Sure, sir. Thank you so much.

Moderator: Thank you. Ladies and gentlemen, due to time constraints, that was the last question for today. I would now like to hand the conference over to Mr. Adil Agarwal for closing comments.

Adil Agarwal: So thank you, everyone, for taking the time. I'm very pleased to report strong financial results for quarter 3 of FY '25, which has been driven by solid year -on-year growth across all metrics. Given our revenue increase by 29.5%, our EBITDA grew by 23%, and PAT grew by 25%, this reflects the strength of our operations and continued strategic execution.

We believe that we will continue to show strong performance as we progress over the next few years, and thank you all for the support. Thanks once again for taking the time, and we'll see you next time.

Moderator: Thank you. On behalf of Dr. Agarwal's Health Care Limited, that concludes the conference call. Thank you for joining us, and you may now disconnect your lines.

Call: Jun 2025

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June 02, 2025

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SCRIP Code: 544350 Symbol: AGARWALEYE

Subject: Analyst/ Earnings Call – Transcript

Pursuant to Regulation 30 and 46(2)(oa) of the SEBI (LODR) Regulations, 2015, please find enclosed the transcript of the analyst/ earnings conference call held on May 29, 2025.

The said transcript is also available on the website of the Company at <https://dragarwals.co.in/dr-agarwals-health-care/#analyst-earnings-call>

Yours Truly,

For Dr. Agarwal's Health Care Limited

Thanikainathan Arumugam

Company Secretary and Compliance Officer

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“Dr. Agarwal's Health Care Limited
Q4 & FY'25 Earnings Conference Call ”
May 29, 2025

MANAGEMENT : DR. ADIL AGARWAL –CHIEF EXECUTIVE OFFICER –
DR. AGARWAL 'S HEALTH CARE LIMITED
DR. ASHAR AGARWAL – CHIEF BUSINESS OFFICER –
DR. AGARWAL 'S HEALTH CARE LIMITED
MR. RAHUL AGARWAL – CHIEF OPERATING OFFICER
– DR. AGARWAL 'S HEALTH CARE LIMITED
MR. YASHWANTH VENKAT – CHIEF FINANCIAL
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MS. AASHNA DHARIA – HEAD INVESTOR RELATIONS –

DR. AGARWAL 'S HEALTH CARE LIMITED

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Moderator: Ladies and gentlemen, good day and welcome to the Q4 and F Y'25 Earning Conference Call hosted by Dr. Agarwal's Health Care Limited. As a reminder, all lines will be on listen -only mode and there will be an opportunity for you to ask questions at the end of today's presentation. Should you need assistance during the conference call, please signal an operator by pressing star and zero on your touch -tone phone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Aashna Dharia, Head Investor Relations from Dr. Agarwal 's Health Care. Thank you and over to you, ma'am.

Aashna Dharia: Thank you, Rhea. A very good afternoon, ladies and gentlemen. Welcome to Dr. Agarwal 's Healthcare's Q4 F Y '25 and F Y '25 earnings call . From the management side, we have Dr. Adil Agarwal , CEO, Mr. Asha r Agarwal , Chief Business Officer, Mr. Rahul Agarwal , Chief Operating Officer, and Mr. Yashwant h Venkat, CFO. We have released the financial results, press release, and investor presentation, all of which are available on our website and the exchanges as well. Before we continue, we want to remind everyone that this call is being recorded, and the transcript will be made available on our website afterwards.

Additionally, please be aware that today's discussion may include certain forward -looking statements which should be considered in light of the risks of business spaces. Please refer to the detailed statement on Page 2 of the investor presentation. It is now my pleasure to hand over the call to Dr. Adil, our Chief Executive Officer, who will share his opening remarks and insights. Dr. Adil, over to you.

Adil Agarwal: Thank you, Aashna. Good afternoon to all of you and a warm welcome to our fourth quarter FY '25 and the annual FY '25 earnings call for Dr. Agarwal 's Health Care Limited. Let me begin by providing you all an update of our full year financial ' 25 performance and our Q4 FY '25 performance.

We are pleased to report a strong financial performance for the full year FY '25. Our total income recorded a growth of 27.6% year -on-year to INR 1,757 crores and revenue from operations grew by 28.4% year -on-year to INR 1,711 crores. Our EBITDA increased by 23.6% to INR502 crores, translating into an EBITDA margin of 28.6% for the year. Our profit after tax stood at INR 110 crores, reflecting a 16% year -on-year growth with a PAT margin of 6.3%. Revenue growth has been largely driven by addition of new facilities as well as growth in our existing facilities. This growth came through an increase in premiumization of surgeries, increase in our surgical volumes, and improved surgical conversions due to our business development and marketing initiatives. Driven by strong operational execution and improvement in our material mix, our gross profit for FY '25 grew 27.2% to INR 1,368 crores, resulting in a gross margin of 77.9%.

Manpower costs for the year totaled INR 574 crores, up by 28.4% compared to last year. This was driven by strategic hiring and increase in our workforce due to addition of new facilities. The EBITDA margin for the year was slightly impacted by increase in other expenses,

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predominantly on account of increase in our marketing costs relating to our brand ambassador and one -time expenses.

Now, moving on to the fourth quarter FY '25, we delivered a strong financial performance in the fourth quarter, with total income growing by 28.9% to INR 476 crores and revenue from operations grew by 31.9% to INR460 crores. EBITDA for the fourth quarter stood at INR 145 crores, reflecting a growth of 13.9%, translating into an EBITDA margin of 30.8% for the quarter. PAT came in at INR43 crores, up 3% year -on-year, reflecting a PAT margin of 8.9% for the fourth quarter.

Our gross ma

rgins for the quarter were flat at 78.3% despite shift towards high-end surgeries.

Our manpower costs for the quarter were INR51 crores, 33.6% increase over the fourth quarter of the previous year. This was driven again by strategic hiring and increase in our workforce due to addition of a large number of facilities in the fourth quarter.

The decline in our EBITDA margins was primarily driven by increase in certain expenses such as our I PO expenses, marketing expenses related to our brand ambassador, and a decline in other income to the tune of INR 8.4 crores.

Now, moving on to our footprint, we have built a network of over 236 facilities which includes 28 hubs and 208 spokes across India and Africa, serving approximately 24.3 lakh patients and performing over 2.8 lakh surgeries during the 12 months ended March 2025. In India, we operate through a network of 218 facilities spread across 14 states and four union territories covering 129 cities. Our presence is highly diversified with 32% of our facilities located in Tier 1 cities, 60% in other cities, and 8% of our centers are located internationally in the continent of Africa.

Now, during FY 2025, we added 59 facilities in this last 12 months, which comprised of 32 primary facilities, 25 secondary facilities, and 2 large tertiary facilities. In the fourth quarter of FY '25 alone, we added 17 new facilities comprising of eight primary and nine secondary centers. Additionally, as on date, we have successfully already launched 10 new green facilities to date, comprising four primary, five secondary, and one tertiary center. We would

also like to highlight a key milestone in our growth journey in northern India.

We have now forayed into Delhi NCR, the heart of the country, with the launch of our first greenfield facility there. We have onboarded Dr. Jeew an Titiyal, who is one of the most renowned ophthalmologists in the country and was previously the head of the renowned AIIMS Institute. He's also a Padma Shri awardee and will be driving growth in our Delhi facility.

Looking ahead, in FY ' 26, we are targeting a launch of 55 to 60 facilities, with 25 to 30 of them being surgical facilities, and the balance will be clinics. 70% of this expansion will be in our core geographies of Tamil Nadu, Andhra Pradesh, Telangana, Karnataka, and Maharashtra.

Now, moving on to clinical excellence, there has been a significant focus around the firm to ensure we strive for clinical excellence while we continue to scale. This is reflected in the Dr. Agarwal's Healthcare Limited

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increase in contribution coming in from high-end cataract surgeries, which has moved up from 19.2% in FY ' 24 to 22.5% in FY '25. Further, we seek to use specialized equipment and techniques to strengthen our diagnosis capabilities , as well as our surgical outcomes.

We have already installed femto -cataract machines, which is the laser cataract machine in cities like Mumbai and Chennai, and have recently added these high -end machines to Bangalore and Hyderabad as well. We have also seen an uptick in the refractive surgery segment, where we witnessed a 44% overall growth in the number of surgeries, and we have done 15,989 refractive surgeries in the year FY ' 25.

As part of our refractive services expansion, we have also added new examiner laser machines in Chennai, Mumbai, and Bangalore. We also plan to add SMILE technology in a few of our key centers, which will further enhance our refractive capabilities. Now, I would like to hand it over to our group CFO, Mr. Yashwanth Venkat , who will take a deep dive into our financial performance.

Yashwanth Venkat: Thank you, Dr. Adil. I'll start by providing an overview of the operational metrics. In Q4 FY '25, we performed 68,724 surgeries, resulting in a 17.4% year -on-year growth. Cataract surgeries have been the major contributor, accounting for approximately 71%, followed by refra

ctive surgeries at around 6.5%.

In Q4 FY ' 25, volume of cataract and refractive surgeries have grown by approximately 13% year-on-year, while other surgeries have experienced a higher growth rate of 36.3%. The payer mix was distributed at 70% via cash, 25% through insurance and TPA, and 5% from government schemes .

Now, in FY ' 25, the surgical revenues form the main pillar of our service offering, contributing to over 65% to group revenue. Diagnosis, consultations, and non-surgical treatments account for 14%, while the sale of optical products and pharmacy contributes 21% to total revenues.

In FY ' 25, we performed 282,326 surgeries, reflecting a 28% Y oY growth. Again, here, as reiterated earlier, cataract surgeries have been the major contributor, accounting for approximately 73%, followed by refractive surgeries at around 6%. In FY ' 25, volume of cataract and refractive surgeries grew by approximately about 27%, while other surgeries experienced a higher growth rate of 42%. The payer mix for FY ' 25 has remained consistent, with around 64% of payments coming through cash, 26% through insurance and TPA, and 10% from government schemes.

Moving on to a few points on financial performance, the revenue from operations at India for Q4 FY ' 25 stood at INR413 crores, reflecting a growth of 34.2% year -on-year. Revenue from mature facilities rose by about 28.9%, totaling INR332 crores in Q4 FY ' 25. Just a small recap, mature facilities are those facilities which have been owned or operated by the company for

more than 3 years. The gross profit margin remained stable in Q4 FY ' 25, despite movement to higher -end surgeries.

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There has been greater focus on India, with its contribution to the overall group revenue increasing from 87.2% in FY ' 24 to 89.9% in FY ' 25. Revenue from operations in India for FY '25 totaled INR1538 crores, marking a growth of 32.3% year -on-year. As of March 2025, we operate about 103 mature facilities across India and Africa. Revenue from mature facilities increased by 18.5%, reaching a total of INR1206 crores in FY ' 25.

Our CFOs to EBITDA were at healthy levels at 77%, which excludes IPO reimbursements, which we have subsequently received in this quarter. I also want to touch base on key one -off costs which occurred in FY ' 25. There was a one -time spend for banker fees and legal fees towards a strategic acquisition, totaling to about INR6.2 cr ores.

IPO costs to the tune of about INR1.5 crores. Brand ambassador and rated production costs to the tune of approximately INR5.5 crores. Increase in ESOP expenses was to the tune of about INR2 crores for the year, totaling about close to INR8 crores for the year.

Now, just to summarize, we are pleased to report strong financial results for FY ' 25, driven by solid year -on-year growth across key metrics. Looking ahead, we remain confident in our growth trajectory for FY ' 26, supported by robust demand fundamentals, continued network expansion, and operational efficiencies. We expect to clock 20% plus revenue growth in FY '26, driven by strengthening presence in existing micro-markets, foray into new micro -markets, and increased adoption of high-end surgeries.

We expect our EBITDA margin to remain stable as ongoing greenfield investments continue to impact profitability. We also expect the profit after tax numbers to rise by 35% plus.

Adil Agarwal: Thank you, everyone. We will open it up to questions now.

Moderator: Thank you very much. We will now begin the question and answers session. First question is from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Thanks for the opportunity. Sir, just comparing FY ' 25 and ' 24's payor mix, so to say, the cash component seems to have increased sort of meaningfully compared to insurance and TPA . Any broad color you would like

to share on this aspect?

Adil Agarwal: Yes. So I'm just going to invite our Chief Operating Officer, Rahul Agarwal, to give you a break up of the payables.

Rahul Agarwal: Okay. Hi, this is Rahul. Well, from our overall perspective, there's not been a significant change. The year before, we were around 27%, this year at 26% from our overall perspective. If you look at our private insurances and the government, first let me go to the government.

Government last year was 11. 7% which this year has come down to 10%.

On the other insurance and TPA, overall from 27.3%, it's down to 26%. So b road, at the overall level, we are continuing. Our private insurances continue to be very strong. That's where a large part of our growth is to happen. On the government side, we continue to remain slightly cautious. Some of the payments get delayed. We want to be cautious on those sides in

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some of the states where some payments get delayed. Otherwise, broadly across India, our insurances remain consistent.

Tushar Manudhane: In fact, cash component has would have shown maximum growth almost from as a percentage of sales as well from 60.7 to 63.8 or even in absolute terms from INR 808 crore to INR 1092 crore. So is it more to do with getting into the Tier two cities? That is what is driving the cash component. Is that the appropriate way to think about this?

Rahul Agarwal: No, not really. I think the way to look at this is, in some cases, our products don't get insurance. So the product business, when that goes, that is not under insurance. Consultancy fees is not under insurance. Some of these aspects, when they go, you will not see insurances. Some part of the investigation also does not come under insurance. It's actually the surgical business. Also, ye s, refractive allowance is not under insurance.

So these parts, when they're growing, some of the new technologies, like when we talk about femto cataracts, these are growing very fast. That is also not under insurance. So you'll see a lot of these portions of the business growing very fast. And that's a percentage of the business which doesn't get covered at that either . Otherwise, as I mentioned earlier, the government business, where we are slightly cautious, that also contributes to the private business growing faster.

Tushar Manudhane: Understood. So secondly, there's a number of facilities which sort of you tend to add in FY '26 on an organic basis and subsequent amount of investment for that, if you could share .

Adil Agarwal: So in FY '25, we added 59 new centers. Our plan is to add about 55 new centers in FY '26.

Now, in FY '25, we spent a total capex of about 220 crores towards new greenfields and eye clinics as well, right. The plan is to spend approximately INR 310 crores , and most of this will be around setting up new centers across the country.

Tushar Manudhane: So INR 310 crores?

Adil Agarwal: And 70% of these new centers will come up in the southern states of India, along with Maharashtra.

Yashwanth Venkat: The capex number mentioned by Dr. Adil, I'll just give you a slightly detailed breakup about INR 180 crores to INR 200 crores will be towards new greenfield capex. Renovation and relocation will be about close to INR 50 crores. Then we are also investing in new technology and growth capex for the existing that will be to the tune of about INR 40 crores to INR 45 crores. Then maintenance capex for existing facilities, which were close to about INR 11 crores last year, that number will move to close to about INR 16 crores this year. So , a broad breakup of about INR 310 crores.

Tushar Manudhane: Understood. Thanks. And just one last, if I may. So this brand ambassador related cost, how much of that came in the fourth quarter of FY '25? And subsequently in FY '26, shall this be considered as the one -off for FY '25 and then the amount

is going to be relatively lesser in FY

'26 or this is something which will sort of have a recurring cost going forward?

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Yashwanth Venkat : Okay, I'll just break this cost up into two parts , Tushar . One is for the brand ambassador. Second is for the production cost for the two ads. The brand ambassador cost is close to about INR 8 crores , plus GST , plus the production cost will be to the tune of about INR 5.5 crores.

This brand ambassador cost is for a period of close to about 2 years. A portion of that has been taken in FY '25, about 50% has already been taken in FY '25. We will not have any recurring production cost. So you can consider this production cost as a one -time hit to the P&L , Tushar .

Tushar Manudhane: And out of that, how much was for fourth quarter in specific?

Yashwanth Venkat : Fourth quarter towards the brand ambassador, it was close to about INR 1.2 crores. I'll come back to you on the exact production cost.

Tushar Manudhane: All right, thank you.

Adil Agarwal: Thank you.

Moderator: Thank you. Next question is from the line of Binay Singh from Morgan Stanley. Please go ahead.

Adil Agarwal: Hi, Binay .

Binay Singh: Hi, team. Congrats on a good set of numbers. Just a follow -up from the earlier question. Could you also give us what is the capex that your subsidiary is doing between this number of capex that you gave? How much is at the subsidiary level, the Dr. Agarwal Eye Hospital?

Yashwanth Venkat: So basically, this capex , whatever we have mentioned, Binay, just includes the newer facilities which will be taking it up for the subsidiary. This excludes the capex requirements for the new flagship facility. Out of this INR 310 crores, nearly about close to INR 60 crores will be towards the subsidiary AEHL. On top of this, for the new flagship facility, we'll be spending around close to about INR 70 crores to INR 80 crores.

Binay Singh: Okay. So in a way, at the subsidiary level, the capex is INR 60 crores plus INR 70 crores , INR 130 crores at the subsidiary level. Is that right, understanding?

Yashwanth Venkat: Out of the INR 310, INR 60 crores will be for growth capex , plus newer greenfields. The additional INR 70 crores will be over and above this INR 310 crores, which will be spending for the flagship facility.

Binay Singh: Okay. Okay. So at the subsidiary level, in their books, the capex is INR 70 crores at the subsidiary level.

Yashwanth Venkat: Yes. Yes.

Binay Singh: And then secondly, could you also give us a breakdown on minority interest? We've seen a quite sharp jump. And also linked to that, the Thind financials, because we had talked about March quarter being very big for Thind, how? Because it seems like it was exceptionally large for them.

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Adil Agarwal: Yes. So from a PAT perspective, approximately 76% of the earnings is attributable to the

owners, and the remaining 24% is towards the minorities. That's predominantly because the Thind facility that we acquired last year, it's a 51 :49 JV, and 49% of that contributes to about INR10.96 crores of PAT. So that is significantly driven the minority earnings. So that's why you're seeing a bit of that skew, which is 76 24.

Binay Singh: So Thind had a INR 10.9 crore of PAT this quarter, March quarter.

Adil Agarwal: Correct. No, we're talking about the whole year, FY '25.

Binay Singh: Okay. So INR 10.9 crore for the whole year for Thind.

Adil Agarwal: Yes.

Binay Singh: Okay. And lastly, just on slide 19, where we've given revenue per mature facility, if you just look at the annual trend of that number, it was growing around 15% YoY. Last year, the revenue per mature facility has grown at around 7% YoY. So earlier, it was 22, 15, 7, so if any thoughts on how to look at this number, because we've seen sort of 15 coming

down to 7.

Adil Agarwal: Are you talking about the mature facilities growth?

Binay Singh: Yes, the revenue per mature facility, because slide 19, the number we've given, which was about INR10.9 crores.

Adil Agarwal: About INR11.7 crores, that's what you're referring to, right?

Binay Singh: Yes, yes. So this number last year per facility grew by around 7%, you know, from 10.9 to 11.7. Earlier, it was growing almost 15%, 16%. So is there any change we've seen or any thoughts on that number coming down from it?

Adil Agarwal: Yes, I got it. I'll just request Yashwanth to answer this.

Yashwanth Venkat: Two points, Binay. One is on Thind, whatever INR 10.9 crore number, which you mentioned, it is a 49% number. The overall PAT generated from Thind is close to about INR 21.5 crores to INR22 crores, one. Second on the point on average revenue per mature facility, we faced some headwinds in a few of our facilities in Africa.

So, if we have to break down the average revenue per mature facility, at India level, it is close to about 15%. But in Africa, it was flattish. And in fact, a few of the facilities had slight decline in growth as well due to currency impact. So that is why the average revenue per mature facility is looking at close to about 8%.

Binay Singh: Oh, that's very good. So that's a very good number then at the India level. Sorry, just on minority interest, could you just give me a breakdown like the INR100 crores minority interest, sorry, INR 10 crore minority interest that you reported for the quarter? What is the breakdown between Thind, Aditya Jyot and any other just on that number? Just last one.

Thanks.

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Yashwanth Venkat: Yes, I'll come back to you on that. Just one more point on the mature facility, average revenue per mature facility part, Binay. Point to note here is a few of the facilities which we started in FY '21-'22 were primary care facilities. And those facilities also, as you know, the primary care facilities ramp up cannot be compared to the surgical facilities. So those facilities have also been added to the mature facility bucket. So that is also one more reason for the slight dip in the average revenue per mature facility.

Probably going forward, we'll publish it as average revenue per mature surgical facility, average revenue per mature facility in India. That would be the right way to look at things, Binay. Your question on the breakup, on Q4, the PAT from Thind was to the tune of INR 5.28 crores, which is the minority number. On Aditya Jyot I'll just -- I'll come back to you, Binay.

Binay Singh: Great, great. Thanks, team. Thanks.

Moderator: Thank you. Next question is from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead.

Alankar Garude: Hi, everyone. Hi, sir. So firstly, can you broadly divide the 55 centre additions for FY '26 across primary, secondary and tertiary?

Adil Agarwal: Sure.

Yashwanth Venkat: Hi, Alankar ji. Good afternoon. Out of 55 facilities, about 30 would be surgical facilities. In this 30 surgical facilities, 27 would be secondary facilities. We are looking at opening three tertiary care facilities. The rest 25 would be primary facilities, Alankar.

Alankar Garude: Understood, sir. Thank you. So the other one was, if I look at CFO to EBITDA, you're given that in the PPT as well. If you look at the number for FY '24, it's 85% and it's 72% in FY '25.

You spoke about the IPO reimbursements. So even if you adjust for that, that number is down to 77% in FY '25 versus 85% in FY '24.

So first is, what has driven the higher working capital in FY '25? And maybe secondly, are there any other factors which you would like to highlight which have impacted the operating cash flow?

Yashwanth Venkat: Sure, Alankar. This year, with us opening more primary care facilities and also opening

facilities in Tier

2 and Tier 3 markets, the inventory levels also have gone up slightly. So, if you discount for that, this 77% would have been actually at close to about 80%. So 3% impact we had because of increase in inventory levels, one.

Second is, in terms of receivables, again receivables also had an impact of close to about 2%. And then in terms of payables, especially the last two quarters, we had continuously increased in paying out our key vendors. So there was an impact of close to about 1%. So if you adjust for all these three points, the number would have stood at close to about 83%, Alankar. In terms of any other challenge as far as cash flow from operations, we do not foresee any other kind of challenge as far as the cash flow from operations go, Alankar.

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Alankar Garude: Understood. That 's helpful. So basically, the number to look at going forward is more in that 80%-85% range. Anyway, the 77% was a bit of, I mean, really impacted by some of the factors you mentioned.

Yashwanth Venkat : Yes, 80% would be quite a comfortable number because, again, there will be significant growth as far as the newer facilities are also getting opened in some of the smaller markets. So definitely 80% will be the right number, Alankar.

Alankar Garude: Perfect. And a couple of bookkeeping ones. One is, what was the rental payout, cash payout in FY '25? And secondly, what was the share of minority in the console EBITDA? You spoke about PA T. But if you could help on EBITDA as well, it would be helpful.

Adil Agarwal: Can you just repeat your first question, Alankar? What was the first question?

Alankar Garude: So the first one was on the rental payouts in F Y '25.

Adil Agarwal: Total rent, total, total what we paid out for rent, is it?

Yashwanth Venkat: It is to the tune of INR 110 crores , Alankar, as far as the rental payouts go. On minority adjustment on EBITDA, just give us a minute. We will give you the numbers.

Adil Agarwal: Yes, Alankar, we'll come back to you on that data for the minority interest upon EBITDA.

Alankar Garude: Sure, sure, sir. Great. That's it from my side. Thank you.

Adil Agarwal: Thank you, Alankar.

Moderator: Thank you. Next question is from the line of Gautam Rajesh from Everflow Partners. Please go ahead.

Adil Agarwal: Hi, Gautam.

Gautam Rajesh: Hi, sir. Good evening. Thank you for the opportunity. My first question was, what sort of acquisition multiples have we paid historically for acquisitions that we have done in terms of EV EBITDA? And do these continue to be operated in the name of the doctor or the name of Dr. Agarwal? And does the original doctor stay on board or transition out?

Adil Agarwal: So our acquisition multiples from an EV EBITDA perspective were anywhere between 8 to 10x. Other than a couple of outliers, where it's either gone above that or less than that, the typical range has been between 8 to 10 times EBITDA.

Now, typically, what we do in many of these acquisitions is, firstly, what is important is the partner continues to stay on in the business, because the business revolves around the partner.

So the doctor partner whom we acquire continues to stay on in the business. In terms of branding, we do something which we call a joint branding. So what we do is, if the doctor has a particular name, like, suppose we have a centre called Soham Eyecare. It is called Soham Eyecare, jointly with Dr. Agarwal Eye Hospital. That's how we do the branding.

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Or in another scenario, what we do is, we call the hospital Dr. Agarwal Eye Hospital, and we call out the doctor partner's names. So there are two typical models that we bring out. But over a period of time, what we aspire to do is, we try to convert all the branding into the Dr. Agarwal Eye Hospital brand. And that's eventually something

which we tell the partners, over a period of 4 to 5 years, we will convert all your hospitals into Dr. Agarwal Eye Hospital brands.

Gautam Rajesh: Understood. My next question was, what portion of the revenue comes from the laser operation, let's say, optical , like corrective surgeries?

Adil Agarwal: You're talking about what is the revenue mix between services and products?

Gautam Rajesh: Yes, yes. Yes, for laser operations, like, optical correction.

Adil Agarwal: So overall, from a distinct, surgeries contribute about 65.3% of revenues. The sales of products and services, which includes optical products and pharmacy products contribute about 20.8%.

And about another 14% comes from diagnosis and consultations.

Yashwanth Venkat: He had a question on refractive. Lasik , I think, was your question, Gautam, if I'm not wrong, on the refractive contribution is close to about 5.7%.

Gautam Rajesh: 5.7% for the year, right?

Yashwanth Venkat : Yes, yes.

Gautam Rajesh: Yes, and how are some of the older centers doing in terms of, as you mentioned, Africa is fairly flat to negative. But what about the older centers, like older mature centers in terms of...

Adil Agarwal: I'll request Mr. Rahul to speak on the older mature centers .

Rahul Agarwal: So historically , if you see our older centers have been growing at around 15 %-16%. Even for this year, we've grown by almost 14% from a SSSG perspective, which is a like -to-like centers. We've grown by around 14%. It's been slightly lower, but a broad range of 15% is what we try and look at.

Gautam Rajesh: Understood. And so I had missed the earlier question on what was your rental payment for FY '25?

Yashwanth Venkat : Close to about INR 110 crores. This doesn't include any short -term rents, Gautam.

Gautam Rajesh: Okay, okay.

Yashwanth Venkat : So, all of that is not included. This INR 110 crores is the pure pay rent which we are paying across facilities.

Gautam Rajesh: Okay, this was how much last year, sir?

Yashwanth Venkat : One second. Last year, this number was around INR 92.5 crores.

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Gautam Rajesh: INR92.5 crores. Thank you, sir. All the best.

Moderator: Thank you. Next question is from the line of Dishant Jain from Quasar Capital. Please go ahead.

Dishant Jain : Yes, thank you for the opportunity, sir. And congratulations on the good numbers. A couple of questions. First, could you tell us what will be the revenue potential of that Cathedral Campus facility that we're going to have?

Adil Agarwal: Which? You're talking about the subsidiary?

Dishant Jain : Yes, in the subsidiary, correct.

Adil Agarwal: How much is the?

Dishant Jain : Revenue potential.

Adil Agarwal: Revenue potential. Okay, so this year, we have done revenue of close to INR 120 crores. We expect that center to grow at about 35% year -on-year once the fully constructed building is up and ready.

Dishant Jain : So that building is already on the board?

Adil Agarwal: We're talking about that particular facility when it's up and running. We should expect that center to do approximately 30% year -on-year once the center is up and running.

Dishant Jain : 30% up and running. Okay . Okay. Sorry, sir, I'm not able to understand. Can you repeat again?

Adil Agarwal: So approximately 30%, you'll see an uptick in revenue once the new facility is up and running.

Dishant Jain : Oh, okay. And, sir, there is a INR 18.5 crores cost that we have paid for some business in the subsidiary company for FY '25.

Adil Agarwal: Can you repeat that?

Dishant Jain : Hello, am I audible?

Adil Agarwal: Yes, yes.

Dishant Jain : Yes, so I was asking that we have paid around INR 18.5 crores for some business acquisition in the subsidiary company

Adil Agarwal: Ah, okay. Okay, got it.

Yashwanth Venkat: Wait, let me explain. In the subsidiary, actually, we acquired a premium pr

actice in Chennai by

name, the Eydox Eye Hospital. We had paid INR 18 crores to the tune for completing that acquisition. That practice is actually led by a very star retina doctor who is currently heading our retina practice at the main facility as well.

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Dishant Jain : Okay, okay. And, sir, on behalf of our last question, I just missed the volume numbers for Q4.

So can you please repeat it again?

Adil Agarwal: Oh, what is the question?

Dishant Jain : Volume numbers for Q4?

Yashwanth Venkat : Yes. In Q4, we performed 6724 surgeries, in which cataract was the main contributor,

accounting for about approximately 71%, followed by refractive surgeries at around 6.5%. In Q4, the volume of cataract and refractive surgeries grew by about close to 13%, while the other surgeries experienced a higher growth rate of close to 36%.

Dishant Jain : Okay, thank you. Thank you for answering all the questions. Wish you all the best.

Moderator: Thank you. Next question is from the line of Harshil Seth from Autotec International Private Limited. Please go ahead.

Harshal Seth : Yes, hi. So just wanted to understand, like, what are your plans for the merger of the subsidiary Dr. Agarwal's Eye Hospital? And secondly, also, I would want to understand, like, where, since you don't conduct consults for a subsidiary, so I just wanted to understand, like, where exactly it is more focused? Like, Dr. Agarwal's Health Care, you know, it's across all over India, but then what part of the country is Dr. Agarwal's Eye Hospital?

Adil Agarwal: Sure. So I'll take the second part of the question first. Dr. Agarwal's Eye Hospital Limited, which is the oldest subsidiary, which was listed in 1994, that predominantly continues to operate all the facilities based out of Tamil Nadu. Although the other company, Dr. Agarwal's Health Care Limited has a few facilities based out of Tamil Nadu, but all the new expansion in the state of Tamil Nadu over the last one, 1, 1.5 years has been happening only in the listed subsidiary. And any new further expansion, along with the large capex, which we are investing in our flagship facility, all of that will happen only in the listed subsidiary.

Now, coming to the merger of both the entities, we had already disclosed in the DRHP that we plan to do the merger of the listed sub with the holdco in the outer span of 3 years. That said, we are speaking to the bankers right now to decide on which bank we're going to go with, and we will soon have some news for you around the merger. Our plan is to get that done as soon as possible.

Harshal Seth : Okay. So just want to understand, so what you meant was that the hospitals that are run in the state of Tamil Nadu, that all are run under Dr. Agarwal's Eye Health, Eye Hospital.

Adil Agarwal: Dr. Agarwal's Eye Hospital.

Harshal Seth : Yes, talking about Eye Hospital.

Adil Agarwal: Outside of the state of Tamil Nadu, which is Karnataka, Andhra Pradesh, Telangana, Maharashtra, Punjab, all that happens in the holdco which is Dr. Agarwal's Health Care Limited, including Africa.

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Harshal Seth : Which happens, sorry, actually, your voice is not clear. Like, could you just repeat it once?

Adil Agarwal: So, all the expansion outside of the state of Tamil Nadu, any new centers in the holding company, which is Dr. Agarwal's Health Care Limited.

Harshal Seth : Okay, okay. Got it, got it.

Adil Agarwal: There's a geographical distinction between where we open any new centers.

Harshal Seth : Okay, okay. Got it, got it. So any new centers that are being opened outside Tamil Nadu, that comes under the holding company, and the other, which are the new facilities that you might open in Tamil Nadu, that comes under Dr. Agarwal's Eye Hospital. Is it correct?

Adil Agarwal: Yes, that's

exactly. Yes.

Harshal Seth : Okay, okay, thank you. That's it from my side.

Adil Agarwal: Thank you.

Moderator: Thank you. As there are no further questions, on behalf of Dr. Agarwal's Health Care Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Adil Agarwal: Thank you very much.

Call: Aug 2025

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August 16, 2025

BSE Limited
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Subject: Analyst/ Earnings Call – Transcript

Pursuant to Regulation 30 and 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the analyst/ earnings conference call held on August 12, 2025.

The said transcript is also available on the website of the Company at : <https://dragarwals.co.in/dr-agarwals-health-care/#analyst-earnings-call>.

Yours Truly,
For Dr. Agarwal's Health Care Limited

Thanikainathan Arumugam
Company Secretary and Compliance Officer

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Q1 FY26 Earnings Conference Call"
August 12, 202 5

MANAGEMENT : DR. ADIL AGARWAL – CHIEF EXECUTIVE OFFICER –
DR. AGARWAL 'S HEALTH CARE LIMITED
DR. ASHAR AGARWAL – CHIEF BUSINESS OFFICER –
DR. AGARWAL 'S HEALTH CARE LIMITED
MR. RAHUL AGARWAL – CHIEF OPERATING OFFICER –
DR. AGARWAL 'S HEALTH CARE LIMITED
DR. VANDANA JAIN – CHIEF STRATEGY OFFICER –
DR. AGARWAL 'S HEALTH CARE LIMITED
MR. YASHWANTH VENKAT – CHIEF FINANCIAL
OFFICER – DR. AGARWAL 'S HEALTH CARE LIMITED
MS. AASHNA DHARIA – HEAD OF INVESTOR RELATIONS

Moderator: Ladies and gentlemen, good day, and welcome to the Dr. Agarwal's Health Care Q1 FY '26 Earnings Conference Call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Aashna Dharía, Head of Investor Relations from Dr. Agarwal's Health Care Limited. Thank you, and over to you, ma'am.

Aashna Dharía: Thank you, Manav. A very good evening, ladies and gentlemen. Welcome to Dr. Agarwal Health Care Q1 FY '26 Earnings Call. From the management side, we have Dr. Adil Agarwal, CEO; Dr. Ashar Agarwal, Chief Business Officer; Mr. Rahul Agarwal, Chief Operating Officer; Dr. Vandana Jain, Chief Strategy Officer; and Mr. Yashwanth Venkat, Chief Financial Officer. We have released the financial results, press release and investor presentation, all of which are available on our website and the exchanges as well. Before we continue, we want to remind everyone that this call is being recorded and the transcript will be made available on our website afterwards.

Additionally, please be aware today's discussion may include certain forward -looking statements, which should be considered in the light of the risks our business faces. Please refer to the detailed statement on Page 2 of the investor presentation.

It is now my pleasure to hand over the call to Dr. Adil, our Chief Executive Officer, who will share his opening remarks and insights. Dr. Adil, over to you.

Adil Agarwal: Thank you, Aashna. Good evening to all of you and a warm welcome to the Q1 FY '26 Earnings Call of Agarwal's Health Care Limited. Let me begin by providing you all an update on the Quarter 1 performance. The new fiscal year commenced with record -breaking results. We crossed the milestone of INR500 crores in total income for the quarter and delivered over a 100% year-on-year increase in profit after tax of INR38 crores.

For Q1 FY '26, total income stood at INR501 crores, up 22.3% year -on-year while revenue from operations rose 20.8% to INR 487 crores. The quarter delivered a robust Ind AS EBITDA of INR 141 crores, growing 28.9% year -on-year with margins at 28.2%, an improvement of 140 basis points at more than doubled to INR38 crores with a PAT margin expanding by 315 basis points to 7.6%.

Next, I would like share our footprint and network growth update. During the quarter ending June 2025, we served over 7 lakh patients and performed nearly 79,000 surgeries through our network of over 249 facilities, comprising 29 hubs and 220 spokes.

In India now, we have a total network of 230 facilities across 14 states and 5 union territories covering 136 cities. Our presence is well diversified now with 32% of our facilities in Tier 1

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cities, 61% in other cities and 8% located internationally. We expanded our footprint by commissioning 13 new greenfield facilities further strengthening our reach and capacity. These included 1 tertiary center in Delhi, 7 secondary centers in Mangalore, Hennur Road in Bangalore, Visakhapatnam, Sambhalpur in Orissa, Redhills in Chennai, Lucknow, and Kolhapur along with 5 primary centers in Andhra Pradesh, in Erode in Tamil Nadu, in Kerala, in Telangana and one in Tanzania.

Of these, two facilities, one is Redhills in Chennai; and second, Bhavani and Erode were opened under our subsidiary, Dr. Agarwal's Eye Hospital Limited and one primary center in Tanzania under our African subsidiary. During the quarter, we marked a significant milestone with our strategic entry into the Delhi market

by opening a state -of-the-art 3-story tertiary facility spread across 10,000 square feet.

We are privileged to have onboarded Padma Shri awardee, Dr. Jeewan Singh Titiyal, an eminent surgeon who was previously the former Chief and HOD of the R.P. Center of Ophthalmic Sciences at AIIMS, New Delhi. He is the first Indian ophthalmologist to have performed a live surgery at ASCRS, USA. Our Delhi facility now is equipped with cutting-edge technologies, including the Excimer Laser, SMILE for refractive surgery, the Alcon Phaco machine for cataract, the ZEISS ARTEVO 800 Operating microscope, amongst others.

In the last month, the center recorded over 1,000 patient visits, underscoring the strong demand for the brand in this particular market. This launch marks a strategic step in our organic expansion plans in North India with plans now underway to establish our presence in Gurgaon and other parts of the Delhi NCR region.

In Uttar Pradesh, following our acquisition in Varanasi last year, we are now focused on strengthening our brand equity to drive future organic growth and have recently expanded our presence with a new state-of-the-art facility in Lucknow as well.

Now moving on to a few highlights on the clinical front. There has been a significant focus around the firm to ensure we strive for clinical excellence while we continue to scale. This is reflected in an increase in the following specialized surgical procedures. In Q1 FY '26, the contribution from high-end cataract surgeries increased to 25.5% out of the total cataract procedures, up 230 basis points year-on-year from 23.3% in the same quarter last year. Overall, our robotic cataract surgeries, what we call as Femto cataract surgeries grew by a robust 72% year-on-year from 674 surgeries to 1,160 surgeries. As shared in our previous earnings call, we have installed new robotic cataract surgery machines at our Bangalore and Hyderabad facilities, which are now cumulatively performing an average of about 110 surgeries a month. Our Lenticular Procedures, what we call as a SMILE also increased from 1,173 to 1,257, reflecting a 7.1% year-on-year growth. Retinal surgeries totalled 2,969 in Q1 FY '26, up 23.5% from last year. Corneal transplants also increased to 242 procedures in the first quarter of FY '26. Further, we continue to invest in specialized equipment and techniques to strengthen both diagnostic capabilities and surgical outcomes.

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In our existing centers, we have upgraded our facility with advanced equipment. For instance, we have installed a new Femto Cataract robotic machine in Velachery, Chennai. We have launched a new Phaco Cataract machine in Tirupur and in Hyderabad and a new retinal machine in our upcoming centers in Aurangabad.

Shifting to some notable developments on publications, training and clinical research. Over the last 3 decades, our doctors have contributed to more than 300 publications in leading international medical journals, underscoring our commitment to advancing ophthalmic science. During the quarter, we also focused a lot on capability building and nearly 90 doctors underwent training across a range of areas. On the clinical research front, we currently have 20 active studies underway. This quarter, we successfully conducted a Phase III clinical trial in patients with wet age-related macular degeneration, a common eye condition that can cause blurred or distorted central vision.

Now moving on to some business updates. Let me first begin with our region-wise performance. Our South region continues to be our largest market, contributing to 65% of the total group revenues. This region delivered INR 350 crores of revenue, representing a strong year-on-year growth of 23.9%.

We have now 159 facilities across the Southern states and this includes 8 new additions. In the Southern mar

kets of Tamil Nadu and Telangana, we are focused on sustaining our market leadership while further strengthening that particular region through adoption of the latest technology.

In Karnataka and Andhra Pradesh, we are strengthening our presence in both these markets by expanding significantly into high-potential underserved markets through strategic facility openings. If you look at the West region, that contributes to about 15% of the overall group revenues and that region delivered INR 73 crores of revenue, up 19% year-on-year. This region operates 41 facilities and we added one new facility in this quarter.

Asian volumes grew by 18% to over 1.05 lakh, while surgeries performed were up 13.7% to 11,691 procedures. In Mumbai, we are leveraging the strong brand equity from our recent acquisitions to drive organic growth through focus on adding new additional greenfield facilities.

In the rest of Maharashtra outside of Mumbai, we are targeting high potential Tier 2 micro markets by capitalizing on our brand strength and operational expertise and have recently launched a facility in Kolhapur and have one coming up in Aurangabad very soon.

In Gujarat, again, our focus is on expanding into new urban markets, and we set up a new facility in Udhna in Surat. North accounts for 7.8% of revenues and this region reported INR 38 crores of revenue this quarter, up 13.4% year-on-year. In Delhi NCR, we are focused on organic brand building while pursuing gradual regional expansion with plans to open up a facility in Gurgaon very soon.

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In Punjab, we are consolidating our leadership position following the Thind Eye Care acquisition. Each contributes to about 2.6% of total revenues with INR 13 crores of revenue done in Q1, up 14.9% year-on-year. Overall, all regions have posted very positive growth with the South leading in both absolute size and year-on-year momentum, while North, West and East continue to build scale.

Moving on to our same-store sales growth. Our mature centers are those that have been open for more than 3 years. As of June 2025, we operate 116 mature facilities. Revenue from these mature facilities has increased 19.4%, reaching INR 354 crores in the quarter 1 FY '26, contributing to 73% of the total group revenues. Revenue from mature facilities in India has grown by 21.1% to INR 309 crores.

Moving on to our vintage performance in Q1 FY '26. Facilities operational acquired prior to FY '22 contributed INR 340 crores in revenue, registering a growth of 15%. Facilities opened in FY '23 generated INR 60 crores, growing at 13.7%, while those launched in FY '24 delivered INR 40 crores in revenue, achieving a strong growth rate of 23.3%.

Our third business update revolves around a case study which we have done in Telangana. This is a topic that always comes up in conversations where we talk about our organic growth strategy. We would like to highlight a case study on our expansion journey in Telangana. Our Telangan journey is a testament to steady and strategically planned growth. We began in FY 2011 with 3 greenfield centers generating about INR 10 crores in annual revenue.

In Phase 2, we tripled our network expansion from network from 3 to 8 facilities and grew revenue sevenfold to INR 70 crores, driven by a strong brand presence and enhanced clinical capabilities. We are now in Phase III and have rapidly expanded to 18 centers, extending into the suburbs of Hyderabad and Tier 2 cities like Warangal and Nizamabad.

We have more than doubled our network to 18 facilities and have almost doubled revenues from INR 70 crores to nearly INR 130 crores, reflecting an accelerated scale-up and deeper market penetration.

Moving on to our fourth business update, our planned facility openings domestically for each quarter of the current year. Over the next 3 quarters, of this financial

year, we are targeting to

launch another 42 new facilities, which will comprise 23 centers in the South, 9 in the West, 5 in the North and 5 in the East. That's the business update from my side.

Now I would like to hand over the call to our CFO, Mr. Yashwanth Venkat, who will take a deeper dive into our financial performance.

Yashwanth Venkat: Thank you, Dr. Adil. I'll begin with the operational update. Surgical services continued to be the main revenue driver, contributing 65.6% to the group revenue. Diagnosis, consultations, and other nonsurgical treatments contributed 13%, and the sale of optical products and pharmacy items accounted for 21.4%. In Q1 FY '26, we performed 78,882 surgeries, marking a 16% year-on-year growth. Cataract surgeries remained the largest contributor, accounting for

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approximately 73.4% of total surgeries, followed by refractive surgeries at around 4.7%.

Volumes for cataract and refractive surgeries grew 14.2% year-on-year, while other surgeries on a blended basis recorded a growth of 22.8%.

Our payer mix for the quarter stood at 61.7% from cash, 29.1% from insurance and TPS and 9% from government schemes. Domestic payer mix for the quarter stood at 72.5% from cash, 22% from insurance and TPS and 5.5% from government schemes.

Moving on to the financial section. I'll start with the revenue split. The group's revenue from operations grew by 20.8% year-on-year, reaching INR 487.26 crores in Q1 FY '26 compared to INR 403.49 crores in Q1 FY '25. Revenue from operations in India stood at INR 440.4 crores, reflecting 21.4% growth year-on-year despite disruptions from the Pahalgam attack and operation Sindoor, which briefly impacted our operations in the Northern region, particularly in Punjab and Jammu and Kashmir. This growth was supported by a mix of volume growth of 9%, value growth of 5.6% and balance from the contribution of new centers opened in FY '25 and FY '26.

Revenue from our Africa operations grew by 15.7% year-on-year in Q1 FY '26, while their contribution to overall revenue declined from 10.1% in FY '25 to 9.65%. Gross profit margins remained stable at 77.5% in Q1 FY '26 despite a rising share of high-end surgeries in our case mix.

Doctor and employee costs cumulatively have remained broadly in line with the previous year.

We reported an EBITDA margin improvement of nearly 1.4% attributable mainly to cost efficiencies. A total of INR 195 crores in loans have been repaid from IPO proceeds, INR 128

crores in Q4 FY '25 and INR 67 crores in Q1 FY '26, leading to lower finance cost and higher profitability versus the same quarter last year.

From 66% in Q1 FY '25, the share of profit after tax attributable to owners has expanded to 79% in Q1 FY '26, signalling improved profit distribution. A HCL stand alone has delivered a positive PAT of INR 10 crores in Q1 FY '26 as compared to a loss of INR 8.5 crores in Q1 FY '25. Thank you all. We'll open the floor to questions.

Mod erator: Thank you very much. First question from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead.

Alankar Garude: Sir in the past, you have spoken about refractive being a strong growth driver for the company. However, if I look at growth in refractive surgeries in this quarter, it seems quite low at 3% year -on-year. Can you help us understand the reasons for this relatively slower growth in refractive compared to the other surgeries?

Adil Agarwal: Yes, Alankar. Nice to hear f rom you. Yashwanth?

Yashwanth Venkat: In terms of refractive surgical contribution over the last 3 years, it has grown up by about 0.4% to 0.5% every year. But there is a certain amount of seasonality as far as refractive surgeries are

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concerned. The stron gest months as far as refractive

surgeries are concerned are one, July and second will be December. So it may not be fair to look at refractive surgeries growth on a Y -o-Y for Q1, Alankar.

Alankar Garude: But then Yashwanth, when you look at Y -o-Y, the sea sonality which you mentioned gets captured, right? So still not able to understand why this 3% year -on-year growth in refractive surgeries?

Rahul Agarwal: Alankar, this is Rahul. So while you're right, the seasonality gets captured, but these are also the months where we don't push from our side with any offers or campaigns. So we don't expect jumps coming in these quarters. The quarters where we also go aggressive when there are more patients available in the entire pool when they're actually looking for s urgeries are the quarter 2 and quarter 3. Those are the 2 quarters where we get the maximum impact. So from our side also, we really do not push in these other quarters.

Alankar Garude: Fair enough. So would it be fair to understand then, Rahul, that broad ly, if you look at the full year, growth in refractive surgery should broadly mirror the overall surgery growth or maybe even grow higher?

Rahul Agarwal: I would say, overall, yes, it should mirror whatever the overall surgical growth is , in line with the overall gro wth is what refractive also should .

Alankar Garude: Understood. And maybe one follow -up question here. Apart from insurance coverage, are there any other key reasons to explain the relatively lower refractive contribution for eye care players in India compared to some of the global peers?

Adil Agarwal: Yes. One second, Dr. Vandana is going to take that.

Vandana Jain: So I think, first of all, you are right. Compared to the other global players in India, the uptake of refractive surgery has been low er because most of the time the payments that happen are out of pocket. Secondly, the incidence of myopia itself, actually, now it has begun to increase. But in certain developed markets like the U.S., Europe or even China, for that matter, China, the incidence of myopia actually started increasing much, much earlier, which now we are seeing the trend in Indian market also.

So going forward, we do believe that in India also, we are going to see a similar uptake of refractive surgery going forward. However, that trend we are not noticing immediately right now. And lastly, as Rahul mentioned, that because overall, this technology, which has now come into India, there is now a greater adoption by more and more players. But still, when you compare it to the West or to China, the number of machines or even if you look at the density of machines in India is much lower compared to the West or to China.

Alankar Garude: Got it. That's really helpful. The second question is, unlike Maharashtra, w herein acquisitions have played a big role for the company, growth in Telangana has been more organic, as you pointed out in your opening remarks. How would you contrast your journey across Telangana

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and Maharashtra? The reason for asking this question is basically to help us under stand the diffe rence in scale of timelines across organic and acquired facilities. Maybe examples of these two states could be useful.

Adil Agarwal: So Telangana, we went through a phase of struggle for the first 4 to 5 years. It takes that much

time to build your brand, Alankar. We were also growing as an organization at that point. Now you have a fairly established presence in Telangana. You've been operational in that market for over 13 to 14 years. And it takes a while for the brand to get established.

And once the brand gets established, now when you put out organic centers, greenfield centers in your own brand, automatically, you get walk-in, which is what is actually helping you provide those kind of results, which is what has helped you actually double your revenue from

INR 70

crores to almost INR 130 crores in Telangana.

Maharashtra, again, we are following that thought process, but Maharashtra is a slightly more complicated market because there are a lot more established players in Maharashtra. We also entered Maharashtra only in 2019. So there's an 8 year gap between when we enter Telangana and Maharashtra. Now we are starting to see traction in the Maharashtra region as well, which is why we have opened 5 new greenfield centers in Mumbai alone.

So we have set up a center in Badlapur, in Kalyan, in Virar, in Dombivli and one more in Vashi.

And we are seeing good traction in these markets because over the last 5 to 6 years, as we have gone about building our brand, the brand is slowly getting established in some of these markets.

And now you are starting to see some of that traction actually help you scale up some of these centers.

So we believe if we continue on this organic path, which we have done in Telangana, similarly in Mumbai and Maharashtra, you will start to see the similar kind of results in this particular region.

Alankar Garude: Understood. And one final question. What was the same-store sales growth for the secondary and tertiary centers in this quarter?

Adil Agarwal: We'll just give you the number right now.

Rahul Agarwal: So I'll give you the overall same-store sales growth. It was to the tune of around 14.5%.

Alankar Garude: Yes. But then that doesn't become comparable. If you can provide it for secondary and tertiary separately, that would really help.

Adil Agarwal: We don't have that data right now, Alankar but what we have is our mature centers which fundamentally will -- it will represent your secondary and tertiary centers because the primary center contribution is much lesser anyways. So this 14% - 15% is pretty much representative of your secondary and tertiary facilities. There would be somewhat difference between that and even if you take out the impact of the primary centers.

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Alankar Garude: Got it. And between secondary and tertiary...

Moderator: Sorry to interrupt you, Alankar. Can you please request you to rejoin the queue?

Alankar Garude: Yes, yes. Sure.

Moderator: We have our next question from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar Manudhane: Sir, just on surgery side, like total surgeries, I see 16% growth in terms of number of surgeries, while the revenue has grown by 21%. And within surgeries also as the earlier question, the refractive was a bit soft for this quarter. So what explains this revenue growth being higher than the number of surgeries growth?

Rahul Agarwal: Sure. So Tushar, overall, the growth, we can look at it as a volume growth and price growth. And part of it would be from the new greenfields that comes in, right? So I would just break it up for you. Overall, from a volume perspective, we had a 9% growth, and I'm largely focusing on the domestic market on this. We had a 9% volume growth, which was led largely by the OPD growth that we got.

Second was on the value growth, which is a combination of our price hike as well as the premiumization that we drove. So that would be to the tune of almost 5.5% to 6%. And finally, another 8%, which came in from greenfield. So that is the entire combination of around 23% overall growth that we drove for domestic business.

Tushar Manudhane: Okay. I was referring specifically for surgery revenue, not the overall revenue. I'm considering the surgeries volume.

Rahul Agarwal: So okay, from the surgical perspective itself, the 16% growth that we are talking about, that contributes to the 16% part of the growth. The remaining comes from the value growth that we drive in those 16% overall growth. So basically, how we are driving the premiumization, if I'm answering your question right, is that you have

high-end lenses, which contribute to higher value

per surgery that comes in. For example, last year, we had our yield per patient on cataract was around 38,000. That has now moved to around 40,000. So that is the value increase that we are getting.

Similarly, what Adil spoke about initially was the Femto Cataract. So including Femto cataracts we did around 1,160 -odd surgeries this year compared to 674 last year. That's an added additional value in our high -end surgeries over and above the regular lens cost that we get. So that contributed around 26.2% in the overall high -end numbers that we got for the quarter. Did I answer your question?

Tushar Manudhane: Yes, yes, very much. So yield per patient for cataract, where do we see or envisage these numbers, say, by end of this year, probably next year, given that we are moving towards high -end lenses or sort of convincing patients to go to the high -end lenses.

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Rahul Agarwal: So I'll just give you the trajectory that we have had so far. We did 32,000 in FY '22. And now in FY '26, Q1, we reached 40,000. So that's been the trajectory around 2,000 kind of a growth every year is what we have been able to achieve. We don't see a reason why it should change as we move forward.

Tushar Manudhane: Got it. And just second one from my side is about the consultation revenue, while reduced dollar proportion, but that has grown at, say, 14%, while we had a very healthy growth on the surgery side, but consultation probably gives more sort of a forward -looking indication in terms of conversion to surgeries. So that piece is a bit soft despite addition of new centers as well as sort of healthy growth at the organic level as well. So if you could just elaborate on that aspect as well.

Rahul Agarwal: Perfect. So largely, I would not say that you should look at consultation revenue as a proxy to the volume growth and how your surgical numbers would pan out. I think new OPD growth would be a better benchmark to look at how the new OPD and overall OPD is growing for us.

That would be a better way to look at how many new surgical advisers will happen and how many surgeries will happen. So I would say consultation may not be because we have various agreements across India with different companies and those kind of things where we might even go down or up on different consultations that we do.

Tushar Manudhane: Got it. Sir, if you could just give some indication about the new OPD growth quarter for last year, how much it was?

Rahul Agarwal: So just on the patients served, I can tell you that we had a growth of 18.6% from 594,000, we moved up to 705, 000.

Moderator: We have our next question from the line of Avnish Burman from Vaikarya.

Avnish Burman: I just have a couple on the Sub -co. The 17% revenue growth in the Sub -co to INR 117 crores, can you also divide this into the volume and the pricing and the new center addition, please?

Adil Agarwal: Sure. So our overall patient volumes, which is our total opening volumes increased by about 16% at the Sub -co level. We saw a total volume growth of about 16% increase from about 1.4 lakhs to about 1.6 lakhs.

Avnish Burman: Yes. So does that mean that the pricing growth and the new center growth was flat this quarter?

Adil Agarwal: No, that's not what it means. Both volume and the pricing have pretty much increased at a steady state, which is what is driving this growth at the subsidiary level as well.

Yashwanth Venkat: I think your question was on the number of facilities. In the Subco, we have opened 2 facilities this quarter. However, just to throw some light, we had 50 facilities at the end of FY '24.

Currently, in the Sub -co, we have 63 facilities, marking a 1.3x growth over the last 15 months in terms of the number of facilities as well in the Sub -co.

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Avnish Burman: Yes. So that I saw in the presentation. My question was that the revenue growth has been 17%. And you're saying if the volume growth is 16%. So I mean, usually, it's a combination of volume price and new center additions. So almost all the growth has come from volume, does that mean that pricing growth in this quarter was just 1%? I mean that was the question.

Adil Agarwal: That's the question. Okay. So we'll just break it up and tell you. So approximately your existing centers, which from last quarter have grown at about 14.8%. Your centers which have opened in FY '26 have contributed to about 2% and about 0.1% has come from the two centers which Yashwanth mentioned, which is contributing about 0.1%. This is what is driving your 17% growth from INR 100 crores in Q1 FY '25 to INR 117 crores in Q1 FY '26.

Avnish Burman: Okay. The second question is , if you look at the Sub -co and if you see the mature facility revenue growth, like you disclosed for the hold -co, how is the mature facility revenue growth here? And that if also you can divide broadly into given price?

Adil Agarwal: Yashwanth?

Yashwanth Venkat: See, in terms of same -store sales growth in Sub -co, it is at about 14.8%. The rest is contributed from FY '25 and FY '26 facilities. In terms of volume and value growth for the Sub -co, we don't have the numbers right now. Probably we can revert later.

Moderator: We have a follow -up question from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Sir, on this secondary and tertiary same -store sales growth, you broadly mentioned same -store sales growth is around 14%, 15%. Would the broader growth rates be similar across secondary and tertiary?

Adil Agarwal: Yes, absolutely.

Alankar Garude: Okay. Great. And just two bookkeeping questions. What was the rental pay -out in this quarter? And secondly, what was the share of minority in this quarter?

Yashwanth Venkat: In terms of overall minority on PAT contribution, Alankar, close to 79% is PAT attributable to owners and 21% is minority. Whereas if you take Q1 of last year, it was close to about 65% and 35%. That's moved up to 79% and 21%. The rental pay -out for Q1 is close to about INR 30 crores.

Alankar Garude: Got it. Yashwanth if you can help us with the minority share of EBITDA as well. You spoke about PAT, but EBITDA as well would be helpful?

Yashwanth Venkat: Yes. minority share of EBITDA close to 87.5% EBITDA attributable to owners. This is 12.5%.

Moderator: We have our next question from the line of Shian Sabu, an individual investor.

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Shian Sabu: Yes, yes. I was just an individual investor. So I was just going through the new hospital that's coming up in the listed entity. So just understanding why there is such a delay in this because it's been there in the last couple of reports. Is there any particular reason?

Adil Agarwal: So we have had a 6 - to 9-month delay predominantly because the facility was affected by floods 1.5 years ago. So I don't know if you remember in December 2023...

Shian Sabu: In Tamil Nadu, there is some flood there?

Adil Agarwal: Yes. So at that point, we faced a significant amount of delay in the putting a process because at that point, and there are videos all over where the site had effectively caved in because of rampant floods in Chennai. So we faced certain flooding of our basements and everything. So we were put back by about 6 to 9 months, which is why there has been a delay in the project. And -- but right now, it's good to know that things are back on track, and we are hoping to get everything up and running by Q4 FY '26.

Shian Sabu: That's great. That's great. Sir, the next -- the last part of my question, two questions, but like they go hand in hand is generally the mode

I that is there is of a lease model, but this time, it's an own model, thereby entailing a higher investment. So would this mean that this would be more margin accretive or any other reason of that?

Adil Agarwal: What do you mean by more margin accretive?

Shian Sabu: Would it be a higher margin accretive than the existing businesses going forward, this particular hospital, if viewed standalone? And why is it moved to an old model compared to a lease model, which is generally the case?

Adil Agarwal: So this was the original facility which belonged to the company. When we were doing the construction of this main flagship facility, we had actually moved out to a rented facility. So we are currently operating out of a rented facility. And now that we are moving back to this particular facility, the rent will only be paid for the land, but the building is being constructed by the company. So that rental outflow will be a little lesser. So from our perspective, the margins will be a little higher in this particular facility.

Shian Sabu: Fair, fair, fair. That will be good. And the subsidiary, like how many more centers do you think will be opening this year, like in the balance period ?

Adil Agarwal: We don't have anything right now in the pipeline in the subsidiary. But that said, we are looking at 3 to 4 potential openings in Tamil Nadu. Those sites are being looked at. Once we have clarity, I think by Q2, we'll be able to give you a better picture in terms of where those centers will be coming up in Tamil Nadu.

Moderator: We have a next question from the line of Tushar Manudhane from Motilal Oswal Financial Services.

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Tushar Manudhane: This insurance share for the quarter moved up really well even if I compare year -over-year or even if I compare with the full year '25. So if you could throw some light here?

Rahul Agarwal: So overall, we had a 2% jump on our insurance. Largely, this was done in the TPA and the

private insurance side. That's again, yes, you're right. That's an area which works well for us. Our DSO is much lower in these areas than the government space. Our payments come in much faster. So that's one area we are very happy when this part of the business does well. Of course, this is driven by higher penetration of the private insurance, which really helps the overall business.

Tushar Manudhane: Yes. But anything peculiar in this quarter that has happened to sort of -- or because it's like a step-up of almost 2 percent age points or any particular region where the insurance.

Rahul Agarwal: No, I wouldn't say this is . I think this is more market -driven phenomenon where more people are coming in with better coverages and more number of people have insurance. So I wouldn't say this is anything to do with us as an organization, but this is more market driven.

Adil Agarwal: Also Tushar, just an additional note, a slightly higher contribution from insurance and TPA came in from the Africa part of the business as well. But otherwise, domestic, like Rahul had mentioned, is pretty much steady state.

Yashwanth Venkat: Domestic payer mix, is close to about 72.5% from cash, 22% from insurance and TPA and 5.5% from government schemes, Tushar. This has been the trend over the last few years, wherein the contribution from government schemes has hovered between 5% to 7%.

Tushar Manudhane: Got it, sir. And just lastly, so the capex number remains steady, right, at INR 310 crores, hopefully in '26 considering the 54 facilities to be coming up?

Yashwanth Venkat: Yes, yes, Tushar. No revision on that front.

Moderator: We have our next question from the line of Amit Kadam from Canara Robeco AMC.

Amit Kadam: So I just have one question. Yashwanth, can you help me with this quarter cash from operations generated? Is it around INR 100 crores - INR105 crores?

Yashwanth Venk

at: No, you're talking about EBITDA?

Amit Kadam: No, no, cash flow from operations for the quarter?

Yashwanth Venkat: Amit, I'll get back to you on the exact numbers.

Amit Kadam: Okay, sure. That's it.

Moderator: Next question is from the line of Ankush Mahajan from Sanctum Wealth.

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Ankush Mahajan: Yes. Sir, a very good set of numbers with the margin expansion also here. Our brand is well recognized and geographical expansion is quite visible now. So when we compare the ROCE as compared to industry, it's at a lower level. Any number for the ROCE, how we should see this ROCE number over the next 2, 3 years? Any number you want to put to?

Adil Agarwal: So as we continue to improve our margins, if you notice, our PAT margins for this particular quarter have moved up to about close to about 8%, right? We believe you will certainly see a consistent improvement in your PAT margins over the next 2 to 3 years. As the PAT margins improve over the next 2 to 3 years and your margins improve steadily, you'll start to see your ROCE profile also change over the next couple of years. So we'll see steadily improvement in the ROCE mix over the next couple of years.

Ankush Mahajan: So in number...

Adil Agarwal: Sorry, go ahead.

Ankush Mahajan: So over the next 2 to 3 years, any numbers that we are looking for the ROCE?

Yashwanth Venkat: On ROCE in March '25, in terms of numbers, we were close to about 16% on a consol basis. This year, you can expect about close to about 1% to 1.5% increase.

Moderator: Next question is from the line of Varun Hemant, individual investor.

Varun Hemant: Congratulations on a good set of numbers. Congratulations on a good set of numbers again. I just wanted to ask any update on the merger between the two companies?

Adil Agarwal: So right now, as we had disclosed in the DRHP Varun, the merger of the listed sub with the hold-co is under consideration. Management is now evaluating this further along with advisors. We continue with the endeavor to complete the merger in outer limit of the next 3 years, but we believe in the next 1-1.5 years, we should be able to get this one.

Varun Hemant: Okay. Great. And I just wanted to ask -- there was an investor before me who was asking some questions about the subsidiary. I just wanted to reconfirm. So there's no other growth capex planned in the subsidiary except for the large hospital, which is currently underway already. There's nothing else as of now.

Adil Agarwal: We are looking at 3 to 4 more potential centers, and we are looking at sites. We're looking at sites in Chennai. We're looking at a place called Tirupattur and some of these other markets. Just that we don't have the property finalized, we're not able to comment on some of those locations, but we are looking at sites actively, both in the Chennai region and in the Tamil Nadu region for us to further expand in the subsidiary level as well.

Varun Hemant: Okay. Fine. And finally, I just wanted to ask, is there any particular reason why the subsidiary will only continue to look at Tamil Nadu as a market for expansion and not venture elsewhere?

Adil Agarwal: We believe this region has the highest potential, and we are able to deliver the best particular margins. We believe we have a certain competitive advantage in this particular region, which is why we continue to focus on this particular market. So if some opportunity comes about in other regions, we will consider. But right now, the thought process is that most of the expansion in Tamil Nadu will only happen in the subsidiary level.

Moderator: We have our next question from the line of Ashish, an individual investor.

Ashish: My question has been answered.

Moderator: We have a next question from the line of San

keth Gupta, an individual investor.

Sanketh Gupta: I have a single question . Actually, I have seen the Google rating of all the Dr. Agarwal centers and the new center in Delhi. Generally, the Google ratings are around 4.8, 4.9 and approximately more than 3,000 to 5,000 people rate these centers. I want any comments regarding how we achieved this great set of Google ratings.

Rahul Agarwal: Sanketh, this is Rahul. Thanks for the question. I think you're absolutely right. In most of our centers, we get a lot of our patients who are happy with our services, and they really are happy to recommend us and give us good ratings. So I think that's one thing that we are very proud of. And I think that's something that really motivates the entire team when we get these kind of ratings. So thanks for really noticing it.

Sanketh Gupta: And sir, one more question about if you see any competition in terms of the Lenskart like businesses where glasses and all these sales, how it is doing in your centers?

Adil Agarwal: Sanketh, see there is a fundamental difference between us and Lenskart. Lenskart is fundamentally an eyewear retail company. We are fundamentally a health care services company, which focuses on eye care. So the business model is completely different. Our primary focus is on eye care. Patients walk in. They go through a rigorous checkup with all our optometrists and our doctors. And to provide an additional service is why we have retail operations within our hospitals where we sell glasses and lenses.

So the business model is completely different. So I don't think it's possible to compare us with the Lenskart in terms of what they're doing. For example, many of the typical patient who walks into our hospital is typically a slightly older patient, whereas the demographic of patient who walks into Lenskart is fundamentally a slightly younger patient, right? Also, they focus a lot on a lot of eyeglasses and a lot of other stuff as well which we don't focus on. Our focus is only on prescription glasses. That's something which we predominantly focus on.

Sanketh Gupta: Okay, sir. And one more question is about the center. How is it performing in New Delhi, the new center? I don't want to add exact numbers, but how is your expectations and performance?

Adil Agarwal: Sure. So that's a very relevant question because this is something which will open up our expansion into the entire Delhi NCR region. I'm happy to share with you all that we have at the Dr. Agarwal 's Health Care Limited

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requisite licenses in our Delhi facility. We got that at the end of June. July was when was our first full month of operations. Over 1,000 new patients have walked into our facilities in our Delhi facility, and that is something which continues to grow.

We have a very strong doctor team right now and we cover all specialties. We also have the best - in-class equipment at our Delhi facility in South Ex. And so far, the results have been extremely positive. So we are happy to see how things are progressing with our first expansion into the Delhi area.

Moderator: As there are no further questions.

Adil Agarwal: Sure. So thank you, everyone, for your time, engagement, and thoughtful questions today.

Should you have any follow -up queries, please feel free to contact our Investor Relations team.

We look forward to speaking with you again next quarter. And as usual, we truly appreciate your continued support. So thank you , everyone, for taking the time to get on the call. Thanks, Manav, and thanks, everyone.

Moderator: Thank you, sir. On behalf of Dr. Agarwal's Health Care Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Sep 2025

Ref: AHCL/2025 -26/C0 35

September 03 , 2025

BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street,
Mumbai – 400001 National Stock Exchange of India Limited
Exchange Plaza, Plot No C/1, G Block,
Bandra -Kurla Complex
Bandra (E), Mumbai – 400051
SCRIP Code: 544350 Symbol: AGARWALEYE

Subject: Analyst Call – Transcript

Pursuant to Regulation 30 and 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the analyst conference call held on August 28, 2025.

The said transcript is also available on the website of the Company at : <https://dragarwals.co.in/dr-agarwals-health-care/#analyst-earnings-call>.

Yours Truly,
For Dr. Agarwal's Health Care Limited

Thanikainathan Arumugam
Company Secretary and Compliance Officer

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"Dr. Agarwal's Eye Hospital Merger into Dr. Agarwal's Health Care Limited Conference Call"

August 28, 2025

MANAGEMENT : DR. ADIL AGARWAL – CHIEF EXECUTIVE OFFICER , DR. AGARWAL'S HEALTH CARE LIMITED
MR. YASHWANT H VENKAT – CHIEF FINANCIAL OFFICER , DR. AGARWAL'S HEALTH

CARE LIMITED

MR. THANIKAINATHAN ARUMUGAM – COMPANY SECRETARY , DR. AGARWAL'S
HEALTH CARE LIMITED

MR. SUNNY KOTHARI – HEAD FINANCIAL CONTROLLER , DR. AGARWAL'S HEALTH
CARE LIMITED

MS. AASHNA DHARIA – HEAD, INVESTOR RELATIONS , DR. AGARWAL'S HEALTH
CARE LIMITED

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Moderator : Ladies and gentlemen, good day and welcome to the Conference Call on the merger of Dr. Agarwal's Eye Hospital into Dr. Agarwal's Health Care .

As a reminder, all participant s' lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*', then '0' on your touchtone phone. Please note that this conference is being recorded.

I would now like to hand the conference over to Ms. Aashna Dharia - Head of Investor Relations from Dr. Ag arwal's Health Care Limited. Over to you, Ms. Aashna Dharia.

Aashna Dharia: Thank you . A very good morning, ladies and gentlemen. Thank you for joining us on this call. Today marks a significant milestone for Dr. Agarwal's Group as we announce the proposed merger of Dr. Agarwal's Eye Hospital into Dr. Agarwal's Health Care . From the Management side, I would like to introduce the participants on today's call.

Dr. Adil Agarwal - Chief Executive Officer; Mr. Yashwanth Venkat - Chief Financial Officer; Mr. Thanikainathan - Company Secretary; Mr. Sunny Kothari - Head Financial Controller.

We have also released the Press Release and Investor Presentation which are available on our website and on Stock Exchanges .

Before we begin, I would like to remind everyone that this call is being recorded and the transcript will be made available on our website afterwards.

It is now my pleasure to hand over the call to Dr. Adil – our Chief Executive Officer, who will share his opening remarks. Dr. Adil, over to you.

Adil Ag arwal: Thank you, Aashna. Good morning to all of you and a very warm welcome. Thank you for joining our Conference Call today.

The merger of Agarwal Eye Hospital Limited into Agarwal's Health Care Limited is an important strategic step in our group's journey and will drive both operational and financial efficiencies through streamlined functions and faster decision making. It will enable unified capital allocation, strengthen the balance sheet to s upport future growth, and provide a simplified legal, regulatory, and governance framework. This long - awaited move towards a simpler and more efficient group structure reinforces our commitment to creating long -term value for all our stakeholders. Achievin g this milestone reflects focused effort and determination of our entire team. As we had Dr. Agarwal's Health Care Limited

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promised, we remain fully committed to completing the merger at the earliest and unlocking the next phase of our growth journey.

Now, to shed some light on the process followed, we adhere to the highest level of corporate governance standards for this merger. PWC Business Consulting Services, LLP and Bansi S. Mehta Valuers, LLP, the Independent Valuers acted as the joint valuation advisors and have recommended a share s wap ratio of 23 equity shares of AHCL for every 2 shares of Agarwal Eye Hospital Limited. The fairness opinion on the share exchange ratio recommended by the valuers has been provided by the SEBI registered Category 1 Merchant Bankers, Kotak Mahindra Capit al Company to AHCL , and by Motilal Oswal Investment Advisors to AEHL, respectively.

With that, I would now like to invite Mr. Yashwant h Venkat – our Chief Financial Officer, to walk you through the specifics of the transaction details.

Yashwant h Venkat : Thank you, Dr. Adil.

Let me split this transaction into two parts to make it easier for the understanding of all. We begin with the preferential allotment of AEHL shares to AHCL and then move on to the proposed merger. In relation to the preferential allot ment, AHCL will require 1,32,827 equity shares of AEHL at a price of Rs. 5,270 per share, amounting to a total investment of approximately Rs. 70 crores. The

funds infused into AEHL will be

utilized to finance its immediate fund requirements for the CAPEX plans, specifically for the construction of the flagship facility at Cathedral Road. Post the preferential

allotment, the total equity share capital of AEHL is expected to increase from 47 lakh shares to 48.3 lakh shares. Within this, the promoter's holding will rise from 71.9% - 72.7%.

The number of shares held by public shareholders will remain unchanged at 13.2 lakh shares. However, their shareholding percentage will dilute from 28.1% to 27.3%. While the preferential issue is announced simultaneously with the merger, it will not be subject to the completion of the merger and will get completed ahead of the merger. Key point to note here is, there is no impact of the preferential issue on the eventual public shareholding in the merged entity.

Now, moving on to the second part, the proposed merger of AEHL into AHCL through a scheme of amalgamation. The scheme provides for merging of AEHL with AHCL, creating a single listed entity, combining business operations of both the entities. Following the merger, AEHL will be absorbed into AHCL and its public shareholders will directly become shareholders of AHCL. We believe it represents a clear win-win.

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for both AHCL and AEHL. From AHCL shareholders' perspective, the merger will result in simplification of corporate and corporate governance structure, better flexibility and ease of conducting integrated business operations, alignment of interests with benefits of combined business accruing to all shareholders.

From AEHL shareholders' perspective, the merger will provide participation in AHCL's high-growth pan India story and access to a larger clinical talent pool, access to a larger cash pool for expansion and growth, ease of operations and better utilization of combined business resources, being a part of the company with larger market cap. As mentioned previously by Dr. Adil, we have adhered to the highest levels of corporate governance standards for this merger. Several market-leading firms have helped us with due diligence across legal, financial and tax diligence. Under this scheme, I reiterate each shareholder of AEHL will receive 23 equity shares of AHCL for every 2 equity shares held in AEHL as per the recommended swap ratio. The swap ratio implies a 15% premium for AEHL over AEHL's 10-day VWAP of INR 4,554 per share.

In terms of the post-merger shareholding of the combined entity, AEHL public shareholders will hold 4.6% stake in the combined entity. AHCL public shareholding will get diluted from 67.6% to 64.5%. Promoter and promoter group will hold close to 30.9% stake in the combined entity. Total share capital of the combined entity would be approximately Rs. 33.13 crores.

Lastly, on indicative timeline and next steps, the implementation of the transaction is expected to take around 12-14 months, subject to requisite approvals. As we move forward, it is important to recognize the milestones that remain in completing this transformative merger. The process will involve obtaining key regulatory approvals, beginning with no-objection letters from the stock exchanges and SEBI, followed by the approval from the NCLT. Shareholder and creditor meetings will play a critical role in the merger scheme. At these NCLT convened meetings, the scheme will require approval from a requisite majority, including approval from majority of minority shareholders of both AEHL and AHCL. Upon receipt of these approvals, the process will culminate with the allotment, listing, and trading of the new shares, marking the successful completion of the merger.

Thank you all. We will open the floor to questions.

Moderator: Thank you. We will now begin the question-and-answer session. The first question comes from the line of Binay Singh from Morgan Stanley.

. Please go ahead.

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Binay Singh: Hi, team. Good to see the transaction going through. Just two questions. First is, in the presentation also, we talk about cost reduction and synergies and compliance costs going down. So at the EBITDA level, is there anything major that you would call out as an expense that is being incurred on both sides which will go away when the entity merges? That will be the first one. If there is any cost item that is happening in two sides which basically drives efficiency now?

Adil Agarwal: Binay, thanks for the question. I will just request our CFO, Yashwanth to just answer what are the cost synergies we are going to get from this transaction.

Yashwanth Venkat: See, in terms of cost synergies, Binay, the main thing is on the compliance costs. One, currently since we have two companies, in terms of the auditors, we have to have two separate auditors for both the companies. That is one major cost. Apart from that, in

terms of cross -charge across both the entities, there will also be a significant reduction in tax leakage from GST perspective, two. Three is also we believe that there will be a little bit of optimization of resources once this becomes a combined entity. Also, other costs such as filing costs, etc., will also come down.

Binay Singh: Thanks for that, Yash. And secondly, just on the transaction, the Rs. 70 crore s preferential issue, any reasons for taking this route instead of giving a loan to the subsidiary? Any thoughts on that? Thanks.

Adil Agarwal: Binay, I can take that. We believe that instead of us putting any more stress on the balance sheet at the AHCL level, we feel that this will be a better route for us to do a preferential allotment. Since we have a good amount of cash reserves at the holding company level, we felt that the preferential allotment done at a fair valuation will make the most sense for us.

Binay Singh: Great. Thanks, team.

Moderator: Thank you. The next question comes from the line of Ankit Shah, an Individual Investor. Please go ahead.

Ankit Shah: Hello. Actually, I am a shareholder of Dr. Agarwal's Eye Hospital, and I have been a shareholder for the last 18 years. So I am joining this call to understand why the merger ratio is so in favor of the holding company rather than the subsidiary company. And since I have been a shareholder for such a long time, what I can definitely vouch for is that the corporate governance levels are at the highest levels, right, even for a very small company, which has been listed for the last 30 years, I think the corporate governance levels are very high. But I am just upset that the merger ratio is in favor of the holding company, right? Because if I were to just do a basic analysis of FY '25 Dr. Agarwal's Health Care Limited

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revenues, right, and I adjust the revenues of the holding company for the minority interest of the AEHL, the ratio should be more towards 15 -17 shares for each share held for the subsidiary company. Even if I look at EBITDA, even if I look at PBT, any metric I look at, right, and I am not even counting for the fact that there has been a massive CAPEX, which has happened at the subsidiary level for the flagship hospital. And I am not even counting the increase in revenues that are going to happen because of that, right? But even if I just look at FY '25 revenues, I think the merger ratio just seems very off. It is 30% lower than what any basic analysis would suggest. I don't know what this fairness opinions are all about, because I am just doing a very basic back of the envelope calculations. Thanks if you can respond?

Adil Agarwal: Thanks, Ankit. Your points are noted. I will request our CFO to explain the rationale behind how we arrived at this share swap ratio.

Yashwanth Venkat: Hi, Ankit. In the

terms of the swap ratio, the swap ratio determined for the merger has been recommended by independent registered valuers on which fairness opinion has been provided by Independent SEBI registered merchant bankers. See, the valuers have used several valuation methodologies, including market value, trading multiples, DCF, and also applied appropriate weights to these methods to arrive at the final value of each business. The valuation was based on latest available historical financials and outlook for the respective businesses as well. So post that, the valuation reports and fairness opinions were placed before the Audit Committee and Boards of the respective committee for their necessary consideration and approval of the transaction. We believe overall, we think it is a fair deal for the shareholders of both companies as it provides them with an opportunity to participate in growth and value creation in the long run, Ankit.

Ankit Shah: No, I definitely agree with both the parts where you are saying the process, which is actually a legal process, which you have sort of used in terms of getting a fairness opinion and all of that. But we all know how those fairness opinions work because, it is all like DCF is all based on assumptions, because who knows the future, right, in terms of what the revenues and profits are going to be? So I don't know how that all, but I am just saying that if I just look at today and take a very dispassionate perspective and just say that the revenues of the subsidiary companies are roughly 25% of the holding company, I am just doing a very basic analysis, right? And as I said before, I gave the context that I have been a shareholder for 18 years and I think the management and founders have been fair in terms of the governance and everything. But this one just seems, right, so that is just my perspective now. The legal process and all I understand because you get a fairness opinion and all of that, that is understandable that every company does. But this is more about just at a very basic level when I am

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looking at it, it just seems unfair to the minority shareholders of the subsidiary company. So that is my point. Anyway, there is no point to discuss further, but that is just the point I wanted to make. And if I would request Adil to see if the merger ratio can be improved. So that is just my humble request. Thanks?

Adil Agarwal: Thanks, Ankit. Your points are noted. We will evaluate. Thanks.

Moderator: Thank you. The next question comes from the line of Siddharth Chandrasekhar, an Individual Investor. Please go ahead.

Siddharth Chandrasekhar: Yes. Hi, sir. So again, my point is also on similar lines to the previous caller.

So as we see, I am doing again the same similar level of comparison when compared to the parent and the subsidiary. So on all parameters, revenue, we can say like one-fourth of revenue we are getting from subsidiary and even EBITDA level or PAT, even I think one-third of PAT is coming. So on all parameters, if I simply compare any level of parameters, except two, one I would say like geographical concentration, the other one is on growth. I just want one information. So why we are restricting our subsidiary only to Tamil Nadu and the second one is on the growth parameter. If we see, growth somewhat looks higher in parent business purely because we have done lot of equity inclusion, which has not happened in the subsidiary level. I don't know like, why because it is already a listed company and in past itself, we could have raised money in the subsidiary and we could have done a better growth here, right. So I am just wondering like it seems like both the companies are not treated fairly, right, in this case. I just want your opinion on that. So I understand like you will get that fair value, but I don't know like, o

n what parameter this becomes a fair value. Because we could not say like, okay, this is where parent holds better perspective than compared to subsidiary. So all the growth and everything comes with the fact like last 5 years, I think you have from VC funds and IPO funds. So everything is equity-based, right. None of them is organic one, right. So organic, if you compare the parent versus subsidiary, subsidiary is a better return profile, right. So just want your opinion. So again, let us not go to the fair value problem, but I am just want to see like what is your opinion on this?

Adil Agarwal: So Siddharth, we believe that we have followed a fair process, which is being advised to us by our respective advisors, and we believe the highest standards of corporate governance have been met. AEHL shareholders shall be benefited from this transaction on multiple grounds, including the fact that they will become shareholders of an entity which has a pan India presence with a much larger critical talent. They will have access to consolidated cash resources, which can be strategically deployed for growth and continued operations. Also, they will be part of a larger growth story and a larger Dr. Agarwal's Health Care Limited

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market company, and they will continue to be part of that growth structure. So we believe from that perspective, it is a fair deal for shareholders of both the companies as it provides them with opportunity to participate both in growth and value creation in the long run.

Siddharth Chandrasekhar: Yes, my question is like why we haven't raised any funds in subsidiary, right? I already understood one could have gone with rights issue or QIP or something like that, and could have increased growth here itself, right? And also like, why we restricted subsidiary only to, now we are saying like, okay, parent is a pan India, but who stopped it, right? So it is not like someone else stopped it, right? So, both companies are under your management only, right? So you could have grown this on pan India level basis, right?

Adil Agarwal: We understand your concern, Siddharth, but this is something which has been happening, and the subsidiary also has been given a lot of support by the parent company and vice versa as well. And we will ensure now that with this entire merger, now all the AEHL shareholders can now participate in that growth, which is happening at a pan national level.

Siddharth Chandrasekhar: No, but that comes at the cost of the shareholders of the subsidiary, right?

Adil Agarwal: See, we have taken a fairness opinion from multiple sources, Siddharth and we believe that whatever is being proposed right now is a fair deal, and both the Boards have approved the matter.

Siddharth Chandrasekhar: Sir, I am just voicing my opinion here. I am very displeased with the way this is. So even, subsidiary was listed for a very long time. We did not do any concall or investor presentation. So suddenly our parent comes in the picture, we are doing all, I am not saying any corporate governance issue on, something on the operating level, just pointing out some facts, right? So somewhere, shareholders of subsidiary were

very much in a disadvantage d stage at this point. Ye s, that is all from me.

Adil Agarwal: We understand Siddharth, we understand.

Moderator: Thank you. The next question comes from the line of Anand ha Padma nabhan from PGIM India AMC. Please go ahead.

Anandha Padmanabhan: Thank you for taking the question. Just wanted to understand the steps ahead. So in terms of shareholder 's approval for th e scheme, for the merger, for Agarwal Eye Hospital, would it require majority of all the shareholders or the majority of the non - promoted shareh olders?

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Adil Agarwal: On the process, I will request our Co

mpany Secretary , Mr. Thanikainath an to just give

you an update on what is the next requisite steps and the policies, right?

Thanikainathan: Yes. Thank you, doctor. On that, the process just require s the majority of minority approval at both AH CL as well as at the AEHL level. As a process, first, we will apply to the Stock Exchange and SEBI. Post getting their clearance, we will approach NCLT. Then NCLT will convene the shareholders and creditors meet ing. At that point in time, we require the majority of minority approval at both the compan y's level.

Anandha Padmanabhan: And what happens in case, for AE HL, majority of the minority shareholders do not approve the merger, then what is the next step in that case?

Thanikainathan: Then , this is subject to outcome from the NCLT. So we have to follow the NCLT process again.

Anandha Padmanabhan: Thank you. That is all from my end.

Moderator: Thank you. The next question comes from the line of Anuj Jain from Eq ui Capital. Please go ahead.

Anil Jain: Yes. I am An il Jain from Equipa ssion Capital. I have been a shareholder since last 8-9 years. In continuation with the previous participants that since the Dr. Agarwal Group is being known for this very good corporat e governance , but looking at this announcement yesterday, we were shocked that the merger ratio was completely in favor of parent company and completely ignoring the minority shareholders who are holding the shares since 30 years, since its listing in '95. And the valuation can be like at any parameter, like revenue, EBITDA, PAT , at all levels, it completely looks in favor of holding company. So my humble request is now on behalf of all minority shareholders is that please relook into this swap ratio. That is all we can do now as a minority shareholder. It is completely unfair. I don't know what is this reasonable valuation by the category I merchant banker, this and that . So, as a minority shareholder, we can just request Dr. Adil and all the management and Board of Directors of the group to reconsider it. That is all from my side.

Adil Agarwal: Thank you, An il. Your point is noted. We just would like to reiterate the fact tha t the proposed transaction is being done at an arm's length. Independent valuation of equity shares of both the listed subsidiary AEHL and the holding company AH CL have been carried out jointly by PWC and Bansil Mehta, who are the valuers, and they have recommended the share exchange ratio under the scheme. The fairness opinion also was provided by a semi -registered category I merchant banker, which is Kotak Mahindra Capital for AH CL and similarly, Motilal Oswal Investment Advisors to Dr. Agarwal's Health Care Limited

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AEHL. These valuers hav e used internationally accepted valuation methodologies, which include market value approach, a comparable complete approach, and a DCF approach. It is not just one metric which was used , three different approaches were used to arrive at a fair valuation f or both entities based on which a shared swap ratio was announced. We are happy at some point to speak with the A EHL shareholder separately to explain how the valuation was arrived at, and you will appreciate the fact that it was done in a very scientific methodology. For the market price method, the valuers have factored in the prices for an appropriate period in the past to avoid any impact of any current volatility. Valuation reports and fairness opinions were placed before the audit committees and the b oards of both the respective companies for their necessary consideration and approval. So if you believe we are running a company which has high on corporate governance, you will appreciate the fact that we have very respected boards on both entities. Overall, again, I reiterate, we think it is a fair deal for the shareholders, but we understand some of the concerns of the minority

shareholders at the A EHL level. We respect you and we value you and we will try to

do our best. Thank you once again.

Anil Jain: Yes, just in continuation, we definitely agree with the fact that this group is definitely known for corporate governance since last 30 years, since listing. But my one question is that I don't know what international practices of valuation they are talking about. The thing is that it is completely unfair since I am also in the market since last 30 years. I don't know on what metrics, because we can see topline, EBITDA, PAT, growth, net worth, debt, equity on all parameters, it looks completely unfair. I don't know which hidden criteria which you are talking about which I can't see and which no layman investor can see. Even a senior pro-investor can't see. I don't know which fairness they are talking about. And my one question is that if these merchant bankers would have been the minority shareholder of Eye Hospital, would that be fair? You just ask them that question. If you get to that question, a yes, it is in favor of Eye Hospital minority shareholder. So only a humble request we can do now that please reconsider this. This is completely unfair. And please look into this matter openly. That is all from my side.

Adil Agarwal: Sure. Your concerns are noted. We value you, again. We will try our best. Thanks.

Anil Jain: And I doubt that this is going to improve your reputation for good corporate governance group. I doubt that it is going to improve your corporate governance. Thank you.

Yashwanth Venkat: Sir, noted your point. Just one point from our end, sir. See, as far as the premium paid to AEHL shareholders is concerned, when you take the 10-day volume weighted average price as on August 2025, the premium to the 10-day VWAP is close to about Dr. Agarwal's Health Care Limited

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15%. The 10-day VWAP is 4,554. So in terms of the 10-day VWAP, the premium itself is close to about 15%, sir.

Moderator: Thank you. The next question comes from the line of Varun Hiremath, an Individual Investor. Please go ahead.

Varun Hiremath: Yes, Hi, Adil.

Adil Agarwal: Hi, Varun.

Varun Hiremath: Hi. So I think, as you can tell from the previous questions that have been raised, people are quite displeased with the ratio, right? And so I think ultimately, these things boil down to a vote and obviously, no one can predict how that pans out. But I just feel that opinions have been voiced and there is a reason the holding company trades at such a higher valuation. Even when the IPO was done, it came at such a high valuation. Look at where other hospitals are trading in the market. The subsidiary is trading at a discount and obviously, somewhere it was felt that something like this might happen. Which is why, I think, there was a sharp fall in the price of the subsidiary post the listing. So I don't know exactly what the thesis was, when deciding this ratio, but I think at the very least, it would be great if you can organize a call separately for the subsidiary shareholders, where you can be a little more transparent about the valuation methodologies, which were utilized. And if truly that is the case and it would be great if we could be educated on that, so we can understand how to process this.

Adil Agarwal: So Varun, your point is absolutely fair, we are more than happy to meet with our AEHL minority shareholders offline and discuss this in detail. We can find a proper time and we can take you all through the details. You would all understand it is a fair and a very transparent process. And we are more than happy to discuss this with you in person.

Varun Hiremath: Yes, I think so. Please just, I will email in about that. Please just set that up for us. I am sure there will be a lot of people who are interested, so would appreciate that.

Adil Agarwal: We will do that.

hat. Thanks, Varun. Thanks for that.

Moderator: Thank you. The next question comes on the line of Ankur Shah from Quasar Capital. Please go ahead.

Ankur Shah: Yes. Hi, thanks. So interestingly, just sharing one perspective. Again we are very small minority shareholders in the AEHL entity. And interestingly, the management has seen both the sides in the last one year where they were selling shareholders while doing an IPO. This is just another perspective, adding to the perspectives which previous Dr. Agarwal's Health Care Limited

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participants have already mentioned. This is a question to the valuation teams of PWC and Bansi Mehta, that in last one year, when the management is a selling shareholder, the valuation is like if EBITDA has to be taken, then 25x plus, even the deals are happening like HCG deal, Rainbow, all the single speciality guys, they trade at 25 plus. With this brand name, same management team, everything same, interestingly, it is like from all angles, it is a very unfair deal. And secondly, on the fairness of opinion,

I would like to mention a conflict of interest that your BRLM managers of the AHCL entity have provided the fairness of opinion. So again, within one year, the AHCL company has given a large cheque of fee to both of these companies who have provided fairness of opinion. So I would actually point out a conflict of interest over there. So yes, otherwise, I think the rest of the points are already very well mentioned. And yes, I think organizing a call to understand any more scientific logic, than what my education serves me, because we are also CAs, we understand, we have worked in transaction advisory departments on how valuations are done. So we would like to understand how this scientific valuation does work. Thank you so much.

Adil Agarwal: Thanks, Ankur. Your concerns are noted. I think doing a separate call offline will definitely help you all understand and assuage some of your concerns which you are having on the valuation.

Moderator: Thank you. The next question comes from the line of Viraj Shah from PGIM. Please go ahead.

Viraj Shah: Hi, just one question from my side. Can you please let me know if the merger is happening via special resolution or normal resolution?

Adil Agarwal: I will just request my Company Secretary to answer that.

Thanikainathan A: So, for this merger, we have to pass a special resolution at both the company level, AHCL as well as A EHL from a company perspective. Also from SEBI perspective, we need to get the majority of minority approval as well.

Viraj Shah: Thank you.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to Dr. Adil Agarwal for the closing remarks.

Adil Agarwal: Thank you, everyone. Thank you for taking the time and thank you for all the support. Thanks.

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Moderator: Thank you. On behalf of Dr. Agarwal's Health Care Limited, that concludes this conference. Thank you for joining us. You may now disconnect your lines.

Call: Nov 2025

Ref: AHCL/2025- 26/C52

November 04, 2025

BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street,
Mumbai – 400001 National Stock Exchange of India Limited
Exchange Plaza, Plot No C/1, G Block,
Bandra -Kurla Complex
Bandra (E), Mumbai – 400051
SCRIP Code: 544350 Symbol: AGARWALEYE

Subject: Analyst/ Earnings Call – Transcript

Pursuant to Regulation 30 and 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the analyst/ earnings conference call held on October 31 , 2025.

The said transcript is also available on the website of the Company at : <https://dragarwals.co.in/dr-agarwals-health-care/#analyst-earnings-call>.

Yours Truly,

For Dr. Agarwal's Health Care Limited

Thanikainathan Arumugam
Company Secretary and Compliance Officer

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"Dr. Agarwal 's Health Care Limited
Q2 & H1 FY26 Earnings Conference Call"
October 31, 2025

MANAGEMENT : DR. ADIL AGARWAL – CHIEF EXECUTIVE OFFICER –
DR. AGARWAL 'S HEALTH CARE LIMITED
MR. RAHUL AGARWAL – CHIEF OPERATING OFFICER –
DR. AGARWAL 'S HEALTH CARE LIMITED
MR. YASHWANTH VENKAT – CHIEF FINANCIAL
OFFICER – DR. AGARWAL 'S HEALTH CARE LIMITED
MS. AASHNA DHARIA – HEAD OF INVESTOR
RELATIONS – DR. AGARWAL 'S HEALTH CARE LIMITED

Dr. Agarwal 's Health Care Limited
October 31, 2025

Moderator: Ladies and gentlemen, good day and welcome to Q2 and H1 FY 26 Earnings Conference Call hosted by Dr. Agarwal's Health Care Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Aashna Dharia, Head, Investor Relations from Dr. Agarwal's Health Care. Thank you and over to you, Ms. Aashna Dharia.

Aashna Dharia : Thank you, Saisha. A very good morning, ladies and gentlemen. Welcome to Dr. Agarwal's Health Care Q2 and H1 FY 26 Earnings Call. From the management side, we have Dr. Adil Agarwal, Chief Executive Officer; Mr. Rahul Agarwal, Chief Operating Officer; Mr. Yashwanth Venkat, Chief Financial Officer.

We've also released the financial results, press release and investor presentation, all of which are available on our website and the exchanges as well. Before we continue, we want to remind everyone that this call is being recorded, and the transcript will be made available on our website afterwards.

Additionally, please be aware today's discussion may include certain forward-looking statements, which should be considered in the light of the risks our business faces. Please refer

to the detailed statement on Page 2 of the investor presentation. It is now my pleasure to hand over the call to Dr. Adil Agarwal, our Chief Executive Officer, who will share his opening remarks and insights. Dr. Adil, over to you.

Adil Agarwal : Thank you, Aashna. A very good morning to all of you, and a warm welcome to the Q2 and H1 FY '26 earnings call of Dr. Agarwal's Health Care Limited. Let me begin by providing you all an update of the Q2 and half yearly performance. In line with Q1 trends, the company maintained a steady momentum in H1 FY '26, surpassing the INR 1,000 crores total income mark for the first time.

PAT stood at an all-time high of INR75 crores for the first half, up by 88.4% year-on-year. For the first half of FY '26, the company reported a total income of INR 1,007 crores, up 20.2% year-on-year, while revenue from operations rose 20.2% to INR 986 crores. H1 delivered a robust IndAS EBITDA of INR285 crores, reflecting a 24.9% year-on-year growth, with margins improving by 100 basis points to 28.3%.

PAT grew 88.4% year-on-year to INR 75 crores, with PAT margins expanding by 270 basis points to 7.2%. For the second quarter of FY '26, the company reported a total income of INR 507 crores, up 18.2% year-on-year, while revenue from operations rose 19.7% to INR 499 crores.

H1 delivered a robust IndAS EBITDA of -- sorry, Q2 delivered a robust IndAS EBITDA of INR144 crores, reflecting a 21.2% year-on-year growth, with margins improving by 70 basis points.

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points to 28.4% for Q2. PAT grew 71% year-on-year to INR 36 crores, while PAT margins expanded by 220 basis points to 7.2% for Q2.

Next, I would like to share our footprint and network growth update. Now during the first half of FY '26, we served over 14.3 lakh patients and performed nearly 157,000 surgeries through our network of over 258 facilities, which comprises 29 hubs and 229 spokes. In India now, we have a total network of 239 facilities across 14 states and 5 union territories covering 141 cities. Our presence is well diversified now, with 31% of our facilities in Tier 1 cities, 62% in other cities and 7% located internationally. We expanded our footprint by commissioning 24 new Greenfield facilities, further strengthening our reach and capacity. Now in addition to the 13 centers which were commissioned in the quarter

after 1, the company added 11 new facilities in quarter 2.

These included 6 secondary centers at Sivakasi and Thudiyalur in Tamil Nadu, Bhopal in Madhya Pradesh, Aurangabad in Maharashtra, Palakkad in Kerala and Hassan in Karnataka, along with 5 primary centers, 2 in Belgaum, 1 each in Gadag, Haveri and Jharsuguda. Continuing from our previous update on our Delhi market entry, operations have begun on a strong note with encouraging traction in patient volumes and revenues scaling up month-on-month.

The response reinforces our confidence in the region's potential and validates the strength of our operational and clinical proposition. Building on this momentum, we plan to significantly expand our presence rapidly in the Delhi-NCR market over the next 12 months. Our focus on clinical excellence and deployment of advanced technology remains unwavering.

This commitment is reflected in the growing share of specialized surgical procedures across our network. To further add, in H1 FY '26, high-end cataract surgeries accounted for 25.8% of total

cataract procedures. 29,697 surgeries were done in total, an increase of 41.7% in the same period over last year.

Robotic cataract surgeries, what we call as Femto Cataract, grew by a robust 69% year -on-year, rising from 1,548 to 2,616 procedures. As shared in our previous earnings call, new robotic cataract systems have now been recently installed at our Velachery facility in Chennai and our Delhi facility, which now performs close to 60 such surgeries per month on average.

Lenticular Procedures, what we call as SMILE surgeries, increased 11.5% year -on-year from 2,393 to 2,668 surgeries, while retinal surgeries total led 6,205 in H1, up by 23% from last year.

Corneal transplants also rose to 578 procedures during the same period. We will continue to invest in cutting -edge equipment to enhance diagnostic precision and surgical outcomes.

Recent upgrade includes a new Visu Max 500 at our centers in Aundh in Pune and Cochin in Kerala. Our Phaco Cataract system at Madeenaguda in Hyderabad and LUMERA 300 operating microscope at our new center, which we have launched in Sambhajinagar in Maharashtra. On the research and capability building front, our clinicians have contributed to over 320

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publications in leading international medical journals over the past three decades, underscoring our commitment to advancing ophthalmic science.

In Q2 FY '26, we closed four clinical research studies and have currently 16 ongoing studies.

During the quarter, nearly 80 doctors underwent advanced training across multiple specialties as part of our continuous L&D initiative. Now moving on to some business updates. Let me first begin with region -wise performance.

Our Southern region continues to be our largest market, contributing to 64% of our total group revenues. This region delivered INR 635 crores of revenue, representing a strong year -on-year growth of 22.2%. We have 165 facilities across our southern states, and this includes 16 new additions in this half of the year.

Four new facilities were added in Tamil Nadu, 7 new facilities were added in Karnataka and two each in Andhra Pradesh and Kerala and 1 in Telangana. Tamil Nadu witnessed a slightly lower growth this quarter on account of significant rains due to the southwest monsoon. However, the states of Telangana, Karnataka and Andhra Pradesh witnessed strong growth. We remain focused on sustaining our market leadership in Tamil Nadu and Telangana, while further increasing adoption of the latest technology.

In Karnataka and Andhra Pradesh, we are strengthening our presence in both these markets by expanding significantly into high- potential underserved markets through strategic facility openings. Now coming to the West region. This region contributes to about 15.2% of the overall group revenues and delivered

INR 150 crores of revenue, up by 16.6% year -on-year despite some impact from the festive season and heavy rainfall during the quarter.

We currently have 43 facilities, including three new facilities opened in the first half. Patient volumes grew by 20% to over 2.21 lakhs, while surgeries performed increased by 14.7% to 24,205 procedures. Maharashtra, as we have mentioned before, continues to be a key region for

us as we aim to deepen our presence in the underserved regions of the state while further expanding in micro markets of metro cities such as Mumbai and Pune.

In Gujarat, we remain optimistic given the strong performance of our new centers in Surat. We plan to further strengthen our footprint in the state by adding more facilities and accelerating adoption of advanced technologies. North region contributes to 7.3% of our group revenues, and this region reported INR 72 crores of revenue this quarter, up by 14.2% year -on-year.

We currently operate 22 facilities in the region. And based on the strong performance of our Delhi main hospital, we plan to further expand our presence across North India with additional

facilities in Delhi-NCR, Punjab and Uttar Pradesh. Overall, our regions delivered positive growth, with the South leading both in scale and year -on-year momentum, while the North, West and East continue to build scale.

Now, moving on to same -store sales growth. Our mature centers are those that have been open for more than 3 years. As of September 2025, we operate 123 mature facilities. Revenue from

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these mature facilities increased by 13.4%, reaching INR 734 crores in the first half of FY '26, contributing to 75% of total group revenues. Revenue from our mature facilities in India has grown by 19%, touching INR 618 crores.

Now, moving on to our vintage performance in H1 FY '26. Facilities operational prior to FY '22 contributed to INR 685 crores,

registering a year -on-year growth of 13.9%, underpinning our strong same-store sales growth performance. Facilities opened in FY '23 generated INR 121 crores, growing at 13.7%, while those launched in FY '24 delivered INR 79 crores in revenue, achieving a strong growth of 18.3%.

Moving on to our next business update, our planned facility openings domestically for each quarter of the current year. Over the next 2 quarters of this financial year, we're targeting to launch another 30 facilities, which will comprise 17 in the South, 6 in the West, 4 in the North and 4 in the East. Every quarter, we share a glimpse of our newly launched facilities to illustrate the quality and the patient-centric design of our network.

Today, I would like to highlight performance of one of our recently relocated facilities in Whitefield in Bangalore. The Whitefield facility was relocated in August 2024 within a kilometer of its earlier site and has been upgraded from a secondary to a tertiary center, driving a marked improved performance in growth and business contribution.

Before relocation, the facility operated with 5,700 square feet on two floors, but now the new facility for the last 12 months has recorded an average of 2,700 OPD visits, 212 surgeries on average and INR 2.1 crores in revenue per month, recording a growth of more than 20% since we have relocated to this new 20,000 square feet facility spread across three different floors. Key drivers post -relocation included diversified revenue streams, a larger and more specialized doctor team, enhanced infrastructure and improved brand visibility through digital marketing. Overall, the relocation established a strong foundation for sustained accelerated growth and strategic expansion.

To conclude, I would like to highlight that our continued emphasis on operational efficiencies and disciplined execution has enabled us to close the first half on a strong note despite the impact of festivals

and unexpected rainfalls across parts of India. Now, I would like to hand over to our CFO, Mr. Yashwanth Venkat, who will take a deeper dive into our financial performance.

Yashwanth Venkat : Thank you, Dr. Adil. I'll begin with the operational update. Surgical services continue to be the main revenue driver, contributing 65.6% to the group revenue. Diagnosis, consultations and other non-surgical treatments contributed 13.6%, and the sale of optical products and pharmacy items accounted for 20.8%.

For H1 FY '26, we performed 137,244 surgeries, marking a 14.6% year -on-year growth. Cataract surgeries remain the largest contributor, accounting for approximately 17.4% of total surgeries, followed by refractive surgeries at around 4.7%. Volumes for cataract and refractive surgeries grew 14.2% year -on-year, while other surgeries on a blended basis recorded a growth of 21.4%.

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Our payer mix for H1 FY '26 stood at 61.9% from cash, 29.6% from insurance and TPA, and 8.5% from government schemes. The domestic payer mix for the half year stood at 72.3% from cash, 22.2% from insurance and TPA, and 5.5% from government schemes. Moving on to the financial section. I will start with the revenue split.

The group's revenue from operations grew by 20.8% year -on-year, reaching INR 498.69 crores in Q2 FY '26 compared to INR 416.57 crores in Q2 FY '25. Revenue from operations in India stood at INR 445 crores, reflecting a growth of 19.8%, despite unseasonal rains in our core market and greater impact on the business due to festivals.

This growth was supported by a mix of volume growth of 8%, value growth of 4.8% and balance contribution coming from the new centers opened in FY '25 and FY '26. Revenue from operations from our Africa business grew by 17.4% year -on-year in H1 FY '26, but their contribution to overall revenue declined from 10.4% in H1 FY '25 to around 10%. Doctor and employee costs cumulatively have remained broadly in line with the previous year.

We reported an EBITDA margin improvement of nearly 1.4%, attributable mainly to cost efficiencies. A total of INR 195 crores in loans have been repaid from IPO proceeds, INR 128 crores in Q4 FY '25 and the balance in H1 FY '26, leading to lower finance costs and higher profitability versus the same quarter last year.

From 72.2% in H1 FY '25, the share of profit after tax attributable to owners has expanded to 80.1% in H1 FY '26, signaling improved profitability in the holding company. AHCL at a

standalone basis has delivered a positive PAT of INR 14.8 crores in H1 FY '26 as compared to a loss of INR 12.2 crores in H1 FY '25. Thank you all. We will open the floor to questions.

Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Binay from Morgan Stanley. Please go ahead.

Binay : Congrats on a steady quarter. The first question is just on your annual guidance. We know second half tends to be stronger for the business, both on the revenue side as well as on the margin side.

In H1, you are already doing 20%. So, fair to assume that if things play out well, you actually could exceed your guidance this year?

Yashwanth Venkat : Binay, Yashwanth here. We would be more or less in line with whatever we had guided towards at the

start of the year, Binay.

Binay : And secondly, when we look at revenue per surgery, that has moved up quite sharply this quarter. It's almost up 9% Y-o-Y, whereas we know that the share of refractive is down, cataract is up.

Is this more because it's more complex cataracts you are doing? Could you share a little bit about that?

Adil Agarwal : Yes. I'm just going to request our C OO, Rahul to just take that.

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Rahul Agarwal : Binay, you are right. Cataract has been a big driver. Our c

ataract YT D has gone up to around

40.5 K which is driven actually largely by what Adil mentioned, Femto Cataract and high-end surgeries -- high-end cataract surgeries. They have driven us fairly strongly this quarter. It's actually something that we've seen on a quarter-on-quarter basis in the last few quarters where both Femto Cataract, where we've invested a lot, as well as high-end surgeries have continuously kept growing as the economy also keeps growing.

Binay : Right. Because I remember last year, this number used to be around 38,000. So, we've seen quite a nice jump in the cataract mix ?

Rahul Agarwal : You are right.

Binay : Lastly, just team on the subsidiary financials, therein also we saw a good margin expansion.

Could you talk a little bit about what played out over there, why the revenue growth decelerated for the reasons we talked about and margin rose quite sharply?

Adil Agarwal : Yes. I'll just request our CFO to just take that.

Yashwanth Venkat : Yes. A couple of things, which we have concentrated on over the last 6 months is in terms of cost efficiencies, Binay. So if you look at the profit and loss statement of our subsidiary, the one major increase which you can see is on the gross margins. For the same quarter of last year, we had gross margins of about 77.1%. The gross margins have moved to close to about 79%, one. The second point is on the doctor and employee costs. So on an overall basis, we have ensured that the doctor and employee costs are more or less in line with revenue growth in few of the facilities. And in few of the newer facilities, they are slightly lower than the revenue growth. So, we have got some cost efficiencies from that as well.

And in spite of investments in newer technologies and also opening up a couple of new facilities, D&A has not moved up significantly. D&A has also remained at about 9% to 9.5%. So as a combination of all of this, the PBT margins have moved from about 18.3% to about 21.7%, Binay.

Binay : Great. Thanks team. I will come back in the queue.

Moderator: Thank you very much. The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane : Sir, particularly for the surgery growth in North is still in like sort of mid single-digit. So if you could just elaborate in terms of effort to sort of scale this growth?

Adil Agarwal : Sure. Rahul?

Rahul Agarwal : Yes. So you're right. Actually, for North, we've had generally a tough H1. All those issues with regards to operations improved starting in Q1 to the festivities being early this Q 2 quarter, and

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then we also had the rains, very heavy rains. So, these 3 impacted us overall from a North perspective.

North typically seasonally is better in the second half of the year. So, we should see improvements coming in North in the second half of the year. However, from an overall expansion perspective as well, there's a lot of focus, which is there in Delhi right now. You will see a lot of more facilities opening in the second half of the year in Delhi.

That's something which is already underway. And slowly, we'll also get deeper into UP. So, these two expansions will start panning out. And next year, you should start seeing a lot of impact coming in with these expansions happening.

Yashwanth Venkat : Point to note more, Tushar, even when you compare the North business vis-a-vis Q1. We had reported an overall revenue growth of about 13.4% in North in half year. But if you compare it with overall in H1, it has about close to 14.2% growth, which signifies a slight improvement as far as Q2 is concerned over Q1. This will definitely move up in Q3, with a very strong finish in Q4, Tushar.

Tushar Manudhane : Sure, sir. And if you could like qualitatively also help us understand how the performance has been f

or the Jalandhar unit -- acquired unit and, let's say, the expansion in and around that unit?

Adil Agarwal : Yes. So typically, like we had mentioned before, H1 is a slightly slower time for the Punjab region, and it significantly picks up in H2. So, you'll start to see significant performance coming through from the Jalandhar region in H2. To give you a detailed breakup, Rahul will just give you some of the numbers for our Thind facility.

Yashwanth Venkat : Before we get into the numbers, a couple of points I would like to add, Tushar. So, Jalandhar unit is supported by two other facilities, one in Pathankot and Hoshiarpur. The growth has been very robust even for H1. One softer aspect is Jalandhar also, we have undertaken a complete refurbishment of the old facility, which has slightly impacted the business for about close to 3 to 4 weeks. So, we believe that once this actually is completed, we'll have a strong H2, Tushar.

Rahul Agarwal : But from an overall H1 perspective, Tushar, we've had a reasonable performance at overall 18.5% growth for the entire Thind Eye Care entity.

Tushar Manudhane : Understood. And likewise, for the acquired Bombay unit, if you could just give some color over there?

Adil Agarwal : Which is the acquired Bombay unit? Overall, Maharashtra business has grown by 16%, Tushar for the region. Entire Maharashtra region has grown by 16% for H1. Are you specifically talking about any particular facility?

Tushar Manudhane : The Tardeo one, sir.

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Adil Agarwal : The Tardeo one. We'll just give you the details on that. Just give us a second. Yes. So, we have grown close to 10% in our Tardeo facility, which is Infiniti Eye Hospital, which is located in South Bombay.

Tushar Manudhane : So like there also, there's a potential to grow at least mid -teens, right?

Adil Agarwal : Yes. So like I mentioned earlier, Bombay, again, will now start to pick up in Q3 and Q4. So, you'll start to see mid -teens growth in the Tardeo facilities coming into Q3 and Q4. Next time, we will give you an update in terms of how Q3 and Q4 have performed for the Tardeo facility.

Tushar Manudhane : That sounds great, sir. And just lastly, in terms of revenue per surgery growth factor, is it safe to assume like 5%, 6% sort of a growth consistently year -over -year given the change in surgery mix?

Adil Agarwal : Sorry, your question was around?

Tushar Manudhane : Sir, revenue per surgery growth?

Adil Agarwal : Okay. So, you have to look at it across different surgeries, right? So the biggest contributor obviously comes from cataract, where we continue to see a good amount of growth. We are seeing a good amount of increase in premiumization, and that has been around 4.5% to 5% for this quarter. We hope to continue with that kind of growth over the next few quarters.

Tushar Manudhane : Got it. Because -- I mean, it was almost like 20% overall revenue growth, out of which surgery growth has been 13.5% to 14%. So effectively pricing...

Adil Agarwal : In terms of volumes, yes.

Tushar Manudhane : Yes. In terms of volume. So 5%, 6% would have come due to better realization either on account of increase in prices?

Adil Agarwal : Absolutely correct. About 5%.

Tushar Manudhane : Understood. So going forward also, this premiumization should support at least 4% to 5% growth?

Adil Agarwal : Definitely, as we continue to add more high -end equipment and as you start to see an increase in throughput from some of the already existing added equipment across many of our regions, this premiumization will continue to be a big factor in our growth story going forward.

Moderator: The next question is from the line of Param Vora from Trinetra Asset Managers.

Param Vora: So, I wanted to ask you regarding your strategy when you enter into a Tier 2 or Tier 3 city because usually in those cities, local players dominate such seg

ment. So, how do you convince the patients to shift?

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Adil Agarwal : Is that the question?

Param Vora: Yes.

Adil Agarwal : So if you look at our current business, only about 31% of our facilities are located across metros and the remaining about 61% to 62% is located across Tier 2 and Tier 3 cities. Expanding into Tier 2, Tier 3 cities has always been a strategy that we have followed since we started expanding in Tamil Nadu. And now we have a diversified presence across many, many states.

Two, three things which we do when we expand into Tier 2, Tier 3 cities is, one is we always try to work with local ophthalmologists. We hire locally, and some of these ophthalmologists are from that particular area and they have a very strong network base in their respective markets.

Second thing is we bring best -in-class technology across all our Tier 2 and Tier 3 facilities that are available across our metro cities. That gives a lot of confidence to our patients, and we're able to deliver superior outcomes. And third, we have a very strong brand and marketing strategy where there is a lot of drive, which is there from our operations team, and that helps bring in walk -ins.

And last but not the least, we have very strong doctors located across all our branches in these markets. And given the size and scale of our network, we're able to bring in that kind of clinical talent. And since you are able to deliver those kind of outcomes, we're able to see stupendous growth in these markets.

Param Vora: Okay. And the next question is can you share the unit economics differential between a Tier 1 - metro cities versus the Tier 2, Tier 3 facilities?

Adil Agarwal : Our Tier 2, Tier 3 facilities are pretty much -- in terms of unit economics are pretty much on par with our facilities in the metros. There's not too much of a difference.

Moderator: The next question is from the line of Viraj Shah from PGIM India Mutual Fund.

Viraj Shah: Just wanted to speak on the same-store sales. You mentioned 123 mature facilities grew at 13.4%, if I'm not -- if I heard it correctly.

Adil Agarwal : Correct.

Viraj Shah: So in Q1, there was a growth of 19.4%. So seems like in 2Q, it was significantly low. Any specific reason?

Yashwanth Venkat : Okay. See, Viraj, I just want to clarify first on the numbers. If you go to the slide on vintage performance for the Q1 up to FY '22, the revenue growth has been 15% and in FY '23, 13.7% and FY '24 23.3%. If you look at H1, up to FY '22 centers from 15%, it has come down to 13.9%.

FY '23 remained the same at close to 13.7% and FY '24 at 18.3%. Now to answer your specific question on mature facilities, here, I will just take the example of FY '22 facilities.

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See, some of our FY '22 facilities are located in our -- I mean, most of our facilities are located in our core markets. And in the month of August, we were hit by unseasonal rains for about a week to 10 days. So, there was a bit of a challenge as far as the month of August was concerned. So, that is the reason for a slight drop of about close to 1.5% to 2% in the quarter -on-quarter growth, Viraj.

Viraj Shah: Okay. So, can you give me the same-store sales growth for the second quarter for mature facilities higher than -- more than 3 years?

Rahul Agarwal : So, specifically for Q2, it will be closer to 12.5%. At a blended basis for H1, it's going up to around 13.5%.

Viraj Shah: Okay. Perfect. And the capex guidance remains the same at INR 310 crores for the year?

Yashwanth Venkat : Yes, INR310 crores, plus INR 70 crores for the flagship figures. It remains the same.

Moderator: The next question is from the line of Rachna Kukreja from Simpl PMS .

Rachna Kukreja : I have a few questions. Could you provide some color on the operating profit margin range for

the smaller acquired clinics within our network? Like we understand Dr. Thind clinics operates at 40% EBITDA and our listed subsidiary at 30% to 35% EBITDA. How do the margins of other units or entities compare within this spectrum? And what factors drive the variation in margin profile? This is my first question?

Adil Agarwal : No, see, are you asking how are our facilities -- what is the margin profile of our facilities at our acquired facilities? Is that the question?

Rachna Kukreja : The smaller acquired entities. Apart from Dr. Thind, our listed subsidiary and core Agarwal Health business, apart from that?

Adil Agarwal : Okay. So, some of our acquired facilities in many of the markets, which we have done over the last few years, they operate at a similar range as what our group level margins are. So, roughly around the 28% to 30% Ind AS EBITDA margins is what they operate at. There's not too much of a difference between them versus what the overall group level is at.

Rachna Kukreja : Okay. My second question would be...

Adil Agarwal : Just to add, some of the advantage to some of these acquired facilities, given they're located in Tier 2, Tier 3 markets, the rental costs are slightly lower compared to some of the metro cities.

So, we are able to actually get better operational efficiency in some of these centers.

Rachna Kukreja : Okay. Understood. My second question would be in Telangana, where we are among the largest hospital network, could you share on our growth trajectory and EBITDA margin range in that market? How does Telangana's performance compare with the similarly established markets like Punjab and Tamil Nadu in terms of both revenue growth and profitability?

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Adil Agarwal : Yes. So, Telangana has seen significant growth in this H1. So, we have grown -- our top line has grown by about 42% compared to last year. And there has been an EBITDA growth of about 44% last year. So, this has come through a combination of both same-store sales growth and by addition of 4 new facilities for the Telangana region, which has continued to be a very strong region for us.

Rachna Kukreja : Okay. My last question would be, as new and emerging facilities mature over 1 - to 3-year period, how do the operating margins typically evolve, especially in markets where we have entered via the Greenfield route? And what level of margin convergence is generally observed within the mature facilities?

Yashwanth Venkat : Sure. So in terms of margins for Greenfield facilities for secondary facilities, we can look at margins close to about 30% to 32% at a store level over a 5- year period.

Rachna Kukreja : Okay. So, how does that margin evolve like, if the emerging facility comes under a mature facility? So how does that margin improve, if you could provide more color on that?

Adil Agarwal : So we'll just -- I'll request Mr. Yashwanth to just give you the unit economics of how a typical secondary center of ours evolves, and that will probably give you a sense. What you should understand also is that the margin performance will vary between your existing markets where your brand is fairly strong versus some of the newer regions where we are entering. So, that margin profile will change, but he will give you an average in terms of how the unit economics are across different time periods. Yashwanth?

Yashwanth Venkat : Yes. Across core and non-core markets, if you take, typically, a secondary facility will breakeven between the 12 to 15 months. The reason I'm putting this number on a blended basis is in core markets, they tend to breakeven at a store level by the 6 or 7 month itself. And this margin by the end of the third year cost is about close to 20% to 22%. By the fourth year, this will be about close to 25% to 27%. And by fifth year, it is between 30% to 32%. That was what I was alluding to earlier.

lier.

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar Manudhane : Sir, given that we have done INR 136 crores EBITDA for 2Q with 27% EBITDA margin post IndAS and given that second half is expected to be better than the first half. So both in terms of absolute EBITDA, will we be able to do better compared to 2Q for, let's say, normalized too for the second half FY 26 and separately on the EBITDA margin, if you could highlight both these aspects?

Yashwanth Venkat : See, in terms of the overall business performance, generally, H2 will be slightly stronger than H1, Tushar. But in terms of the overall margins with the pace of expansion and with more Greenfields getting launched, the overall margins on a percentage basis should be on similar levels as to whatever we have reported on for H1, Tushar.

Tushar Manudhane : So which is around 26%, 26.5% at least ?

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Yashwanth Venkat : So, what margins you are talking about this, you are removing other income at least for competition.

Tushar Manudhane : Yes. I'm assuming.

Yashwanth Venkat : Yes it is similar to that.

Tushar Manudhane : All right, sir. This is helpful. Thank you.

Moderator: Thank you very much. On behalf of Dr. Agarwal's Health Care, that concludes this conference.

Thank you for joining us and you may now disconnect your lines. Thank you.

Call: Feb 2026

BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street,
Mumbai - 400 001 National Stock Exchange of India Limited
Exchange Plaza, Plot No C/1, G Block,
Bandra -Kurla Complex, Bandra (E),
Mumbai – 400 051

Scrip Code – 544350 Symbol : AGARWALEYE

Dear Sir / Madam ,

Sub: Analyst / Earnings Conference Call - Transcript

Ref: Regulation 30 read with Schedule III of the Securities and Exchange Board of India
(Listing Obligations and Disclosure Requirements) Regulations, 2015

Pursuant to the captioned subject and reference, Transcript of the discussion on the Unaudited Financial Results (Standalone and Consolidated) of the Company for the quarter and nine months ended December 31, 2025, at the Analyst / Earnings conference call held on February 04, 2026 , is enclosed herewith .

The said transcript is also available on the website of the Company at:
<https://dragarwals.co.in/dr-agarwals-health-care/#analyst-earnings-call>.

For Dr. Agarwal's Health Care Limited

Thanikainathan Arumugam
Company Secretary and Compliance Officer

Encl.: as above. Ref: AHCL/2025 -26/C0 69 February 10, 2026

"Dr. Agarwal 's Health Care Limited
Q3 & 9M Earnings Conference Call "
February 04 , 2026

MANAGEMENT : DR. ADIL AGARWAL – CHIEF EXECUTIVE OFFICER ,
AGARWAL'S HEALTH CARE LIMITED
DR. ASHAR AGARWAL – CHIEF BUSINESS OFFICER ,
AGARWAL'S HEALTH CARE LIMITED
MR. RAHUL AGARWAL – CHIEF OPERATING OFFICER ,
AGARWAL'S HEALTH CARE LIMITED
DR. VANDANA JAIN – CHIEF STRATEGY OFFICER ,
AGARWAL'S HEALTH CARE LIMITED
MR. YASHWANTH VENKAT – CHIEF FINANCIAL OFFICER .
AGARWAL'S HEALTH CARE LIMITED
MS. AASHNA DHARIA – HEAD, INVESTOR RELATIONS , DR.
AGARWAL'S HEALTH CARE LIMITED .

Dr. Agarwal 's Health Care Limited
February 04, 2026

Moderator: Ladies and Gentlemen , Good Day and Welcome to the Q3 & Nine Months -ended December 31st ,
2025, Earnings Conference Call of Dr. Agarwal 's Health Care Limited.
As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity

for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. I now hand the conference over to Ms. Aashna Dharia, Head , Investor Relations from Dr. Agarwal 's Health Care Limited. Thank you and over to you, Aashna.

Aashna Dharia: Thank you, Ryan. A very good morning, ladies and gentlemen. Welcome to Dr. Agarwal 's Health Care Q3 and Nine Months -ended December 31st , 2025, Earnings Call .

From the Management side, we have Dr. Adil Agarwal – Chief Executive Officer, Dr. Ashar Agarwal – Chief Business Officer, Mr. Rahul Agarwal – Chief Operating Officer, Dr. Vandana Jain – Chief Strategy Officer, and Mr. Yashwant h Venkat – Chief Financial Officer.

We have also released the Financial Results, Press Release and Investor Presentation , all of which are available on our website and the exchanges as well.

Before we continue, we want to remind everyone that this call is being recorded and the transcript will be made available on our website afterwards.

Additionally, please be aware, today 's discussion may include certain forward -looking statements which should be considered in the light of the risk our business faces. Please refer to the detailed statement on Page #2 of the investor presentation.

It is now my pleasure to hand over the call to Dr. Adil Agarwal, our Chief Executive Officer, who will Share his Opening Remarks and Insights . Dr. Adil, over to you.

Dr. Adil Agarwal: Thank you, A ashna and Ryan. A very good morning to all of you and a warm welcome to the Q3 FY26 Earnings Call of Dr. Agarwal 's Health Care Limited.

Let me begin by providing you all an update on the performance of the Company :

For the nine months -ended December 2025, the Company reported a total income of Rs .1,548 crores, up 20.8% year -on-year, while revenue from operations rose 21.2% to Rs1,516 crores.

We delivered a robust IndAS EBITDA of Rs .440 crores, reflecting a 23.6% year -on-year growth, with margins improving by 64 basis points to 28.4%.

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Our profit after tax grew 74.3% year -on-year to Rs .118 crores, with PAT margins expanding by 234 basis points to 7.6%.

Moving on to the Third Quarter Performance :

I am pleased to share that we delivered a robust and well -rounded quarter, reflecting strong execution across our network.

For the 3rd Quarter , the Company reported a total income of Rs .540 crores, up 21.9% year -on-year, while revenue from operations rose 23% to Rs .530 crores.

Q3 also delivered a robust IndAS EBITDA of Rs .155 crores, reflecting a 21.3% year -on-year growth, with margins of 28.4% for Q3.

Profit after tax grew 55% to Rs .44 crores, while PAT margins expanded by 171 basis points to 8.1% for Q3.

Next, I would like to share our Footprint and Network Growth Update :

During the nine months -ended December 2025, we served over 22 lakh patients and performed nearly 2,38,283 surgeries. Every day, nearly 10,000 patients now walk into our facilities across our network, up from about 8,000 patients in the previous financial year. This represents a 25% growth in walk -ins. This consistent growth in footfall reflects not just the scale of our operations, but also the strength of our brand recall and our ability to deliver quality eye care close to where patients live.

During the year, we continued to invest in the expansion and stabilization of our newly launched facilities. The ramp -up across recently launched facilities has been

faster than earlier phases,

supported by strengthening brand equity, reinforcing our confidence in scaling expansion more aggressively , while continuing to deliver superior clinical outcomes.

In India now, we have a total network of 253 facilities across 14 states and five union territories, covering 148 cities.

Our presence is well diversified now, with 31% of our facilities in Tier-1 markets, 62% in other markets, and 7% located internationally.

We expanded our footprint by commissioning 14 new Greenfield facilities this quarter, further strengthening our reach and capacity. These included nine secondary centers, including one in Tamil Nadu in Tirupat tur, Kolhapur in Maharashtra, Kanakpura Road in Bangalore, and Raichur in Karnataka.

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We also launched a project in Haryana in Gurgaon, Chand kheda and Vadodara in Gujarat, Preet Vihar in Delhi, and Ongole in Andhra Pradesh, along with five primary centers across Tamil Nadu and Odisha.

Let me now turn to an Update on our Clinical Excellence Initiatives and our Growth in Complex Surgeries :

For the nine-month period ending December 2025, high -end cataract surgeries accounted for 43.5% of total cataract procedures , 45,459 surgeries were done in total, an increase of 43.5% over the same period last year. Within the high -end surgeries, robotic cataract surgeries, what we call as the Femto cataract , grew by a robust 83% year -on-year, rising from 2,616 to 4,400 procedures.

As shared in our previous earning calls, new robotic cataract systems have now been installed at our Velachery facility in Chennai and our Delhi facility, which is now performing close to 60 such surgeries on a monthly basis on average.

Additionally, we are strengthening our advanced surgical capabilities with the addition of robotic systems in Gurgaon and in Vashi.

Lenticular procedures, what we call as the SMILE surgeries for refractive, increased 17.7% year -on-year from 4,223 to 4,970 surgeries, while retinal surgeries totaled 9 ,437 (mispronounced during the call as 940) surgeries, up 23.2% from last year.

We also did a total of 792 corneal transplants over this entire period.

We continue to invest in cutting -edge technology to enhance diagnostic accuracy and surgical outcomes. Recent additions include the CATALYS and ELITA S ystems at our Gurgaon center, as well as the LUMERA 300 and ORTELI FAROS at our Preet Vihar center.

On the Research and Capability Building Front, our clinicians have contributed to over 340 publications in the leading international medical journals over the past three decades, underscoring our commitment to advancing ophthalmic science.

During the quarter, nearly 50 doctors underwent advanced training across multiple specialties as part of our continuous learning and development initiatives.

Now, moving on to Some Business Updates :

Let me first begin with our Region -wise Performance :

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Our Southern region continues to be our largest market, contributing to 63% of our total group revenues. This region delivered Rs.950 crores of revenue, representing a strong growth of 22.4% year-on-year. We have 172 facilities located across our Southern states, and this includes 12 new additions in this year, with four new facilities being added in Tamil Nadu, five in Karnataka, two in Andhra, and one in Kerala. We remain focused on sustaining our market leadership in both Tamil Nadu and Telangana, while further incr easing adoption of the latest technologies in these regions. In Karnataka, Andhra Pradesh, and Kerala, we are strengthening our network presence in both these markets by expanding significantly into high -potential underserved markets through strategic facility opening.

Now, coming to the West Region :

This region contributes to about 16% of our overall group revenues and delivered Rs.244

crores in

revenue, up by 18.4% year -on-year, despite some impact of the festive season during the quarter. We currently have 46 facilities in the west region, including six new facilities which are open ed in these nine months.

Maharashtra, as we have highlighted earlier, continues to be a key region for us, as we aim to deepen our presence in the underserved regions of the state, while further expanding into the micro -markets of our metro cities such as Mumbai and Pune.

In Gujarat, we remain optimistic given the strong performance of our new centers in Surat. We also commissioned a secondary facility in Chandkhe da in Ahmedabad, and also launched a facility in Vadodara . We plan to further strengthen our footprint in the state by adding more facilities and accelerating adoption of advanced technologies across the region.

Now, coming to the North Region :

This contributed to 8.3% of our group revenues, and this region reported Rs.126 crores of revenue this quarter, up by 19.7% year -on-year. We currently operate 24 facilities in the entire north region.

Continuing from our Previous Update on our entry into the Delhi NCR region :

Operations have commenced on a strong note, with strong traction in the patient volumes and steady month -on-month revenue growth.

Building on the success of our Delhi main flagship facility, we launched a secondary facility in Gurgaon in November 2025. December marked its full month of operations, during which it witnessed strong patient traction.

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We have also commissioned facilities in Pre et Vihar and Rajouri Garden, which is currently in the early ramp -up phase, with focused efforts underway to build local market awareness and progress on key accreditation milestones.

Overall, performance remains strong across regions, with all markets delivering healthy double -digit year-on-year growth.

Now, moving on to our Vintage Performance of this year:

Facilities operational prior to FY22 contributed to about Rs.1,025 crores (mispronounced during the call as 1,125 crores), registering a year -on-year growth of 14.2%, underpinning our strong same - store sales growth performance.

Facilities opened in FY23 generated Rs.187 crores, growing at 14.3%, while those launched in FY24 delivered Rs.124 crores in revenue, achieving a strong growth of 18%.

Moving on to our Next Business Update :

Our planned facility openings for each quarter of the current year. Up to December 2025, we strengthened our network by adding 38 new facilities, out of which 23 were surgical centers. Over the next quarter of this financial year, we plan to launch another 16 centers, including five in the South, five in the West, and six in the North. Out of the total planned additions, 11 facilities are expected to be the surgical centers.

So, to conclude, our sustained focus on operational efficiency and disciplined execution has driven a strong performance across the first three quarters, and we are well -positioned to meet the guidance committed at the beginning of the fiscal year.

Now, I would like to hand it over to our CFO, Mr. Yashwanth Venkat, who will take a Deeper Dive into our Financial Performance .

Yashwanth Venkat: Thank you, Dr. Adil.

I will begin with the Operational Update :

Surgical services continue to be the main revenue driver, contributing 67% to the group revenue.

Diagnosis, consultations, and other non -surgical treatments contributed 11.6%, and the sale of optical products and pharmacy items accounted for 21.5%.

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For year -to-date December 2025, we performed 2 ,38,283 surgeries, marking a 11.6% year -on-year growth.

Cataract surgeries remained the largest contributor, accounting for approximately 72.7% of total surgeries, followed by refractive surgeries at around 5%.

Volumes for cataract and refractive surgeries grew 9.6%

year -on-year, while other surgeries on a blended basis recorded a growth of 18.9%.

Our payer mix for YTD December 2025 stood at 62.4% from cash, 28.5% from insurance and TPA, and 9.1% from government schemes.

The domestic payer mix stood at 70.9% from cash, 22.9% from insurance and TPA, and 6.3% from government schemes.

Moving on to the Financial Section :

I will start with the revenue split. The group 's revenue from operations grew by 23% year -on-year, reaching Rs.530 crores in Q3 FY26 compared to Rs.431 crores in Q3 FY25.

Revenue from operations in India stood at Rs.480 crores, reflecting a growth of 23.1%, despite unseasonal rains in our core markets and impact on the business due to festivals. This growth was supported by near -equal contributions from both volume and value of close to 13%, with the remaining contribution coming from new centres opened in FY25 and FY26.

Gross margin remained stable year -on-year and decreased slightly quarter -on-quarter, despite a higher contribution of surgeries in segment mix, with high -end cataract procedures rising to 26.6% in YTD December 2025 when compared to FY25 of 24.6%.

Doctor and employee costs cumulatively have increased by 140 basis points to 33.4% of total revenues for Q3 FY26 as compared to Q3 FY25.

I would also like to briefly address the Impact of the New Labor Codes :

We have undertaken a detailed evaluation of the labor co des across the group based on the information currently available and the provisions notified so far. Our existing salary structures and employee benefit policies are already broadly aligned with the requirements of the labor co des and the incremental impact for us is not material currently . We will continue to monitor the finalization of central and state rules and any further clarifications from the government and we will make appropriate adjustments if required.

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Other expenses as a percentage of total revenue have reduced to 15.5% of total revenues. A total of INR95 crores in loans have been repaid from IPO proceeds, INR128 crores in Q4 FY25 and INR67 crores in YTD December 2025, leading to lower finance costs and higher profitability versus the same quarter last year.

From 75.1%, the share of profit after tax attributable to the owners has expanded to 79.1% in YTD December 2025, signaling improved profitability in the holding Company .

Thank you all. We will open the floor to Questions .

Moderator: Ladies and gentlemen, we will now begin the question -and-answer session. We take the first question from the line of Binay Singh from Morgan Stanley. Please go ahead.

Binay Singh: Hi, team, congratulations on another steady quarter. Two questions from my side. When we look at the surgery mix for the quarter, taking from the YTD numbers, we know that the cataract surgery growth has slowed down to around 4%, but at the same time, the refractive surgery growth has risen quite sharply. So, could you comment on these trends? And also, typically, we see refractive, March quarter tends to be the biggest, but this time your December itself is very strong. So, how should we see that trend going ahead? So, that is the first question. Thanks.

Dr. Adil Agarwal: Hi, Binay, thanks for the question. I will just request our COO Mr. Rahul Agarwal to just take this.

Rahul Agarwal: So, Binay, hi. Actually, cataract has been quite strong this quarter; we have grown by almost 18.5%. So, as I was saying, our cataract for this quarter has been stronger at 18.5% growth. In fact, refractive has been slower for us this quarter and largely for this year. It is more to do with the overall industry being slightly slow on the refractive side this year. Hopefully, we will bounce back on refractive next year. That is how we are seeing it. This quarter, I think from a cataract perspective, which is the bulk of business that is coming in,

has been fairly strong.

Binay Singh: So, just to clarify, you are saying the number of surgeries are up 18%?

Rahul Agarwal: The value is up 18% and the number is up 13.7% on cataracts.

Binay Singh: Okay. I will separately take this and share. And similarly, when I do the emerging facility revenue growth for this quarter, therein also we see that the mature facility revenue growth has picked up, whereas the emerging facility revenue growth has slowed down. So, again, this is subtracting the nine-months versus last presentation data.

Yashwanth Venkat: Binay, Yashwanth here. Just a small clarification. See, if you go to the Investor Deck on Slide # 13, it is better to look at the revenue growth on a vintage-wise basis, Binay, because the mature facility

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definition is a rolling definition. So, it will not be appropriate to calculate. So, if you look at up to FY22 centers on a YTD basis, it is close to about 14.2%. For the half year, it was close to around 13.5% to 13.8%. So, similarly, other vintages also, if you can calculate, that would give you the right picture, Binay.

Binay Singh: Right. Thanks, team. And just lastly, if you could share the CAPEX number for the year, how much have you done YTD? And also, what was Thin d's contribution this quarter? That is it. Thanks.

Yashwanth Venkat: So, on CAPEX, we have done about close to Rs.275 crores, Binay, out of the Rs.310 crores which we had committed. And on the spends in the subsidiary for the Cathedral Road facility, we have done close to about Rs.35 crores out of Rs.70 crores which were committed, Binay.

Binay Singh: Okay. Thank you.

Moderator: We take the next question from the line of Ananya Khanna from Alpha Alternatives. Please go ahead.

Ananya Khanna: Yes. Hello, sir. First of all, congratulations on a steady quarter. Secondly, my question to you is, you mentioned you commissioned a few new facilities, right? So, I want to understand when those are supposed to sort of become operational and when are you expecting for them to breakeven?

Rahul Agarwal: I think Dr. Adil had mentioned about commissioning of a couple of facilities in Delhi, one in Pr eet Vihar, which got launched at the end of December, and one in Rajouri Garden, which has already got launched in January. See, in terms of break-even, currently, it will be difficult for us to comment on Delhi facilities. However, we expect the facilities to broadly breakeven around the 15 to 18 months kind of a number. That is a very broad estimate.

Ananya Khanna: All right, sir. But you also mentioned a few other facilities apart from the ones in Delhi. I am talking about the primary health care centers, etc. So, what about those?

Rahul Agarwal: Okay. So, we have totally launched about 23 surgical centers and about 14 primary centers. Out of these 23 centers, I will just give you the exact break-up in a second. So, we launched four new facilities in Tamil Nadu, five new facilities in Karnataka, two in Andhra, and one in Kerala. If you look at the western region, we have also launched two new centers, one in Vadodara and one in Chandkheda, which is in Gujarat, and we launched one in Kolhapur and one in Aurangabad in Maharashtra. The unit metrics of our centers which have been launched in our core markets are pretty much meeting the estimates in terms of what we have previously guided. The centers which have been opened in some of these non-core markets, as Yashwanth has just explained, will meet breakeven in about 12 to 15 months. That is the current run rate of these centers.

Ananya Khanna: All right. Thank you.

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Moderator: We take the next question from the line of Sucrit D. Patil from Eye sight Finance Private Limited. Please go ahead.

Sucrit D. Patil: Good afternoon to the team. I have two questions.

The first question to Mr. Adil is, taking a slightly

forward-looking lens, as the network continues to scale, how does the management balance capacity expansion, doctor availability, and case mix to protect returns on capital? What are the operating

signals typically that tell you whether to accelerate expansion in a cluster or pause and optimize the current existing centers that you have? That is my first question. I will ask my second question after this.

Dr. Adil Agarwal: So, the first important metric for you to look at is despite the scale of our expansion, so, like I had mentioned, we have launched about 52 facilities in the last 12 months. In these last nine months alone, we have added about 37 new facilities. The goal is to end up at about 52 to 55 facilities by the end of this year. One of the healthiest signs which we believe of us expanding the right way and us being able to scale up our operations in the right manner is, if you see our EBITDA margins, we have been able to maintain them at a 28.5%. So, despite us being able to scale up these many new centers, we have been able to maintain our margins, which basically means that we are ramping up our new centers a lot faster to breakeven. That is the first sign. A second very important sign is our same-store sales growth. That is a very, very important metric that we track. And like Rahul had mentioned, our same-store sales growth is north of about 13.5%, which is again a very, very healthy sign. One of these signs lead to the fact that we are able to generate significantly healthy cash flows, which we are actually able to deploy towards ramping up more centers. So, I would say these two are very, very important signs for us, which we look at. How are you able to scale up our new centers, at the same time, what is the kind of same-store sales growth we are able to maintain? If we are able to do this significantly, and we are actually able to better your margins, or at least maintain your margin, which you will start to see the return on capital improving over the next two to three years, which is what we have already demonstrated over the last nine months.

Sucrit D. Patil: My second question is to Mr. Venkat, is, from a financial monitoring point of view. Beyond the reported margins, what internal unit level metrics do you track most closely to assess, whether the pricing power, utilization, or cost efficiency is trending according to the way that you have planned, or it is starting to weaken across the board? I just want to understand your view on this.

Yashwanth Venkat: See, every center, in terms of the performance, is tracked at the branch EBITDA level. The branch operating margin of every center is tracked on a monthly basis wherein we go about looking at projection for the particular year. First, when we start the center, we have a five-year projection, and we also track it on a monthly basis. Here, if the center starts to, say, not breakeven within the stipulated particular time for that particular market, then we bring in a few interventions in terms of both the revenue ramp-up, as well as we also look at unit level costs, such as our doctor costs,

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employee costs, and also what we spend on marketing and a few other administrative expenses as well, which are in our control.

Sucrit D. Patil: Thank you and best of luck for the next quarter.

Dr. Adil Agarwal: Thank you, Sucrit.

Moderator: We take the next question from the line of Nikhil from SIMPL. Please go ahead.

Nikhil: Yes, hello! Thanks for the opportunity, and congrats on good set of numbers. So, my question was, if I look at our revenue per facility over like the last nine months, across regions, we have seen that there is a drop in revenue per facility, while our surgical mix has improved. So, can you just help us understand why the revenue per facility

is dropping across the regions? How does this play out

over a period of time, like over last five, 10 years, how would these numbers play out?

Rahul Agarwal: Hi, Nikhil. A small correction. See, when you are calculating the revenue per facility, you have to break it down into surgical as well as primary. For example, I think you are using the Slide # 12 on the investor deck while calculating the revenue per facility. So, in terms of the actual revenue per surgical facility has grown at a healthy CAGR of about 14% to 15% over the last four years. If you get into Slide # 13, we have given the breakup of both the mature and emerging facilities, both the surgical as well as the non-surgical. So, in terms of the non-surgical facilities, they contribute about close to 1.5% to 2% of the overall group revenue. So, if you remove that from your working, then you can see that there is a clear track of increase in revenue per surgical facility, Nikhil.

Nikhil: Okay. So, where I am coming from is like, if I look at only East as a region and consider it based on the revenue which we have mentioned and divide by the number of facilities, the average for last year was around Rs.2 crores, which today, like based on the rough calculation comes to Rs.1.57, 1.6 crores, while our surgical, like the number of patients and the surgery mix and everything has improved, and East is also a geography, which is the number of facilities are less. So, is it like when we enter these new geographies, we have to play the pricing game in order to get the footfalls? I am just trying to understand from how do we develop these new geography perspectives?

Rahul Agarwal: Okay, Nikhil. I will answer your question. In East, currently, I will just take Odisha as a market. We have three facilities. So, we launched the third surgical facility just last year. It has not even completed one year of full operations. So, generally, when the facility is launched, it also takes, say, about close to three years to reach a certain level of maturity. So, the couple of other facilities in Odisha have continued to grow in the mid-teens kind of a number where there is a significant amount of maturity for those facilities as well. And one more point is, when the facility gets launched, some of them would not have completed one whole year of operations as well. So, when you calculate the

data by looking at whatever we have presented in the investor deck, there is a slight skew to that as well.

Nikhil: Okay. Second question is, see, if I try to remove, like, from the consol, the standalone numbers and try to see the revenue from the subsidiaries come to around Rs.80 crores for this quarter. And here, I think, largely, it is Thind, which is the major revenue contributor. And if I do a P&L subtraction, I get, like, for the last nine months, the subsidiaries are operating at almost 37 %-38%, where, again, Thind is the major contributor. Now, what I am trying to understand is that I know Thind is the established chain in Punjab , and if I draw comparables to Tamil Nadu, which is Dr. Eye which is the other subsidiary, even that does not operate at such high kind of a margin. So, what is the differential that we are able to operate at such high margins in a specific entity like Thind, while even in our markets like Tamil Nadu, we are operating at 28 -30%, so, what brings this differential? And can we close these differentials over a period?

Rahul Agarwal: See, Thind is a unique case, where bulk of the revenue comes from a single facility based out of Jalandhar. There are no further major costs as far as the regional resources and corporate resources are concerned. As I was explaining, Thind , the majority of the revenue comes from a single facility, and there are no costs as far as the regional resources are concerned, and very minimal costs a

s far as

corporate is concerned. Our subsidiary, AEHL, which is predominantly consists of mature facilities and operating out of Tamil Nadu, there also the margins are close to about 33% as far as the corporate margins are concerned. When you also add back your regional and corporate costs, those margins would be slightly more comparable to what the current margins are at the TECPL.

Nikhil: Okay . See, based on the brand equity that we have in Thind , my idea would have been and I could be wrong here, since Punjab and Haryana have always been sister states, there would have been a patient flow from nearby states as well. So , have we not expanded in Punjab beyond when we had acquired this entity, or how many new facilities we have opened based on brand equity , if you can share what have we done with this, beyond what it was here ?

Dr. Adil Agarwal: I will request Dr. Ashar Agarwal to come in. He is our Chief Business Officer, and he is heading the entire expansion piece. He will give you an update in terms of what is happening.

Dr. Ashar Agarwal: Hi, Nikhil. Nikhil, basically we are working right now on the three new facilities under the Thind banner. Ludhiana is under work right now, so it should launch soon, but the other two centers are a little more futuristic. So, there is the expansion play coming into Thind as well. And Haryana is already underway, Gurgaon is already launched, but that is under Dr. Agarwal 's banner.

Moderator: We take the next question from the line of Tushar Manudhane from Motilal Oswal Financial Services Limited. Please go ahead.

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Tushar Manudhane: Yes, thanks for the opportunity, sir. So, just coming back on the surgery revenue growth, while the revenue growth has been very healthy for the quarter, but number of surgeries, growth rate has been a little soft. If I put that to let 38,000 surgeries into different categories, both overall number of surgeries as well as if I have to break that into cataract and refractive, if you could just explain that first?

Rahul Agarwal: Okay. Hi , Tushar, this is Rahul. From an overall surgery perspective, if you see while the surgery growth is at 11.6%, we are also getting a lot of premiumization which is happening. So, you will have to see the overall growth from both the number and the value perspective. One of the things that Adil also mentioned initially was that we are doing a lot of technology upgradations in a lot of our centers, both on refractive as well as on cataract . In cataract, we have added quite a few centers with our catalyst machines and dimer machines, which are the ones which do Femto cataract for us or robotic cataract as the market knows it as. So, those are very, very value accretive . And as the numbers go up there, that is where we are seeing a huge impact coming in, into our premiumization play. Just to give you an example, only the Femto cataract numbers that we had achieved last quarter was to the tune of almost 1 ,800 surgeries. If I just look at our high -end surgeries, it is a mix of almost 13% which are getting converted to Femto cataract . So, that is something which is adding a huge value to us and helping us do our overall surgical value growth also. So, it is not just the volumes, but it is the total value which you should look at.

Tushar Manudhane: That is interesting. So, if I have to, let us say, put that as a specialized surgical procedure, maybe Femto cataract , lenticular or retinal surgery , if I have to club all this, it is not an individual segment, but a specialized surgical procedure by revenue, how much that would be contributing to our surgical revenues if that number is handy?

Rahul Agarwal: So, just if I have to give you high -end cataract numbers, that is for the last quarter, we are at closer to 28%. And out of that, cataract I mentioned is 1 ,800 surgeries, which i

s 13% of the overall

high-end cataract numbers.

Tushar Manudhane: 28%, sorry -

Rahul Agarwal: 28% is the high -end cataract percentage of the overall cataract numbers.

Tushar Manudhane: As per value or by revenue?

Dr. Adil Agarwal: In terms of overall value, which is the total income coming in from overall cataract surgeries, but 28% comes in from high -end cataract surgeries, is what Rahul is saying.

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Tushar Manudhane: Got it. And s ir, secondly, this formation of this Ethiop ia subsidiary, what are the sort of measures over there, if you could throw some light over there?

Dr. Adil Agarwal: Ethiopia is a high -potential market, which we are looking at entering right now. We are in the process of looking at assets where we can actually start our facility. It has got a steady patient base, and they have a lot of respect for our Indian doctors, which is why we believe that is a significantly potential market for us.

Yashwanth Venkat: I just add here . Ethiopia, one is, there is no cash going from India. It is completely funded by the Orbit entity. Second is, operationally, it is an easier location for us to handle because it is a neighboring state to Kenya, and we have a very strong team in place in Kenya. Plus, Ethiopia is one of the growing markets in entire Africa. So, the potential opportunity that exists today with the lack of services, with the number of people coming into Addis Ababa is lucrative according to us right now.

Tushar Manudhane: Whether it goes or do not go from India, but at a consol level, if you can share some ballpark number in terms of any investment amount made in terms of building the facility or any inorganic opportunities you are thinking o ut there, any ballpark number that you have earmarked for this?

Dr. Adil Agarwal: Tushar , opportunity is purely going to be organic only. But , once we have a few more details, we will come back to you in terms of what is the estimated CAPEX s pend which we are looking at. Right now, what we are doing is , we are doing a feasibility study of the market . And what we have seen right now, we like that the overall size of the eye care market in Ethiopia is quite large. Second, it has a very high cash revenue mix, which is something which we like a s well a nd there is a lot of respect for Indian doctors , we believe there is a good potential for us to scale the business. But we have not reached a stage where we have estimated the CAPEX spend and projection. Next time when we come back, we will come back to you with more details on CAPEX sp end estimated and what are the projections going to look like.

Tushar Manudhane: Thanks a lot. Very helpful.

Dr. Adil Agarwal: Thank you.

Moderator: We take the next question from the line of Prat eek from Bandhan AMC. Please go ahead.

Prateek: Yes, hi. So, just one small question. Could you just let me know the Greenfield losses for the quarter and for the first nine months, please?

Dr. Adil Agarwal: Greenfield losses. Ye s, so overall, at a regional level, our total losses are Rs.28.5 crores as total Greenfield losses, which includes branches launched over the last three years what we consider as

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emerging centers. So, over the three years, everything put together cumulative is about Rs.28.57 crores. And this is something which we have been saying that despite some of the early Greenfield losses which we face d, we have still been able to maintain our margins, which is a healthy sign in terms of how we are ramp ing our centers. Most of these losses will come from the centers which have opened in the last nine months. FY24 and FY25 vintage centers have already started to become profitable. So, losses coming in from those centers is much lesser.

Yashwanth Venkat: I will just add here one point. This is including all the centers th

at are loss -making. When you go into

cohort analysis, FY25 centers at a branch level, cohort is not loss -making. The entire cohort is positive. It is not loss -making at all. What Dr. Adil just mentioned was all the cumulative centers of only loss -making centers.

Prateek: Okay. And the corresponding revenue to this and this first nine months, right , that is a correct understanding, right?

Yashwanth Venkat: This is what Dr. Adil has mentioned is including FY25 and FY26 all loss -making centers.

Prateek: No, no, I am saying for this for the first nine months, right , the Rs.28 crores ?

Yashwanth Venkat: Yes, for the first nine months , correct. Also includes pre-operating losses as well.

Prateek: And the revenue corresponding to this on the Slide # 30, is it Rs.300 crores? I just did 20%.

Yashwanth Venkat: You are talking about the revenue coming in from those particular centers. It is basically FY24, FY25, and FY26. We have already shared the breakup.

Prateek: No, no. My question was, let us say the Rs.28 crores loss would be on a revenue base, right? That revenue base is 300 crores for the first nine months, just to get a like -to-like comparison.

Dr. Adil Agarwal: No, that is not how you look at it. When you are looking at the revenue base, it includes your profitable centers as well. Only for the loss -making centers -

Prateek: Yes, I understood. Could we just call out that how much is the revenue from these Rs.28 crores of loss? What is the corresponding revenue of those?

Dr. Adil Agarwal: We do not have that exact number right now. On a separate call, we can share some of those numbers with you.

Prateek: Surely, sir. That was really helpful. Thank you, sir and all the best.

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Moderator: We take the next question from the line of Kartick Bane from Bajaj Life Insurance. Please go ahead.

Kartick Bane: Thank you for the opportunity. I would like to know more about the same -store growth, which is of 13%. How is split there between price growth and the patient volumes?

Yashwanth Venkat: Sure. So, I can give you an overall perspective on what the breakup looks like. On the overall 13%, we have a volume growth, which is half of it, close to 6.5% and the value growth, which is 6.5% . In this 6.5% of value growth, we have both price hike and premiumization. Large part of the value growth is coming from premiumization, which I already spoke about ; almost close to 5% has come in from premiumization. Even in volume growth, I would break it down into two parts --one is the basic opening growth, which is closer to the 4%, 4.5% range, which we get on a year -on-year basis , and the remaining is what we do with the same walk -ins, which comes in, where we are able to get better conversion rate on the same patients. As the facilities start maturing and confidence and the trust starts growing higher, our conversion also on the same walk -in numbers starts increasing higher. So, that is another 2%. So, that's broadly how the 13% breakup comes in.

Kartick Bane: Okay. And second question is on the robotic surgery. So , firstly, are these robots on the lease -and-operate basis, or have we already bought the robots? And secondly, 60 surgeries that you mentioned per month per robot , is this the highest capacity, or is there a scope for further improvement in the capacity utilization for robots?

Dr. Adil Agarwal: So, the 60 robotic cataract surgeries, the number we mentioned was what we have started to do from our Delhi facility. It is not overall. Some of high -volume centers do a number which is much higher than that as well. So, overall, we have done about 4,400 robotic surgeries across the group, which signifies approximately 83% growth over the last year. All the machines which we have are all fully owned by the Company and they are not leased out, just to clarify. Most of our hubs ac

ross major

cities now have a robotic cataract surgery machine.

Kartick Bane: Okay. Thank you.

Moderator: We take the next question from the line of Dishant Jain from Quasar Capital. Please go ahead.

Dishant Jain: Hello ! Thank you for the opportunity. One question is on the subsidiary financial. There has been a sequential growth on the employee side. So , any reasons particularly for that?

Dr. Adil Agarwal: So, we are talking about the subsidiary , AEHL. So, that is a regular part of the business where as we continue to expand our business, we are continuing to add more and more employees to our network.

We have also added a few primary centers. And as we continue to strengthen our business, we continue to keep adding employees, nothing out of the ordinary in AEHL.

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Dishant Jain: So, it is a business as usual ?

Dr. Adil Agarwal: Yes, it is a business as usual.

Dishant Jain: And sir, is it possible to give the operating cash flow numbers for nine months for holding as well as subsidiary?

Yashwanth Venkat: See, operating cash flow is close to about 80%, which has been our average for the last three years.

Dishant Jain: Okay. And , sir, two more questions on the merger update. One, on the facility in the subsidiary Company , the large facility , what is the update on it? And another one was on merger update , what is the progress over there?

Dr. Adil Agarwal: So, quickly on the merger update , right now, we have filed for a no -objection certificate from the stock exchanges. We expect receiving this no -objection certificate from the stock exchanges very shortly. Following this, we will proceed to the National Company Law Tribunal (NCLT) to convene meetings for our shareholders and creditors. We expect these meetings to occur around two to three months post the receipt of the NOC from stock exchanges.

Dishant Jain: Okay. And on the facility that is going to come in a subsidiary Company , when are we going to start it?

Rahul Agarwal: So, I think we expect the entire merger process to be completed by Q3, Q4 of 2027.

Dishant Jain: Q3, Q4 of 2027. And lastly on the update on the facility in the subsidiary Company that is going to come ?

Rahul Agarwal: So, the estimate on that facility is we are expecting to complete everything and get all our licenses and approvals by Q2 of FY2027. So, hopefully by sometime end of Q2 we should be able to get it.

Dishant Jain: End of Q2 ? Okay. Those are my questions. Thank you. Thanks for the opportunity.

Moderator: We take the next question from the line of Amar Ahil from Radian Capital. Please go ahead.

Amar Ahil: Sorry, I might have missed out if you mentioned this. What I wanted to ask is , how many new

facilities are you going to add going forward?

Dr. Adil Agarwal: So, the guidance which we have given is we are looking at adding anywhere between 55 -to-60 facilities every year.

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Dishant Jain: Okay, 55 to 60 facilities every year, right?

Dr. Adil Agarwal: Yes, roughly the math is we are looking at increasing our network size by about 20% every year.

Dishant Jain: Okay . 20% every year ? And it gets a breakeven in 15 -to-18 months, right?

Dr. Adil Agarwal: No, I just wanted to clarify what we mean by breakeven.

Rahul Agarwal: See, here if we consider our core markets of Tamil Nadu, Andhra, Telangana, Karnataka, and now Maharashtra has also become our core market. The facilities breakeven at a store level within six to seven months. What I was earlier speaking about is in the ne wer regions, where it is about close to 15-to-18 months. So, on a blended basis, all our facilities will breakeven within the 12th month kind of a mark.

Dishant Jain: Okay . And what sort of a CAPEX -

Moderator: I do apologize to interrupt you, but your audio is not clear. Could you please repeat

your question?

Dishant Jain: What sort of CAPEX it might require for the 50 to 60 additions every year?

Dr. Adil Agarwal: For the surgical secondary facility, the CAPEX ranges between Rs.5.5 to 6 crores , for tertiary facility, generally the CAPEX ranges close to Rs.11 to 12 crores , and for a primary facility, the CAPEX is close to about 35 lakhs.

Dishant Jain: Okay , got it. And what would be the ratio of primary, secondary, and tertiary in terms of additions?

Yashwanth Venkat: See, the way you want to think of it is these are not decisions that we plan one year before. We understand the market and then see what the market requires. So , if you see a usual split, will be about 75% will be surgical, 25% will be primary centers. But, in the surgical, the secondary and the tertiary is not something we decide that much prior. I hope that is clear.

Dishant Jain: I am so sorry. I just could not hear the last line you mentioned. Can you please repeat?

Yashwanth Venkat: Between the surgical facilities, we do not plan which one should be a tertiary or a secondary so much prior to the opening. We understand the market. At the time of understanding the market, we understand what is required for the market, and then we decide whether it has to be a tertiary or a secondary facility. For example, for Delhi, it is a tertiary facility. But for Kolhapur, it is not a tertiary facility. But those decisions are made during market understanding.

Dishant Jain: Okay, s ir, got it. That is from my side. Thank you so much, sir.

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Moderator: Thank you. Ladies and gentlemen, we take that as the last question and conclude the question -and-answer session. On behalf of Dr. Agarwal 's Health Care Limited, that concludes this conference.

Thank you for joining us and you may now disconnect your lines.

Call: May 2026

BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street,
Mumbai - 400 001 National Stock Exchange of India Limited
Exchange Plaza, Plot No C/1, G Block,
Bandra -Kurla Complex, Bandra (E),
Mumbai – 400 051

Scrip Code – 544350 Symbol : AGARWALEYE

Dear Sir / Madam ,

Sub: Analyst / Earnings Conference Call - Transcript

Ref: Regulation 30 read with Schedule III of the Securities and Exchange Board of India
(Listing Obligations and Disclosure Requirements) Regulations, 2015

Pursuant to the captioned subject and reference, Transcript of the discussion on the Audited Financial Results (Standalone and Consolidated) of the Company for the quarter and year ended March 31, 2026, at the Analyst / Earnings conference call held on May 21, 2026, is enclosed herewith .

The said transcript is also made available on the website of the Company at:
<https://dragarwals.co.in/dr-agarwals-health-care/#analyst-earnings-call>.

For Dr. Agarwal's Health Care Limited

Thanikainathan Arumugam
Company Secretary and Compliance Officer

Encl.: as above. Ref: AHCL/202 6-27/C013 May 27, 2026
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"Dr. Agarwal 's Healthcare Limited
Q4 FY26 Earnings Conference Call"
May 21, 2026

MANAGEMENT : DR. ADIL AGARWAL – CHIEF EXECUTIVE OFFICER –
DR. AGARWAL 'S HEALTHCARE LIMITED
DR. ASHAR AGARWAL– CHIEF BUSINESS OFFICER –
DR. AGARWAL 'S HEALTHCARE LIMITED
MR. RAHUL AGARWAL– CHIEF OPERATING OFFICER –
DR. AGARWAL 'S HEALTHCARE LIMITED
DR. VANDANA JAIN– CHIEF STRATEGY OFFICER – DR.
AGARWAL 'S HEALTHCARE LIMITED
MR. YASHWANTH VENKAT – CHIEF FINANCIAL
OFFICER – DR. AGARWAL 'S HEALTHCARE LIMITED
MS. AASHNA DHARIA – HEAD OF INVESTOR
RELATIONS -- DR. AGARWAL 'S HEALTHCARE LIMITED

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Moderator: Ladies and gentlemen, good day, and welcome to the Q4 and FY '26 Earnings Conference Call hosted by Dr. Agarwal's Healthcare Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference, please signal an operator by pressing star t hen zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Aashna Dharia, Head, Investor Relations from Dr. Agarwal's Healthcare. Thank you, and over to you, ma'am.

Aashna Dharia : Good evening, ladies and gentlemen. Welcome to Dr. Agarwal's Healthcare Q4 and Full Year Ended March 31, 2026 Earnings Call. From the management side, we have Dr. Adil Agarwal, Chief Executive Officer; Dr. Ashar Agarwal, Chief Business Officer; Mr. Rahul A garwal, Chief Operating Officer; Dr. Vandana Jain, Chief Strategy Officer; and Mr. Yashwanth Venkat, Chief Financial Officer. We've released the financial results, press release and investor presentation, all of which are available on our we bsite and the exchanges as well.

Before we continue, we want to remind everyone that this call is being recorded, and the transcript will be made available on our website afterwards. Additionally, please be aware today's discussion may include certain forward-looking statements, which sho uld be considered in the light of the risks our business faces. Please refer to the detailed statement on Page 2 of the investor presentation.

It is now my pleasure to hand over to Dr. Adil Agarwal, our Chief Executive Officer, who will share his opening remarks and insights. Dr. Adil, over to you.

Adil Agarwal : Thank you, Aashna. A very good evening to all of you, and a warm welcome to everybody for the Q4 FY '26 earnings call of Dr. Agarwal's Healthcare Limited. Before I get into the update, I want to pause and let you know about our standard performance in FY '26. For the first time in our company's history, we crossed over INR 2,000 crores of revenue, a milestone made possible by the trust of over 30 lakh patients and the dedication of every doctor and team member across our network. On the talent front, this marks a landmark year for us. And for the first time in our history, we have a total doctor strength of over 1,000 doctors across our network.

Now let me begin by providing you all with an update of the performance of the company. We set out this year with clear guidance on growth, profitability and expansion, and we delivered on each one. For the full year ended March 2026, the company reported a total income of INR 2,125 crores, up 20.9% year-on-year, while revenue from operations rose 21.6% to INR 2,080 crores. We delivered a robust Ind AS EBITDA of INR 614 crores, reflecting a 22.2% year-on-year growth with margins improving by 31 basis points to 28.9% . Our profit after tax grew 52.4% year-on-year to INR 168 crores with PAT margin expanding by 164 basis points to close at 7.9% for the year.

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Now moving on to quarter four performance. I'm pleased to share that we delivered a strong and well-rounded quarter, reflecting disciplined execution across our network in what is seasonally one of our best quarters. For the fourth quarter, the company reported a total income of INR 577 crores, up 21.2% year-on-year, while revenue from operations rose 22.6% to INR 564 crores. Q4 also delivered a robust Ind AS EBITDA of INR 174 crores, reflecting an 18.9% year-on-year growth with EBITDA margin of 30.2%, our highest quarterly margin for the year. This was delivered despite 19 new facility openings in the quarter, reflecting strong operating leverage and seasonal strength of the March quarter. Profit after t

ax grew 17.4% to INR 50 crores with PAT margins of 8.7% for Q4.

Next, I would like to give you an update on our footprint and our network. For the quarter ended March 31, 2026, we expanded our footprint by commissioning 19 new greenfield facilities, strengthening our reach and capacity. These included 7 surgical center s, including two in Telangana, one in Maharashtra, one in Delhi, one in Kerala, one in Rajasthan and one in Haryana.

Additionally, we launched 12 primary facilities across the state of Andhra Pradesh, Odisha, Telangana, Tamil Nadu and Maharashtra. India now, we have a total network of 269 facilities with 19 facilities across our African region. We are spread across 14 states and five union territories covering 155 cities.

During the full year ended March '26, we served over 30 lakh patients and performed over 3.23 lakh surgeries. Our presence is well diversified now with 30% of our facilities in Tier 1 markets, 63% of our facilities are in other markets and 7% of our facili ties are located internationally. Every day, nearly 10,000 patients now walk into our facilities across our network, up from approximately 8,000 patients in the previous financial year. This represents a 25% growth in walk-ins. This consistent growth in footfall is a testament not just to the scale of our network,

but to something more fundamental, which is the trust patients place in us and our ability to bring quality eye care to the doorstep.

Now in terms of expansion, our greenfield engine has been the defining driver of our network growth over the last four years, with FY '26 representing a culmination of a well-strategized and disciplined expansion journey. Since FY '23, we have added 148 new greenfield facilities in total, a 5.7x growth in annual greenfield additions from just 10 facilities four years ago.

In FY '26, we have launched one new facility every week, adding our footprint across 26 new cities. Notably, the mix remains balanced across both surgery and primary facilities, allowing us to deepen our clinical presence while expanding patient access across markets. This pace of expansion reflects the strong operational foundation we have built over the past several years across site selection, facility fit-outs, clinical talent onboarding and new market entry capabilities.

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Now let me turn to our clinical excellence initiatives and our focus on complex surgeries. Now for the full year ended March 2026, high-end cataract surgeries accounted for 26.3% of the 62,800 total cataract procedures performed. Within the high-end surgeries, robotic cataract surgeries, what we call as Femto Cataracts grew by a robust 87% year-on-year, crossing 5,900 Femto Cataract procedures for the full year.

Now in terms of Refractive procedures, the high ends are what we call as the Lenticular Procedures, what we call as a SMILE, and these increased by 19% year-on-year, while retinal surgeries crossed 12,800 procedures for the full year, up 23% from last year. We also completed over 1,150 corneal transplants over this entire period.

Now on the research and capability building front, our clinicians have contributed to over 360 publications in leading international journals over the past three decades, underscoring our commitment to advancing ophthalmic science. During the year, over 140 doctors underwent advanced training across multiple specialties as part of our continuous learning and development initiatives.

Now moving on to business updates. Let me first begin with our same-store sales growth update for FY '26. Our most mature facilities, those which were opened prior to FY '22, contributed INR 1,375 crores in revenue, reflecting a strong 14% same-store sales growth, accounting for 66% of our group revenues. Facilities opened in FY '23 delivered INR 257 crores, growing at 14%, while those launched in FY '24

contributed INR 170 crores, growing at a healthy 16%.

Now the FY '25 vintage, which is still early in its ramp-up phase, has generated INR 223 crores, reflecting a 72% growth over the previous year. And our latest additions of FY '26, which is the most recent cohort have contributed about INR 49 crores of revenue.

Looking at our region-wise performance. Our Southern region continues to be our largest market, contributing to 61% of our total group revenues, while continuing to deliver steady, consistent growth. This region delivered INR 1,273 crores of revenue, representing a growth of 22.6% year-on-year. We now have 182 facilities located across the five southern states, and this includes 34 new additions done in the last one year.

We continue to remain focused on sustaining our market leadership in Tamil Nadu and Telangana while further pursuing increasing adoption of the latest technologies in these regions. In Karnataka, Andhra Pradesh and Kerala, we are further strengthening our network presence by expanding significantly into high potential underserved markets through strategic facility openings.

Coming to the West region. This region contributes to 16% of our overall group revenues, and this region has delivered INR 341 crores in revenue, up 19% year-on-year. We currently have 47 facilities in the West region, including eight new facilities, which will open during the year.

In FY '26, our expansion strategy in Maharashtra was largely focused on strengthening our presence in Tier 2 markets in the rest of Maharashtra.

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In FY '27, a significant part of our expansion focus now will shift towards scaling more aggressively within the Mumbai region. This will be driven by the strong traction of our recently launched facilities in Bombay, where the brand equity also has steadily refreshed.

Maharashtra continues to be a key region for us, and we aim to deepen our presence in the underserved regions of the state. Along with this, Gujarat is where we continue to remain optimistic given the strong performance of our centers in Ahmedabad and Surat.

Now coming to the North region. This is now becoming one of the fastest -growing regions in our network. The North contributed 9% of our group revenues and reported a total revenue of INR191 crores in the last year, up 20.7% year -on-year. We currently operate 26 facilities in the North region.

Now I would like to provide you a detailed update on our expansion in the Delhi NCR region. We entered the Delhi market at the end of May 2025 with our first facility in South ex, marking the beginning of our Delhi NCR expansion journey. This facility has emerged as the anchor of our Delhi NCR cluster, delivering strong performance from the very first month of operations. Building on this momentum, we launched our second facility in Gurgaon Sector 29 in November 2025, followed by Preet Vihar in December '25, Rajouri Garden in Jan '26, Rohtak in March '26 and latest was Faridabad in April '26. All these facilities are witnessing strong traction in patient volumes as well as increasing brand recognition in their respective micro markets. We expect to open approximately seven to eight additional centers in Delhi -NCR over the next 12 to 18 months, further deepening our micro market penetration across the NCR region, including Noida, Ghaziabad and key towns in Haryana .

Now moving on to our next business update. We are planning to commission 60 new facilities across the year, 30 each in the first and second half of the year. Out of these 60 facilities, 40 will be surgical centers and 20 will be clinics. The South, our largest market, will add a further 24 facilities, deepening our presence in existing clusters and entering new micro markets. Regionally, the North will see its highest ever addition of 16 facilities, reflecting the rapid build -out of our Delhi N

CR cluster, while the West also is expected to meaningfully contribute another 15 new centers across Mumbai, rest of Maharashtra and Gujarat. Notably, both West and North regions are expected to more than double their facility additions in FY '27 as compared to FY '26.

I'm pleased to share that in this new financial year alone, we have already launched 11 new facilities. Some of the ones are in Faridabad, Ludhiana, Bikaner, Kollam in Kerala, Mulshi in Pune, Ahilyanagar in Maharashtra, Alamgiri in Jammu and Kashmir, Nellore in Andhra, Ulhasnagar, Narasaraopeta and one more center in Kerala. We also have 30 letters of intent ready for properties, and this gives us a strong visibility on the pipeline ahead.

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Moving on to our last business update. We have a new software called Neo, which is a next -generation AI -ready hospital management system developed by our in -house technology team to enable scalable and efficient eye care delivery across multi -branch networks, currently powering all our entire network and managing an average of 20,000 patients a day, Neo is built to scale beyond 5,000 branches and support over 2 million patients daily.

So looking ahead, we expect growth to sustain at a similar pace compared to the previous year. This will be underpinned by deeper penetration of existing micro markets, expansion into newer geographies and the accelerated adoption of innovative surgical procedures and advanced technologies across our network. On the margin front, we expect our EBITDA margins to stay stable even as we continue to invest aggressively in greenfield expansion, reflecting the underlying operating leverage in our model.

To conclude, our sustained focus on operational efficiency and disciplined execution has driven a strong performance across all four quarters of FY '26, and we remain well positioned to carry this momentum going forward into FY '27.

Now I would like to hand it over to our CFO, Mr. Yashwanth Venkat , who will take a deeper dive into our financial performance.

Yashwanth Venkat : Thank you, Dr. Adil. I'll begin with the operational update. Surgical services continue to be the main revenue driver, contributing 67% to the group revenue. Diagnosis, consultations and other non-surgical treatments contributed 12% and the sale of optical products and pharmacy items accounted for 21%.

For the full year ended March 2026, we performed approximately 323,000 -plus surgeries, marking a 14.5% year -on-year growth. Cataract surgeries remained the largest contributor, accounting for approximately 73% of total surgeries, followed by refractive surgeries at around 5%. Volumes for cataract surgeries grew 14.3% year -on-year, while other surgeries on a blended basis recorded a growth of 19.2%.

Our pay or mix for the full year FY '26 stood at 62.4% from cash, 28.5% from insurance and TPA and 9% from government schemes. The domestic pay or mix stood at 72% from cash, 22% from insurance and TPA and 6% from government schemes.

Moving on to the financial section. I'll start with the revenue split. The group's revenue from operations for Q4 FY '26 grew by 22.6% year -on-year, reaching INR565 crores in Q4 FY '26 compared to INR460 crores in FY '25.

Revenue from operations in India for Q4 stood at INR505 crores, reflecting a growth of 22.2% year-on-year. This growth was supported by near equal contributions from both volume and value of close to 14% with the remaining contribution coming from new centers opened in FY '25 and FY '26.

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For the full year, revenue from operations in India stood at INR1,871 crores, reflecting growth of 21.7% year-on-year. Revenue from our Africa operations grew by 20.8% year-on-year for the full year, though their contribution to overall revenue remained broadly stable at approximately 10.1%.

Gross margins slightly improved year-on-year despite the higher contribution of surgeries in segment mix with high-end cataract procedures rising to 26.3% in FY '26 when compared to FY '25 of 22.5%. Doctor and employee costs have remained stable at approximately 33% of total revenues in FY '26 despite the addition of talent across all facilities launched in Q1 FY '27. Other expenses as a percentage of total revenue have remained stable at 16.6% of total revenues. On the finance cost front, the improvement has been driven by two key factors: a total of INR195 crores in loans have been systematically repaid from IPO proceeds, INR128 crores in Q4 FY '25 and INR67 crores in H1 FY '26. Concurrently, interest on deferred consideration arising from past acquisitions have also been declining as outstanding payments are made, providing further reduction in our overall finance cost.

From 75.6%, the share of profit after tax attributable to the owners has expanded to 79.2% in FY '26, signaling improved profitability in the holding company. Lastly, our CFOs to EBITDA were at healthy levels of approximately 84%.

Now just to summarize, we are pleased to report good financial results for FY '26, driven by solid year-on-year growth across key metrics. Looking ahead, we expect growth to sustain at a similar pace to the prior year, underpinned by deeper penetration of existing micro markets, expansion into new geographies and the accelerated adoption of innovative surgical procedures and advanced technologies across our network.

Before opening the floor to questions, I would like to provide a brief update on the merger process. For the proposed merger of Dr. Agarwal's Healthcare Limited and Dr. Agarwal's Eye Hospital Limited, we have continued to make steady progress on the regulatory process. During February 2026, we received stock exchange observation letters with no adverse objections following which we filed the NCLT application. In May 2026, the Honorable NCLT Chennai bench allowed the joint first motion application and directed the convening of meetings of equity shareholders, secured and unsecured creditors of both companies on July 2, 2026. We will continue to update the exchanges as we progress through the remaining steps, including shareholder approvals and the final NCLT sanction.

Thank you all. We'll open the floor to questions.

Moderator: Thank you very much. We will now begin the question-and-answer session. Our first question is from the line of Tushar Manudhane with Motilal Oswal Financial Services. Please go ahead.

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Tushar Manudhane : Thanks for the opportunity and congratulations on good set of numbers. So just on the capex breakdown, if you could give or -- and like for the 60 facilities which you're going to add in FY '27? That's my first question.

Adil Agarwal : Yes. So Tushar. Hi, Tushar.

Tushar Manudhane : Yes, sir, you're audible.

Adil Agarwal : So for this year, we are looking at an overall outflow of about INR 380 crores to INR 400 crores, which includes all our capex for this year, including our new CMS facility. We're looking at adding about 40 surgical facilities and 20 clinics, and this would envisage a total CapEx spend of about INR 380 crores to INR 400 crores. And this includes the CapEx spend for our new CMS facility.

Tushar Manudhane : And how much would be the outflow -- cash outflow in terms of the acquisition-related payments, which are expected to be happening in FY '27?

Yashwanth Venkat : Tushar, it will be approximately to the tune of about between INR 60 crores to INR 65 crores, Tushar.

Tushar Manudhane : And if you could just clear me, sir, how much was that for FY '26?

Yashwanth Venkat : For FY '26, it was around close to INR 85 crores.

Tushar Manudhane : Got it. Sir, just secondly, on the West, as I see for the full year, the tertiary facility

was not added,

while we do have a reasonable number of centers. Any particular reason? And subsequently, how are we thinking out of 16 to be added in North, any tertiary facility that would get added?

Adil Agarwal : So you're talking about in the last year, we are adding -- we have added a tertiary facility in Thane West.

Tushar Manudhane : Sorry, I meant West region, not North, sorry.

Rahul Agarwal : Hi, Tushar, this is Rahul. We have added one tertiary facility last year. In this year, we are looking at adding one more tertiary facility in West. In fact, we've just started that facility in Thane, which we are calling the Thane Main Hospital. And that's a facility which is having all the technology, and we're just in the first few days of launch.

Tushar Manudhane : Got it. Got it. That's it from my side, sir. Thank you.

Moderator: Thank you. Our next question is from the line of Rachna Kukreja with SIMPL . Please go ahead.

Rachna Kukreja : Thank you for the opportunity and congratulations on a good set of numbers. My questions are related to the listed subsidiary. In Q4, other expenses saw an increase both on a yearly and quarterly basis. So what were the reasons for that?

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Yashwanth Venkat : In Q4, the other expenses actually pertain to onetime expenses incurred towards the merger process. Also, we had -- we have recently moved our corporate office in December. So the other expenses are comprised of that expense -- rental expense as well. On the merger, it is more towards the finance opinion provider and the legal consultant and the retail outreach consultant.

Rachna Kukreja : Okay. So would these expenses continue or were they onetime? We expect this to be there in...

Yashwanth Venkat : Merger expenses are onetime. The corporate office rent, it will continue.

Rachna Kukreja : Okay. My second question would be on the subsidiaries facility in Chennai. If you could give an update and the plans to complete it, the time line?

Adil Agarwal : So we are right now in the process of seeking all the final approvals to get the project ready,

Rachna. So we should be -- we are targeting a launch of -- hopefully, by October 1, 2026, is when we should have the new project up and running. We are working towards these milestones.

Rachna Kukreja : Okay. And you said...

Moderator: Sorry to interrupt, ma'am, we request you to please return to the queue if you have any further questions. Thank you. Our next question comes from the line of Maulik Varia with 360 ONE Capital . Please go ahead.

Maulik Varia : Hi, sir. Thank you for the opportunity and congratulations on good set of numbers. I hope I'm audible. I just wanted to understand...

Yashwanth Venkat : Yes, Maulik, we can hear you.

Maulik Varia : Yes, I just want to understand how much has been our value and volume growth? I think you clarified, but I missed the statement, I guess, in terms of our surgeries revenue.

Adil Agarwal : Sure. Rahul will take this.

Rahul Agarwal : Yes. So overall, for the year, we've had a 22% -odd revenue growth, out of which, if I were to just look at the SSSG, that will be in the range of around 14%. In this SSSG, our volume growth is close to 7% and the value growth also is similar at 7%. Now in the volume growth, comprising of OPD growth is in the range of around 5.5% and the remaining comes from increased conversions. In the value, premiumization would be in the range of 5% and another 1.5% comes from price hike. So that's broadly the breakup of value and volume for us for FY '26.

Maulik Varia : Understood. Sir, you said 5.5% from premiumization, right, in the value growth?

Rahul Agarwal : Correct. Right.

Maulik Varia : Yes. Okay, okay. Thank you. I have more questions, I'll join back the queue. Thank you.

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Moderator: Thank you. Our next question

is from the line of Dishant Jain with Quasar Capital. Please go ahead.

Dishant Jain : Yes. Thanks for the opportunity and congratulations on a good number. Sir, can you quantify the onetime expense that we incurred in subsidiary financials?

Yashwanth Venkat : Yes, we can. As far as the merger expenses are concerned, it is about close to INR 80 lakhs.

Then the rental payout -- the cross charge towards the rental payout towards corporate office, all put together for the year, it is about close to about INR 80 lakhs to INR 1 crore. And there was also an increase in marketing expenses, especially on Facebook marketing. For the year, it was about to the tune of about INR 3.3 crores.

Dishant Jain : Is it possible to give it for the quarter, last Q4?

Yashwanth Venkat : Right now, probably on the call, no, probably we can connect separately.

Dishant Jain : Sure. Okay. Thank you.

Moderator: Thank you. Our next question comes from the line of Lakshminarayanan K G with Tunga Investments. Please go ahead.

Lakshminarayanan K G: Yes. Thank you. Exceptional performance across all parameters. Two questions. One is how many facilities are, maybe -- my questions pertain to AEHL. How many facilities we opened in FY '26? And what is the plan for FY '27? And the second question is that for AEHL, what has been the cumulative rental expenses across all the facilities and the corporate office put together? These are my two questions.

Yashwanth Venkat : Good evening, sir. In terms of number of facilities, we had opened four facilities, sir, two surgical facilities, one in Red Hills and other in Tirupattur , and also we had opened two eye clinics as well. And during the year, we had shut down two eye clinics, in Karaikal and other in Ariyalur. So the number of facilities on a network basis grew from 61 to 63 facilities.

Lakshminarayanan KG: Sorry, hello?

Moderator: Excuse me, sir, this is the operator. If you're speaking right now at the moment, you are not audible.

Lakshminarayanan KG: No, I can't hear the management speak.

Aashna Dharia : Can you hear us?

Moderator: You are audible, yes.

Lakshminarayanan KG: Yes.

Yashwanth Venkat : Sir, in terms of the overall

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Lakshminarayanan KG: Sorry, just Yashwanth , I lost you. You mentioned that there were 63 network hospitals we closed at FY '25. Is it FY '26? Is that what I hear?

Yashwanth Venkat : Yes, sir. From 61, we moved to 63, sir. We have opened four facilities during the year, and we have closed two eye clinics. In those four facilities, two were surgical facilities, Red Hills and Tirupattur and two other -- two eye clinics we had opened in Bhavani and Thirukovilur . Two eye clinics in Karaikal and Ariyalur, we had closed. In terms of the rent...

Lakshminarayanan KG: I also asked on FY '27, how many facilities you intend to open and the rental, yes.

Yashwanth Venkat : Sir, currently, we'll not be able to comment on the number of facilities for FY '27. On the rental expenses, the overall number will be to the tune of about close to INR 32 crores, sir.

Lakshminarayanan KG: INR 32 crores for AEHL .

Yashwanth Venkat : Yes, sir .

Lakshminarayanan KG: For AEHL ?

Yashwanth Venkat : For AEHL, sir. Correct, sir.

Lakshminarayanan KG: Okay. Thank you.

Moderator: Thank you. Our next question comes from the line of Rachna Kukreja with SIMPL . Please go ahead.

Rachna Kukreja : Thank you for the opportunity. If you could repeat the quantum of other expenses for Q4 and the entire year, it will be really helpful.

Yashwanth Venkat : Ma'am, we have the numbers for the entire year. I'll just repeat it again. As far as the marketing expense is concerned, there was an increase in spends towards Facebook to the tune of about close to INR 3.3 crores. Then the rental payment process towards corporate office for the whole year was close to INR

1 crore. For Q4, there was a onetime expense increase towards merger, which was close to about INR 80 lakhs split across three vendors, ma'am.

Rachna Kukreja : Okay. You provided about the approval -- about the update on the merger. But if you could also give the time line by when we can expect the merger to be completed?

Aashna Dharia : Hi, ma'am. So we are expecting the -- as mentioned earlier, July 2 is when the physical EGM will be held for voting of the shareholders, secured and unsecured creditors. Post the voting and post the results of vote public, we will again go back to the NCLT and await their approval. All of this -- to conclude all of this, we are expecting another five months or so, five to six months.

By end of this year, Q3 of this year, we'll expect to conclude it.

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Rachna Kukreja : Okay. And one more question. We have plans to complete the CapEx related to the listed facility by Q2 FY '27. And it was mentioned that it will be done by October. So any reason for a delay of a month?

Adil Agarwal : We have been going through certain challenges when it comes to getting approvals because of what has been happening with the entire election and because of code of conduct and everything

so getting these approvals have taken a bit of a delay. But other than that, we don't see too much of a challenge in getting this done in a timely fashion.

Rachna Kukreja : Okay. Thank you.

Adil Agarwal : Thank you.

Moderator: Thank you. Participants, you may press star and one to ask a question. Our next question comes from the line of Maulik Varia with 360 ONE Capital . Please go ahead.

Maulik Varia : Thank you for the opportunity once again, sir. Just wanted to have some broad understanding or some direction regarding any margin accretion or maybe if you can quantify or give some direction for our mature facilities or for our facilities, which have been doing well. So how much has been the margin accretion for those facilities in terms of operating leverage, which you were mentioning? I understand the reported margin would have a blend of some facilities which were new and investments towards those. But for the existing facilities, any understanding?

Adil Agarwal : Just one second? Rahul?

Rahul Agarwal : Hi, Maulik . So can't give you exactly what the margin accretion has been in our mature facility. But what I can tell you is that last year, so FY '26, the number of branches that new greenfields that we opened, we sustained a loss of around INR30 crores in -- from a unit economics perspective of those branches. So that's the kind of losses which has taken away some of our margin from that. So you could do a back calculation on what the other part of the business has grown.

Maulik Varia : Okay. We had a burn of INR 30 crores in FY '25. So that has...

Rahul Agarwal : No, no, branches which opened in FY '26.

Maulik Varia : Okay. Those new branches have had a burn of INR 30 crores in FY '26. Understood. Understood. Yes, sir. Okay, okay this will be helpful. Thank you.

Moderator: Thank you. Our next question comes from the line of Nikhil Upadhyay with SIMPL. Please go ahead.

Nikhil Upadhyay : Yes. Hi. Good evening. Congrats on the good set of numbers. I hope I'm audible.

Adil Agarwal : Yes, Nikhil, yes, you can go ahead.

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Nikhil Upadhyay : Yes, just two questions. One is a bookkeeping. So post FY '26, what is the amount of payments which are pending for the acquisition related? I think we had to -- like it was to be completely paid off by '28. So just some sense, how much is left? And what would be the payout for next two years?

Yashwanth Venkat : Okay. For FY '27, it will be to the tune of about close to INR 65 crores. This is -- I'm just talking about the current -- I mean, the acquisitions which are on presently.

Then in FY '28 -'29 and small payments in '30 put together, that will be to the tune of about INR60 crores.

Nikhil Upadhyay : INR 60 crores?

Yashwanth Venkat : Yes, INR 66 crores. Then FY '28 it will be -- FY '28, '29 and '30, put together INR 60 crores.

Nikhil Upadhyay : Okay. And just one question. See, to the last participant, you mentioned that the negative delta or loss for the new facilities which we opened was INR 30 crores in '26. What was the similar number in '25? And to be a little more specific in regions where we opened in '25 and when we opened in '26, have the losses come down? Or is it at a similar level?

Rahul Agarwal : So of course, now we'll be talking about the FY '25 branches which opened, and we are giving the losses which happened in FY '25. So that number was closer to around INR 21 crores, which was around INR 30 crores for the FY '26 branches, which opened in FY '26. So different cohort of branches, but I'm giving you the operating losses of those cohorts.

Nikhil Upadhyay : Okay. Just an extension. Say, in North, if you opened five branches in FY '25. And if you open a similar kind of branches in FY '26, would the losses be lower? Or are they at a similar level? Just to understand that how has the brand evolved over -- like as we are opening the stores, are the losses coming down in the year one? Or are they at a similar rate?

Rahul Agarwal : No. So FY '26 is where we have opened the Delhi branches. Of course, there's been significant losses in the INR 30 crores from Delhi. We opened a tertiary branch there, large facility first time in Delhi. I'm quite sure that at a per branch level, our loss per branch would start reducing from FY '27 onwards, while our growth in revenue will continue very strongly and the number of branches that we are planning to open also, as you would have seen from the presentation, is quite strong for FY '27 in Gurgaon and Delhi.

Nikhil Upadhyay : Okay. Fine. Okay. Thanks. Yes.

Moderator: Thank you. Our next question comes from the line of Harsh Bhatia with Bandhan Mutual Fund. Please go ahead.

Harsh Bhatia : Yes. Thank you. Hope I'm audible ?

Adil Agarwal : Yes.

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Harsh Bhatia : Sure. Thank you. Just one clarification. What was the sales for that particular cohort where you're calling out the INR 30 crore s drag? What was the sales for the centers that were opened in FY '26?

Yashwanth Venkat : I mean we have shared with the -- in this -- we'll come back on this exact number.

Harsh Bhatia : Sure. And do we expect a similar sort of incremental drag given that your North presence will significantly pick up this year in FY '27. So should we build in that INR 30 crores to INR 40 crores drag -- fresh drag as well for the centers that will open up in FY '27?

Rahul Agarwal : Yes. To a large extent, you're right. Just that this year, our number of branches that we are -- surgical branches that we're looking at opening would be higher. But overall, yes, directionally, you're absolutely right.

Harsh Bhatia : Sure. Thank you.

Moderator: Thank you. Participants, you may press star and one to ask a question. Our next question is from the line of Lakshminarayanan K G with Tunga Investments. Please go ahead.

Lakshminarayanan KG: Yes. Thanks again. One question. I mean, as you scale up, you are going across different parts of India. And how do you intend to homogenize the protocols and the talent? Do you intend to -- you already have a training infrastructure where you would actually get your paramed ical staff and the others to ensure that the similar protocols and the similar systems and processes are followed. And how are you planning to ensure that the doctors and even the senior doctors would be aligned in terms of having the same process?

Adil Agarwal : So Mr. Lakshmi, we firstly have something which is interesting, which is cal

led the clinical

board. Now the clinical board is handled by some senior doctors in the company who coordinate and oversee activities of the doctors across the network. They effect ively ensure that there is certain standardization, which is maintained across the country at a group level. So whether you go to Srinagar or whether you go to Tirunelveli, you'll maintain a certain level of quality in terms of clinical outco mes and the clinical board ensures that those outcomes are maintained.

Now when it comes to paramedical training, one of the most important paramedical roles is that of an optometrist. Optometrists play a very supportive role to the doctors. So we run our own institute of optometry firstly, where students come in and actually undergo intense training program for three years and then they do one year of internship. And they become your foot soldiers and they end up joining your hospitals and they end up doing a lot of optometry work. I remember listening on to the earnings call by Peyush Bansal when he was talking about there being a dearth of optometrists. This is something which we have actually figured out many, many years back, and we actually run our own schools of optometry. So they effectively come in and play a very significant role in t erms of managing a lot of the paramedical work, which supports the doctors.

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Now in terms of training the doctors, we have three institutions, one in Chennai, one in Tirunelveli, one in Cuttack, where we actually run post -graduate training programs. We have also recently set up a training program, state-of-the- training program in Vashi in Navi Mumbai. And these act as training centers for many of our doctors who can undergo super specialty training. And that's how we continue to focus on learning and development for both our doctors and our paramedical staff. I hope this answer s.

Lakshminarayanan KG: Yes, yes. So that's good. And in terms of attrition at paramedical, what kind of attrition you see and what kind of inflation in remuneration you see?

Yashwanth Venkat : In terms of attrition at paramedical levels, it has ranged from 21% to 25% over the years, sir. And in terms of -- I think you asked about fixed increments, more or less in line with industry between close to 9.5% to 10%.

Lakshminarayanan KG: Got it. Got it. Thank you.

Moderator: Thank you. Our next question is from the line of Punit Pujara with Invesco Mutual Fund. Please go ahead.

Punit Pujara : Yes. Thanks for taking my question and I hope you can hear me.

Adil Agarwal : Yes, hi, Punit.

Punit Pujara : Yes, hi. So just one question, Dr. Adil. If we look at over the last three years, your number of facilities that you add each year, I mean, FY '25, '26 and the outlook for '27 is remain -- has remained in that 52 to 60 kind of a range. What are the bottlenecks for you to, let's say, accelerate this number from 60 to a bigger numb er? While I hear you that the surgical mix , the salience has increased from 20, 30 and now 40, but the overall number, how can you accelerate that further

and what are the bottlenecks? Thank you.

Adil Agarwal : Yes. So I think what we have actually -- what we're actually doing is actually accelerating the rate at which we're opening our centers. So just from that perspective, if you see last year, we -- this year, we have opened 30 surgical centers. We are planning to open 40 surgical centers, which is a 33% increase in the volume of new center additions.

Now we can go and put up 70, 80 centers, but the biggest bottleneck for us, obviously, will be getting the right real estate because one is you want to ensure that you get the right real estate at the right cost is where you want to be cognizant of some of the costs.

Second, you have to get the real estate with the right kind of compliance because we are governed by each local state,

local clinical board. So we have to ensure that we get all the license in line with some of the local authorities. And getting that compliance in most of the cities is a bit of a challenge. That's why we see sometimes a bit of a delay.

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If we didn't have that bottleneck, then we can be like many of the retail stores and just we don't need to worry about clinical licenses, given that we need clinical license across all of our hospitals, that one extra layer of compliance is what makes it a bit tricky for us to get the right real estate.

That said, whatever the team has done to actually open one hospital a week, one center a week is no small task. And some of the work for getting this done has actually been put into place many, many years back, and we have built that kind of a pipeline. We have built that kind of a team, which is actually able to deliver these kind of time lines in terms of opening new centers. But that said, we will continue to monitor and see how our centers perform. And if we are able to do well, then we will definitely look at ramping up our network presence from the current 60 that we have envisaged for this year.

Punit Pujara : Sure. No, I agree that the team has been able to deliver fantastic outcomes. And hence, the teams, those deliver the outcomes like this get asked these questions, right? Thank you so much and all the best.

Adil Agarwal : Thanks, thank you.

Moderator: Thank you. Our next question comes from the line of Harith Ahamed with Avendus. Please go ahead.

Harith Ahamed : Hi, Dr. Adil and team. Thanks for taking my questions. So the first one is on the growth by vintage, which you shared for the FY '22 cohort, if I heard correctly, you shared a 14% growth, and that's quite impressive. So I'm just trying to understand if there is a volume versus realization breakup for this 14%? And is it more skewed towards realizations because mature centers typically do more advanced procedures? And how should we think about this 14 -odd percent growth for mature centers? Are these levels sustainable?

Rahul Agarwal : Hi. Yes. This is Rahul. So I mentioned this a few minutes back. But overall, our 14% growth can be broken down into 7% volume and 7% value growth. 7% volume growth, almost 5.5% would come in from our walk-ins, which is growing and the remaining would be superior surgical conversions. Again, on a value growth of 7%, around 5.5% would come in from premiumization and the remaining 1.5% would come in from price hikes. So that's the broad composition of the 14% value and volume breakdown in all our mature facilities.

Adil Agarwal : Which is sustainable.

Rahul Agarwal : I feel broadly, it has been sustainable for us for the last three to four years. So directionally, yes, I can't go to the last percentage, but I think directionally, OPD growth has been growing. So we don't see an issue there. Premiumization has been on track. And one of the things, for example, right now, we are at INR 45,000 – INR 41,000 average cataract per eye. And we do see opportunities with our new technologies with higher penetration with more disposable income for the premiumization to continue for a while. So price hike, 1.5% has been -- 1% to 1.5% is

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average that we do end up getting. So in this range, we do feel comfortable from a broad few years perspective.

Harith Ahamed : Got it. The second question is on the surgical volumes on the refractive side. So this year, we've seen a softness, which is quite different from the strong trends we've seen in previous years. So what are some of the drivers here, both from a near-term recovery standpoint as well as slightly longer-term contribution from refractive surgeries as a percentage of our overall surgeries, what can take this up?

Rahul Agarwal : So I would sa

y that between cataract and refractive, cataract has grown faster. And while refractive has also grown, but the growth has not been commensurate to the cataract growth. So yes, that is one -- definitely one part. We've seen slight slowness. However, if you see our high - end procedures, which is the Lenticular Procedures, they have grown much faster at 19% . So we are also slowly premiumizing our procedures. Our yield per patient is growing in refractive as well. So that's also been our focus to start looking at some of the higher -end procedures. From a long -term perspective, I think refractive as a market continues to remain. I think the vision issues and all of that continues. So we don't see a long -term issue in refractive as a procedure. But yes, I think there's also some support that we are seeking in the insurance side of it, and we are talking to the insurers. Once that gets settled, I think that's also an area where we can continue to grow from a long -term perspective.

Harith Ahamed : Thank you. That's all from my side.

Adil Agarwal : Thanks.

Moderator: Thank you. Our next question comes from the line of Tushar Manudhane with Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane : Thank s for the opportunity again , sir. So interestingly, this year, FY '26, we had all the facility addition on an organic basis. Likewise, can you share FY '27 facility addition? Is that all organic is what currently you are building in?

Adil Agarwal : As of now, Tushar, all the 60 centers that we have planned, which is the 40 surgical centers and the 20 clinics are all organic. But that we continue to look if we are able to get partners who are align with us and if we get them at the right price, then we will consider but that is not included in the pipeline slide. Our plan is to focus on organic growth ..

Tushar Manudhane : Got it. And broadly, how many facilities would have closed in FY '26 to give that net addition of 54, 57 something?

Adil Agarwal : Yashwanth.

Yashwanth Venkat : Tushar, we have closed five eye clinics in FY '26.

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Tushar Manudhane : Got it. And lastly, out of these 60 new facility additions , how much would be greenfield or basically getting into new regions, either city -wise, you may say or Tier 2 cities, whichever way you want to put in ?

Adil Agarwal : So we have entered 26 new cities, Tushar, this year? 26 new cities we have entered in this new financial year.

Tushar Manudhane : Sir, I meant to ask for this with respect to 60 facilities for FY '27.

Adil Agarwal : FY '27, I'll have to come back to you on the exact number in terms of how many are new market. I'll come back to you to you with the exact number.

Tushar Manudhane : Got it. Thanks a lot for addressing my questions .

Moderator: Thank you. Our next question comes from the line of Varun Hiremath , an individual investor. Please go ahead.

Varun Hiremath : Yes. Hi, to the whole team and thanks for the opportunity. I wanted to ask regarding the facility which we're commissioning, which is the large new hospital in Chennai. Now that we're nearing the completion of that and a lot of money has been spent on establishing this facility. Could we get some kind of guidance as to what we might expect that will contribute to our revenue and profitability going forward?

And the second question I had is about the other expenses having gone up quite considerably in the subsidiary this quarter. So was there any one -off items which resulted in this? Or is there a material change in the other expenses going forward as well?

Adil Agarwal : Yes. Yashwanth will just answer this other expenses.

Yashwanth Venkat : Hi, Varun, on the other expenses, as explained earlier, the one -off is towards the merger expense, Varun, that will be to the tune of close to about INR 80 lakhs. And also I had mentioned in terms of

the increase in Facebook spends to the tune of about INR 3.3 crores for the year. And we also moved to a new corporate office in December. So the rental payout charge towards the corporate office is also part of the other expenses, Varun. The one -off is the merger expense.

Varun Hiremath : Okay. Okay.

Adil Agarwal : And Varun, on the new CMS facility, we will be commissioning that facility. Hopefully, if we get all the approvals and everything depends on us getting those approvals by October 1, 2026. And we're hoping to see a sizable improvement in both revenue and pr ofitability once we move on to the new facilities. But as with any new movement whichever happens, we will have to give it some time 12 to 18 months to see where this business will stabilize there. It's difficult to predict that right now. But we're hoping to see a significant increase in both volumes and realization

once we move into the new facility.

Varun Hiremath : Okay. Okay. And just we'll be closing down the existing facility that is currently...

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Adil Agarwal : No, no. We'll be running both the facilities.

Varun Hiremath : Okay, perpetually?

Adil Agarwal : We don't know. We don't know that yet. It's too early to say. But once we move in, we hope to create enough volumes that we should be able to run both the facilities. That's our endeavour.

But we have to see 12 to 18 months down the line in terms of how feasible it is to run both facilities simultaneously.

Varun Hiremath : Okay. All right, okay. Thank you so much.

Adil Agarwal : Thanks.

Varun Hiremath : Best of luck, yes.

Moderator: Thank you. Ladies and gentlemen, we will end our question -and-answer session here. On behalf of Dr. Agarwal's Healthcare, that concludes this conference. Thank you all for joining us. You may now disconnect your lines.

Adil Agarwal : Okay. Thank you very much.

Yashwanth Venkat : Thank you.